

Department of Health



Annual Report 2025



Department of Health

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The United Nations Relief and Works Agency for Palestine Refugees in the Near East (UNRWA) is a United Nations agency established by the General Assembly in 1949 with a mandate to provide humanitarian assistance and protection to registered Palestine Refugees in the Agency's area of operations, namely the occupied West Bank, including East Jerusalem, the Gaza Strip, Jordan, Lebanon and Syria, pending a just and lasting solution to their plight. Thousands of Palestine Refugees who lost both their homes and livelihoods because of the 1948 War have remained displaced and in need of significant support for more than 75 years. UNRWA helps them achieve their full potential through high-quality services in education, healthcare, relief and social services, protection, camp infrastructure and improvements, microfinance, and emergency assistance. UNRWA is funded almost entirely by voluntary contributions.

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A Palestine Refugee baby receiving child healthcare services from an UNRWA nurse at Deir 'Ammar Health Centre in the occupied West Bank, including East Jerusalem. ©2025 UNRWA photo.

Table of Contents

Vision and purpose of the 2025 Annual Health Report.....	9
Holistic care themes.....	9
Health profile.....	10
Plain summary.....	11
Health workforce and infrastructure.....	14
Registered population.....	15
Serviced population.....	18
Workload.....	18
Contexts.....	19
Data management digitalisation for monitoring, support, and credibility: The e-health software.....	19
Health communication.....	20
 Outcome 1: Palestine refugees' maternal and child health.....	 22
Maternal health.....	22
Overview.....	22
Trend of mother's health statistics and indicators over the past years.....	24
Gender-based violence (GBV).....	28
Infant and child health.....	29
Infant and childcare.....	29
Infant and child (<5 years) mortality.....	30
School health.....	31
Screening of children under five for disability.....	32
Nutrition.....	33
Nutrition surveillance system on child malnutrition in the Gaza Strip.....	33
Nutritionists' work in the occupied West Bank and Lebanon.....	34
Nutrition education.....	35
Multiple micronutrient supplementation (MMS).....	35
Expanding Health Department staff capacity for nutrition knowledge and practices at UNRWA.....	35
 Outcome 2: Improved control status of non-communicable diseases.....	 36
Non-communicable diseases (NCDs).....	36
Overview of the programmes.....	36
Measures, standards, interventions, and guidelines followed to ensure high-quality care.....	37
Outcomes achieved and expected impact of implemented outputs.....	37
Challenges and limitations hindered optimal outcomes and impact.....	37
Health service delivery amid crises and instability.....	38
Future outlook, planned actions, and strategies for sustaining achievements and managing emergencies.....	39
Diabetes mellitus (DM) and hypertension (HTN).....	40
Prevention of diabetic foot and retinopathy among Palestine Refugees in Syria and Lebanon: World Diabetes Foundation (WDF) Project.....	45
Mental health.....	46
 Outcome3: Improved status of communicable diseases and infectious disease prevention and control.....	 48
Infectious disease surveillance.....	48
Immunisation.....	49

Table of Contents

Outcome 4: Access to quality and safe health services for Palestine Refugees.....	51
Pharmaceutical services.....	51
Training.....	52
Hospitalisation.....	53
Outpatients.....	59
Oral and supportive health services: Expanding access to quality care.....	60
Radiology services.....	60
Physical rehabilitation.....	60
Oral health.....	61
Service delivery in crisis contexts.....	62
Strengthening laboratory quality and capacity.....	62
Programmatic initiatives: Promoting spinal health among UNRWA students in the occupied West Bank,.....	62
Telemedicine.....	63
How telemedicine improves health and service.....	63
Achievements during 2025.....	64
Overview of UNRWA's humanitarian response and healthcare challenges amidst the crises in the Gaza Strip and occupied West Bank.....	66
UNRWA health response in the Gaza Strip during the 2025 emergency.....	66
Health services provided in the Gaza Strip in 2025.....	70
How data was reported and validated.....	70
Reporting from bombardments, incursions, and IPC Phase 5 conditions: data quality amid a severe humanitarian crisis.....	70
Reviving Gaza Strip health reports and indicators in 2025.....	74
The occupied West Bank, including East Jerusalem.....	74
Northern occupied West Bank.....	78
East Jerusalem.....	78
Where services exist but not for all: UNRWA's role in addressing gaps in East Jerusalem.....	78
Telemedicine in the middle of the escalation.....	79
Evaluation of UNRWA's Health Department emergency response in the Gaza Strip and the occupied West....	79
A step forward: Moving to a new electronic medical record software.....	80
Strategic health indicators across (Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem).....	81
Research and information dissemination.....	83
Health Department research activities and published papers 2025.....	83
Disseminated Abstracts: Delivering life-saving diabetes care during the crisis.....	84
Technical staff in the Department of Health, HQ-A.....	86
Department of Health organisation chart.....	87
Annexes.....	88
References.....	88

Table of Figures

Figure 1: Distribution (%) of registered refugees by field in 2025, including the Gaza Strip.....	15
Figure 2: Age and sex distribution of registered Palestine Refugees in the five fields in 2025.....	17
Figure 3: Age and sex distribution of registered Palestine Refugees in the Gaza Strip in 2025.....	17
Figure 4: Age and sex distribution of registered Palestine Refugees in Jordan in 2025.....	17
Figure 5: Age and sex distribution of registered Palestine Refugees in Lebanon in 2025.....	17
Figure 6: Age and sex distribution of registered Palestine Refugees in Syria in 2025.....	17
Figure 7: Age and sex distribution of registered Palestine Refugees in the occupied West Bank, including East Jerusalem, in 2025.....	17
Figure 8: Number of individuals serviced by UNRWA health centres in its five fields, in 2025.....	18
Figure 9: Number of medical consultations by sex in UNRWA's five fields in, 2025.....	18
Figure 10: Average daily medical consultations per medical officer in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.....	18
Figure 11: Number of women newly registered in preconception care service in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, from 2017 to 2025.....	24
Figure 12: Number of newly registered pregnant women in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, from 2022 to 2025.....	24
Figure 13: Percentage (%) of pregnant women registered in antenatal care service in the first trimester in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, from 2022 to 2025.....	25
Figure 14: Number of pregnant women attending antenatal care services at any trimester in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, from 2017 to 2025.....	25
Figure 15: Number of pregnant women with live births in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, from 2017 to 2025.....	25
Figure 16: Percentage (%) of women covered by postnatal care services in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, from 2017 to 2025.....	26
Figure 17: Number of new family planning acceptors in Jordan, Lebanon, Syria, the occupied West Bank, including East Jerusalem, from 2017 to 2025.....	26
Figure 18: Percentage (%) of pregnant women with alert risk status in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, from 2022 to 2025.....	26
Figure 19: Percentage (%) of pregnant women with high-risk status in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, from 2022 to 2025.....	27
Figure 20: Percentage (%) of deliveries by caesarean section relative to total births in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, from 2022 to 2025.....	27
Figure 21: Numbers of pregnant women screened for gender-based violence and those identified with gender-based violence exposure, in Jordan, Lebanon, Syria and the occupied West Bank, including East Jerusalem, in 2025.....	28
Figure 22: Percentage (%) of gender-based violence survivors identified out of the total women screened for gender-based violence, in Jordan, Lebanon, Syria and the occupied West Bank, including East Jerusalem, in 2025.....	28
Figure 23: Proportion (%) of growth problems in children (<5 years) in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, from 2022–2025.....	30
Figure 24: Distribution of causes of infant and child (<5 years) mortality by living setting (In-camp, outside camp) in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.....	30
Figure 25: Distribution of causes of infant and child (<5 years) mortality by sex in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.....	30
Figure 26: Number of children newly registered in schools and those medically examined in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.....	32
Figure 27: Proportion (%) of patients with noncommunicable diseases with controlled conditions in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.....	40
Figure 28: Percentage of diabetes patients with controlled HbA1c in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, from 2022 to 2025.....	40
Figure 29: Proportion (%) of patients with diabetes mellitus controlled by sex in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.....	41

Table of Figures

Figure 30: Proportion (%) of diabetes mellitus patients by glycated haemoglobin level in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.....	41
Figure 31: Distribution (%) of diabetes mellitus patients by HbA1c level, four fields excluding the Gaza Strip, in 2024-2025.....	41
Figure 32: Prevalence (%) of risk factors among patients with noncommunicable diseases by disease type in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.....	42
Figure 33: Distribution of NCD patients by their risk score in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.....	42
Figure 34: Distribution (%) of overall noncommunicable disease patients by their risk score in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.....	42
Figure 35: Prevalence (%) of late complications among patients with diabetes mellitus and hypertension in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.....	43
Figure 36: Prevalence (%) of late complications among noncommunicable disease patients by morbidity in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.....	43
Figure 37: Prevalence (%) of late complications among noncommunicable disease patients by control status in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.....	43
Figure 38: Distribution (%) of types of late complications among patients with noncommunicable diseases in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.....	44
Figure 39: Number of disease-specific deaths in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.....	44
Figure 40: Distribution of disease-specific mortality in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.....	44
Figure 41: Distribution of NCD defaulters in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.....	45
Figure 42: Number of individuals screened, identified, and assisted with general health questionnaire-12 (GHQ-12) screening and mental health and psychosocial support programme in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.....	47
Figure 43: Number of immediate notifiable disease cases in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.....	48
Figure 44: Number of expanded programmes on immunisation (EPI) targeted disease cases in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.....	49
Figure 45: Vaccination coverage (%) of infants (<1 year) in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.....	49
Figure 46: Coverage (%) of vaccine booster doses and vitamin a in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.....	50
Figure 47: Antibiotic prescription rate (%) in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.....	50
Figure 48: Number of healthcare workers who earned the professional diploma from the course for primary care nurses/ midwives across UNRWA's five fields of operation, from 2022 to 2025.....	53
Figure 49: Number of practical nurses who completed the online course in primary healthcare and reproductive health, across UNRWA's five fields of operation, from 2023 to 2025.....	53
Figure 50: UNRWA inpatient hospitalisations across UNRWA's five fields of operation from 2021 to 2025.....	53
Figure 51: UNRWA inpatient hospitalisation by gender, across UNRWA's five fields of operation, from 2024 to 2025.....	54
Figure 52: UNRWA inpatient hospitalisation by gender across UNRWA's five fields of operation in 2025.....	55
Figure 53: Chronic disease and cancer patient supported across UNRWA's five fields of operation, 2024 vs 2025.....	55
Figure 54: Chronic disease and cancer patient supported, disaggregated by gender across UNRWA's five fields of operation in 2025.....	56
Figure 55: Total admissions and bed occupancy trends in Qalqilya Hospital, from 2020 to 2025.....	57
Figure 56: Outpatient growth by specialty across UNRWA's five fields of operation, from 2020 to 2025.....	58
Figure 57: Total emergency room consultations across UNRWA's five fields of operation, from 2020 to 2025.....	58

Table of Figures and Tables

Figure 58: Average length of hospital stay (days), across UNRWA's five fields of operation, from 2020 to 2025.....	58
Figure 59: Number of newly registered patients with noncommunicable diseases in the Gaza Strip in 2025.....	60
Figure 60: Proportion (%) of patients with noncommunicable diseases with controlled conditions in the Gaza Strip in 2025....	60
Figure 61: Prevalence (%) of risk factors among patients with noncommunicable diseases in the Gaza Strip in 2025.....	71
Figure 62: Prevalence (%) of late complications among patients with diabetes mellitus and hypertension in the Gaza Strip in 2025.....	71
Figure 63: Number of women newly registered in preconception care service in the Gaza Strip in 2025.....	72
Figure 64: Number of registered pregnant women and ante-natal care visits in the Gaza Strip in 2025.....	73
Figure 65: Number of family planning consultations provided in the Gaza Strip in 2025.....	73
Figure 66: Number of post-natal consultations in the Gaza Strip in 2025.....	76
Figure 67: Number of vaccination sessions in the Gaza Strip in 2025.....	76
Figure 68: Number of persons who had MHPSS screening and number of persons found exposed to GBV in the Gaza Strip in 2025.....	76
Figure 69: The central and northern occupied West Bank.....	76
Figure 70: Percentage (%) of patients registered in the three closed clinics who received healthcare from UNRWA health installations between January- June 2025.....	77
Figure 71: A map of the occupied West Bank showing the locations of the three closed health centres and the established emergency clinics in June 2025.....	77
Figure 72: Geographic coverage and health services of UNRWA emergency clinics provided to patients of the closed Jenin Health Centre.....	77
Figure 73: Geographic coverage and health services of UNRWA emergency clinics serving patients of closed Tulkarm Health Centre.....	77
Figure 74: Geographic coverage and health services of UNRWA emergency clinics delivered to patients of closed Nur Shams Health Centre.....	77
Figure 75: Number of patients from the active cohort of the three closed HCs who received healthcare from the pre-existing UNRWA HCs across the occupied West Bank in 2025.....	77
Figure 76: Percentage (%) of NCD patients registered in the three closed health centres who received NCD care from UNRWA installations in the occupied West Bank in June 2025.....	78
Figure 77: Number of patients who received healthcare through the emergency clinics, disaggregated by the type of service provided	78
Table 1: Distribution of infant and child (<5 years) mortality by age and place of death in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.....	31
Table 2: Age- and sex-specific screening and detection of diabetes mellitus (DM) and hypertension (HTN) in 2025.....	40
Table 3: Prevalence of MHPSS positive cases (GHQ-12) and service coverage among identified cases by sex and disability status in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.....	47
Table 4: Identified mental health gap problems in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.....	47
Table 5: Annual trend of reported cases across UNRWA's five fields of operation, from 2021 to 2025.....	54
Table 6: Distribution of ward admissions by specialty and total number of beds, across UNRWA's five fields of operation in 2025.....	57
Table 7: Infectious disease cases (cumulative numbers) in the Gaza Strip in 2025.....	74

acronyms and abbreviations

AFP	Acute Flaccid Paralysis	KPI	Key Performance Indicator
AJS	Acute Jaundice Syndrome	LFO	Lebanon Field Office
ANC	Antenatal Care	MCH	Maternal And Child Health
ARI	Acute Respiratory Infections	MHF	Medical Hardship Fund
AWD	Acute Watery Diarrhoea	MI	Myocardial Infarction
CFHP	Common Health Financing Platform	MMS	Multiple Micronutrient Supplementation
CDC	Centres for Disease Control and Prevention	MMR	Maternal Mortality Rate
CMAM	Community Management of Acute Malnutrition	MOs	Medical Officers
COVID-19	Coronavirus Disease 2019	MHPSS	Mental Health and Psychosocial Support
CPD	Continuing Professional Development	MUAC	Mid-Upper Arm Circumference
DASH	Dietary Approaches to Stop Hypertension	NCDs	Non-Communicable Diseases
DM	Diabetes Mellitus	NGOs	Non-Governmental Organisations
DPC	Disease Prevention and Control	OCHA	United Nations Office for the Coordination of Humanitarian Affairs
EMR	Electronic Medical Record	PB	Programme Budget
EPI	Expanded Programmes on Immunisation	PCC	Preconception Care
EWARS	Early Warning, Alert and Response System	PHC	Primary Health Care
FHT	Family Health Team	PNC	Postnatal Care
FP	Family Planning	PLW	Pregnant and Lactating Women
GAM	Global Acute Malnutrition	PRCS	Palestine Red Crescent Society
GDM	Gestational Diabetes Mellitus	PRs	Palestine Refugees
GHQ-12	General Health Questionnaire-12	SFO	Syria Field Office
GFO	Gaza Field Office	SOPs	Standard Operating Procedures
GBV	Gender-Based Violence	SSNP	Social Safety Net Programme
GM	Growth Monitoring	TB	Tuberculosis
HCs	Health Centres	T1D	Type 1 Diabetes
HMAO	Hospitalization Monitoring and Assessment Officer	T2DM	Type 2 Diabetes Mellitus
HQ	Headquarters	TOHS	Tele-Oral Health Services
HSP	Hospitalization Support Programme	UN	United Nations
HTN	Hypertension	UNFCCC	United Nations Framework Convention on Climate Change
IEC	Information, Education, and Communication	UNFPA	United Nations Population Fund
IFMSA	International Federation of Medical Students' Associations	UNIMMAP	United Nations International Multiple Micronutrient Antenatal Preparation
IFA	Iron and Folic Acid	UNRWA	United Nations Relief and Works Agency for Palestine Refugees
IDPs	Internally Displaced Persons	WDF	World Diabetes Foundation
Imm	Immunisation	WHO	World Health Organization
IMT	Incident Management Team	WHO EMRO	World Health Organization Regional Office for The Eastern Mediterranean
IPC	Integrated Food Security Phase Classification	WFO	West Bank Field Office
IYCF	Infant and Young Child Feeding	WFP	World Food Programme
JFO	Jordan Field Office		
JNC	Jordanian Nursing Council		

Foreword by the UNRWA Commissioner-General

I am pleased to introduce the UNRWA Health Annual Report for 2025. This report details UNRWA's efforts to protect the health of Palestine Refugees by strengthening service delivery, ensuring the continuity of care, and responding to urgent healthcare needs in exceptionally difficult conditions.

In 2025, Palestine Refugees continued to endure prolonged conflict, displacement, and deepening socioeconomic hardship, with implications for both physical and mental health. In this context, UNRWA's health services proved vital in all fields of operation, reaching millions of children, women and men with primary healthcare and emergency assistance. In 2025, UNRWA delivered approximately nine million medical consultations across the region.

Operational conditions in the Occupied Palestinian Territory remained highly challenging in 2025. Access and movement restrictions, and safety-related constraints repeatedly disrupted services. Even so, UNRWA health teams maintained the provision of essential care—often under extremely challenging circumstances and at great personal risk—to those most in need.

I pay tribute to UNRWA health staff for their professionalism and dedication. Their life saving work—delivered day after day in conditions of uncertainty and insecurity—exemplifies the

best of humanity and the Agency's steadfast commitment to delivering on its mandate to protect and assist Palestine Refugees.

UNRWA's health work is enabled through close collaboration with the World Health Organization and other partners supporting critical interventions such as midwifery services, immunization, and the supply of essential medicines.

The sustainability of our efforts also depends on the continued and steadfast support of Member States. Adequate and predictable funding remains indispensable to sustaining and strengthening UNRWA's health services. I wish to express my profound gratitude to all donors who have supported the Agency's health programmes over the past year, and to appeal to Member States to consider providing additional financial contributions in the coming year to help bridge the gap between the critical needs of the Palestine Refugee population and the Agency's financial capacity to meet those needs.

With the commitment and support of its staff, partners and donors, UNRWA will continue to adapt its health programme to the meet evolving needs of Palestine Refugees, while striving to ensure access to high-quality of services for those in need.



Christian Saunders
UNRWA Commissioner-General
ad interim

message from the who regional director for the eastern mediterranean

I am pleased to extend my congratulations on the release of UNRWA's Department of Health Annual Report 2025. This report reflects the Agency's continued commitment to delivering essential health services to Palestine Refugees under exceptionally challenging and evolving conditions.

Over the past year, the operating environment has remained highly constrained, particularly in the Gaza Strip, where the health system continues to function under severe limitations despite a fragile ceasefire. In this context, partnership with World Health Organization – Eastern Mediterranean Regional Office (WHO-EMRO) has been more important than ever in sustaining essential health services and protecting public health.

The continued restriction on the entry of medical supplies into the Gaza Strip has significantly impacted the availability of essential medicines and health commodities. This has stressed the importance of close coordination among health partners to optimise limited resources and maintain priority health interventions.

In 2025, WHO and UNRWA worked closely, alongside partners including UNICEF, to implement routine immunisation catch-up campaigns, reaching approximately 600,000 children across the Gaza Strip. UNRWA played a key role in this effort, contributing 1,700 health personnel and supporting service delivery through 555 mobile and fixed teams at nine health centres and 50 medical points. These efforts have been critical in mitigating the risk of vaccine-preventable disease outbreaks in a highly vulnerable population.

WHO has also continued to support the broader health response through strengthened disease surveillance systems, enabling timely detection and response to public health threats. In a context of disrupted supply chains and constrained service delivery, sustained collaboration has been essential to maintaining core public health functions. Looking ahead, WHO EMRO reaffirms its commitment to working closely with UNRWA to sustain essential health services and strengthen health system resilience. As the situation remains fragile, our shared focus on advancing health equity and ensuring access to quality healthcare for all Palestine Refugees remains firm.



Dr. Hanan Balkhy
World Health Organization,
Regional Director for the
Eastern Mediterranean

vision and purpose of the 2025 health annual report

This annual report presents a comprehensive overview of the work undertaken by UNRWA's Department of Health, highlighting the Agency's continued presence, mandate, and indispensable role in delivering essential services to Palestine Refugees. It showcases UNRWA's achievements in sustaining quality and effective healthcare services despite increasingly complex and protracted emergencies across its five fields of operation, including the Gaza Strip and the occupied West Bank. In recent years, UNRWA and its staff have operated under extraordinary pressure marked by conflict, displacement, and resource constraints. Despite these challenges, the Agency has remained committed to ensuring equitable access to care and safeguarding public health.

This report aims to document operational performance and programmatic outcomes and reaffirm the critical importance of UNRWA's mandate and operational role. In doing so, it highlights the Agency's responsiveness during crises, its continuous efforts to strengthen emergency preparedness and response capacities, and its strategic investments in health system digitalisation, telemedicine, and service improvement. Simultaneously, the report provides forward-looking insights into priorities, challenges, and opportunities, outlining a vision for enhancing resilience, efficiency, and sustainability in health services delivery.

Ultimately, this report serves as both a record of accountability and an instrument of advocacy, demonstrating that, amid ongoing adversities, UNRWA remains essential, adaptive, and committed its mission to protect and promote the health and dignity of Palestine Refugees, now and into the future.

Holistic care themes

UNRWA's Department of Health provides holistic healthcare to registered Palestine Refugees across its five fields of operation: the Gaza Strip, the occupied West Bank, including East Jerusalem, Jordan, Syria, and Lebanon.

Primary healthcare is the main pillar of the health programme, ensuring adequate coverage and comprehensive healthcare. Additionally, support is provided to patients requiring secondary and tertiary healthcare, including coordination of referrals and financial coverage of outpatient consultations and inpatient care (i.e., hospital admission for more specialised treatment or surgery).

Vaccination, newborn screening, and detection of growth problems, along with referrals, are offered to children below five. The essential programme of immunisation follows the host country programme, and UNRWA serves as a partner in essential immunisation and in reporting on vaccine-preventable outbreaks.

Services provided to women include pre-conception, antenatal and post-natal care, as well as family planning services, multiple micronutrient supplementation (MMS), support against gender-based violence, and other outpatient services. Nutrition services are provided to pregnant women, infants, and children, especially in the Gaza Strip and Lebanon, where malnutrition is more common than in other fields due to conflict, instability, and displacement.

Nursing and midwifery services play a pivotal role in linking maternal and child health interventions and in delivering planned training programmes to frontline workers. This work is carried out through workshops and discussions held at field offices and headquarters, thereby opening avenues for contact across the fields.

In the context of non-communicable diseases (NCDs), UNRWA provides a robust programme for preventing and managing diabetes and hypertension. Prevention interventions include screening for NCDs and health promotion, while management includes counselling, medications, and insulin supply, following technical instructions designed by UNRWA which are aligned with the latest WHO protocols.

Mental health is a key domain of UNRWA's services. The Mental Health and Psychosocial Support (MHPSS) programme systematically screens vulnerable population groups, including pregnant women, children, and NCDs patients with uncontrolled disease, for mental health and psychosocial conditions. Individuals identified as being in need receive appropriate, tiered psychosocial and mental health interventions, including support for depression, with different levels of assistance provided for those with higher depression scores.

Outpatient services include dental care, physiotherapy, laboratory services, and radiology. Oral screening is performed for first-year school students, along with medical and physical examinations.



A Palestine Refugee baby having MUAC measurement taken by an UNRWA nurse at the Japanese Health Centre in the Khan Younis area, the Gaza Strip. ©2025 UNRWA photo.

health profile

For over 75 years, UNRWA has been a key provider of healthcare for Palestine Refugees. Since 1950, when the refugee population numbered approximately 750,000, the number of Palestine Refugees registered at UNRWA was around 6.8 millions in all fields, including the Gaza Strip, by the end of 2025. Today, in the five fields, approximately 3.25 million people have either sought healthcare or are potentially in need of health services provided through UNRWA's health system. Nearly one-third of the population across the five fields lives in 58 official refugee camps, often in overcrowded settings with fragile infrastructure and limited resources.

Palestine Refugees faced unprecedented challenges in 2025. The war in the Gaza Strip, escalating settler violence in the occupied West Bank, instability in Syria, and economic collapse in Lebanon have severely disrupted health service delivery. In the Gaza Strip, widespread destruction of health facilities, repeated displacement, and restrictions on the entry of medicines and supplies have left many without essential care. Health workers themselves face extreme stress and trauma while continuing to serve their communities. In Lebanon and the occupied West Bank, displacement and movement restrictions further limit access to health services.

Despite these conditions, UNRWA remains committed to delivering primary healthcare with minimal interruptions. Maternal and child health remain core priorities. With 23 per cent of registered Palestine Refugees aged 18 years or younger, the need for targeted interventions is urgent. In 2025, across the Agency's four fields of operation, excluding the Gaza Strip, UNRWA supported approximately 65,442 pregnant women through maternal health services. Preconception care and family planning services were provided to 29,436 women registered in the services in 2025. In addition, UNRWA teams delivered care to 200,033 infants and children under five, across the four fields.

The incidence of non-communicable diseases continued to rise. In 2025, UNRWA treated over 87,135 patients with hypertension, 31,399 patients with diabetes mellitus, and 98,088 patients with both conditions across the four fields, excluding the Gaza Strip.

Communicable diseases have re-emerged due to overcrowding in camps, poor sanitation in some fields and areas, and displacement. In 2024, polio and cholera were recorded in the Gaza Strip and Lebanon, respectively. Immunisation campaigns were implemented urgently. No vaccine-preventable outbreaks were declared last year.

¹ Four fields refer to the occupied West Bank, including East Jerusalem, Jordan, Lebanon and Syria, excluding the Gaza Strip.

Malnutrition, especially among children under the age of two in the Gaza Strip, has reached alarming levels. Sustained investment in surveillance systems, immunisation coverage, and multisectoral nutrition responses remains critical to prevent further public health deteriorations.

The mental health needs have increased sharply across all fields. Despite severe operational constraints, all UNRWA health centres continue to integrate mental health and psychosocial support into primary healthcare services. This integrated approach continues to be essential to ensure comprehensive, people-centred care, particularly in protracted emergency settings.

Today, the health and dignity of millions of Palestine Refugees depend on sustained international support. Without increased funding and protection of humanitarian access, the already fragile health system risks further deterioration. UNRWA continues to provide care, protect public health, and uphold the right to health for one of the world's longest-standing refugee populations.

The section on the Gaza Strip is presented separately and appears at the end of this report due to the distinct operational context and reporting methodology applied during the reporting period. Since October 2023, the unprecedented humanitarian situation, large-scale displacement, damage to infrastructure, disruptions in telecommunications and internet connectivity, and constraints on data collection and verification have affected the availability and comparability of information from the Gaza Strip.

To ensure methodological transparency and maintain the integrity of the analysis, data and findings from the Gaza Strip are therefore presented separately from those of the other fields of operation. The placement of this section should not be interpreted as reflecting its importance or priority. Rather, it acknowledges the unique circumstances under which services were delivered and information was collected, and allows the report to present Gaza-specific findings within their appropriate operational and methodological context.

Note: The source of the health data from Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in this report is the e-Health. The source of the health data from the Gaza Strip is the reports shared by the field office colleagues inside the Gaza Strip and in Cairo-Egypt. Data collecting and reporting processes are explained in the Gaza Strip section.

plain summary

Challenges

UNRWA faced a complex set of operational and public health challenges, driven primarily by conflict, access constraints, and resource limitations. In the Gaza Strip, the situation was particularly severe, with continuous outbreaks of respiratory diseases, diarrheal diseases, and Hepatitis A reported throughout the year. The humanitarian crisis escalated further in August 2025 when famine (IPC Phase 5) condition was confirmed by IPC in Gaza City, necessitating urgent, life-saving nutrition interventions. Community-based Management of Acute Malnutrition (CMAM) programmes were scaled up despite damaged infrastructure and significant access constraints, highlighting both the scale of need and the operational difficulties. Infrastructure destruction and widespread displacement severely limited service capacity, while service suspensions led to a sharp decline in hospitalisations and created critical gaps in access to care.

In the occupied West Bank, service delivery was directly affected by the forced shutdown of three main health centres in Tulkarm, Jenin and Nur Shams. Across all fields, physician workload exceeded WHO standards. On average, doctors handled 71.11 consultations per day, reflecting sustained pressure on frontline services and raising concerns about the quality of care and staff wellbeing amid ongoing staffing constraints. Supply chain disruptions posed another major challenge. Approximately 30 per cent of health centre pharmacies experienced shortages of essential medicines and supplies at various times due to military restrictions and instability. In the Gaza Strip, pharmaceutical shortages were particularly acute, with up to 58 per cent of medicines out of stock. These shortages, combined with severe conflict, restricted access, and logistical barriers, significantly constrained service continuity.

At the same time, population health needs intensified. The burden of care remained high, with 25 per cent of pregnant women in the fields excluding the Gaza Strip classified as high-risk, requiring more intensive and higher-quality primary healthcare. The demand for chronic disease care remained high, while funding constraints continued to limit system capacity. Mental health needs surged under the strain of conflict and displacement, with 4 per cent of women screened for mental health illnesses identified as being exposed to gender-based violence in the four fields, excluding the Gaza Strip. Although services expanded, they did so amid severe access and resource gaps. Overall, the combination of rising health needs, limited resources, and a volatile operating

² Integrated Food Security Phase Classification (IPC), "Famine confirmed in Gaza Governorate", 15 August 2025.
<https://www.ipcinfo.org/ipcinfo-website/countries-in-focus-archive/issue-134/en/>

environment created substantial and persistent challenges in 2025, while UNRWA continued to adapt and sustain the delivery of comprehensive health services despite these difficulties.

Achievements

In the Gaza Strip, UNRWA sustained primary healthcare through six–eight health centres³ and 53 medical points despite system collapse. A total of 1.7 million internally displaced persons were reached, with services delivered through 112 health teams. High service volumes were maintained, including approximately 18,400 monthly NCD consultations under crisis conditions. Maternal and child health services continued, with more than 154,000 antenatal care visits and 127,000 vaccinations delivered. The nutrition response was also scaled, with 163,704 MUAC screenings conducted among children aged 6–59 months, and 12,181 cases of global acute malnutrition referred to therapeutic feeding programmes.

Despite the ongoing crisis, the health teams in the Gaza Strip continued to report reliable data throughout the period, ensuring continuity of monitoring and accountability. In parallel, the health teams in the occupied West Bank established emergency clinics to maintain health service provision for displaced populations. In the occupied West Bank, more than 90 per cent of patients with diabetes in Jenin, Tulkarm and Nur Shams were fully covered, and patients in the north of the occupied West Bank continued to receive insulin, chronic medications, and appropriate care.

Health communication efforts through multiple channels, including social media and information, education, and communication (IEC) materials, strengthened UNRWA's outreach and visibility during crises, as well as its partnerships and emergency response by expanding access to public health information and services. At the same time, data analysis and research conducted under UNRWA's health programme continued to reinforce the Agency's global health role.

The life-cycle primary health care (PHC) model ensured integrated care for refugee families, with strengthened preconception care improving early detection of abnormalities and maternal outcomes. Maternal services continued to integrate quality, mental health, and protection standards, while integrated programmes enhanced resilience and equity across all fields. In 2025, more than 43,000 pregnant women were registered across the four fields of operation, excluding the Gaza Strip. The percentage of women newly registered in PCC services increased by 3.72 per cent in the four fields excluding the Gaza Strip.

Across the four fields, excluding the Gaza Strip, the maternal mortality ratio (MMR) in 2025 reached 18 per 100,000, compared to 17 per 100,000 in 2024. Between 84–92.4 per cent of patients displaced in the northern occupied West Bank received healthcare through UNRWA emergency clinics. Infectious disease surveillance was maintained through manual data collection and Early Warning, Alert, and Response System (EWARS), and health centres in East Jerusalem remained vital for ensuring accessible, affordable, and holistic healthcare.

Overall, 95.9 per cent of strategic indicators were fully achieved in the four fields, excluding the Gaza Strip. Maternal mortality reached 18 per 100,000 live births, while neonatal mortality remained extremely low at 0.3 per 1,000 live births. In addition, 72.78 per cent of pregnant women registered at UNRWA health centres during the first trimester, and 86.11 per cent received at least four antenatal care visits. Postnatal care coverage remained high, with 91.55 per cent of women attending postnatal consultations within six weeks after delivery.

Child health indicators remained strong, with 97.91 per cent of children in the four fields (excluding the Gaza Strip) receiving full booster vaccinations at 18 months of age, 97.74 per cent receiving two doses of Vitamin A, 98.13 per cent of children aged 36–60 months of age screened for disabilities, and 99.65 per cent of new school entrants receiving health examinations. Screening for noncommunicable diseases reached 52.3 per cent of individuals aged 40+, while 34.16 per cent of diabetes patients were under control, with higher control among women (36.8 per cent) and persons with disabilities (38.31 per cent).

Continuity of care for chronic conditions remained high, with 82.77 per cent of NCD patients receiving consultations and treatment regularly despite ongoing operational pressures. Mental health and psychosocial support (MHPSS) services expanded detection and response efforts, with 11.65 per cent of individuals screened through GHQ-12 showing high scores for mental health problems, rising to 19.21 per cent among persons with disabilities. Across the four fields excluding Gaza, 98.65 per cent of identified cases received support.

The immunisation system remained stable across UNRWA's five fields of operation, with zero reported outbreaks of vaccine-preventable diseases, and antibiotic prescription rates were maintained at 20.34 per cent within controlled levels. Nutrition services continued to integrate NCD and maternal and child healthcare across all stages, with

³Through 2025 the number of operating health centres varied due to security issues.

163,707 child screenings conducted and more than 12,000 children treated for malnutrition. Multiple micronutrient supplementation (MMS) was scaled systemwide, improving maternal anaemia outcomes, reaching approximately 20,000 pregnant women per year in Jordan, while nutritionists in Lebanon delivered approximately 712 consultations and 40 sessions per month.

Additionally, 98.9 per cent of children under five were screened for disability in the four fields, excluding the Gaza Strip. The percentage of diabetic patients with controlled blood sugar levels improved over the past four years, reaching approximately 34 per cent. The integrated PHC model sustained NCD care for 216,622 patients across the Agency's four fields excluding the Gaza Strip, while early detection efforts were strengthened through the screening of 160,000 individuals, contributing to improved diabetes control rates.

Integration of Mental Health and Psychosocial Support (MHPSS) into other services, such as maternal health and non-communicable diseases, expanded significantly, with 139,000 individuals screened. Following the screening, 12 per cent of patients were identified with MHPSS needs, and 99 per cent of identified cases received support. Strong system resilience preserved health gains despite extreme humanitarian pressures, supported by relatively stable medicine supply in most fields through partnerships. UNRWA also ensured access to secondary and tertiary care, prioritising vulnerable Palestine Refugees, with 84,189 patients supported through hospitalisation services. Hospitalisation services contributed to improving access to high-cost care and cancer outcomes and treatment continuity.

Innovations

UNRWA continued to advance innovations to sustain service delivery. The e-Health system supported the digital transformation of primary healthcare through real-time, high-quality data. Centralised data systems and dashboards enabled faster, evidence-based decision-making, while digital systems ensured continuity of care and data reporting during crises.

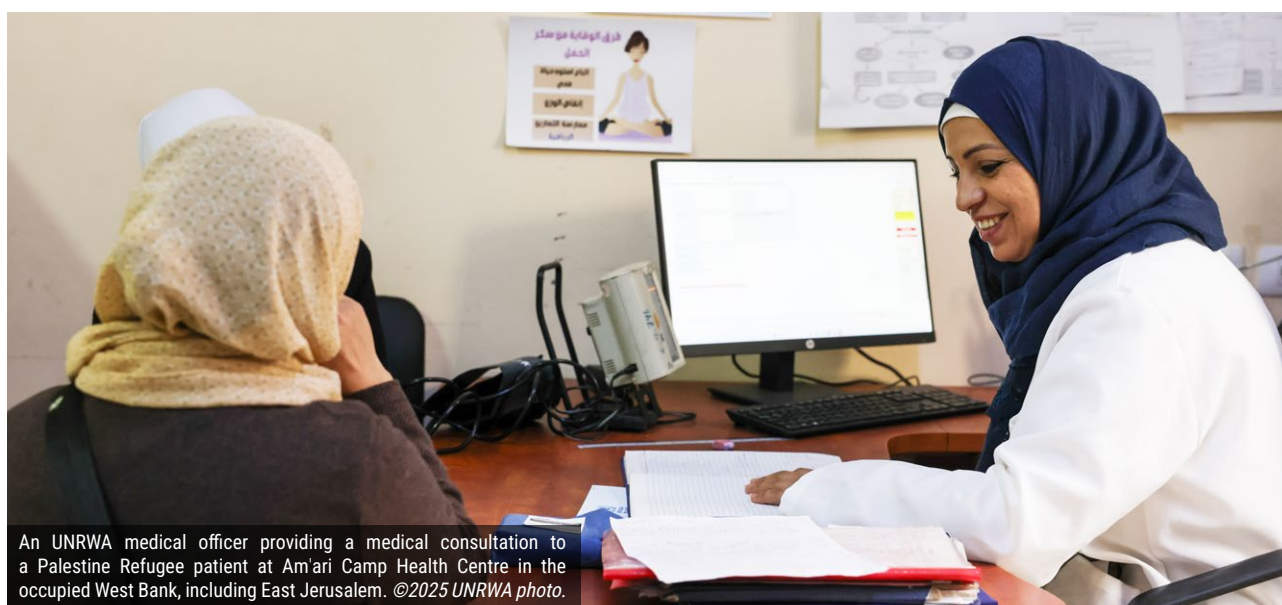
Adaptive service delivery models, including mobile teams, telemedicine, and medical points, ensured continuity of care, particularly in the Gaza Strip. Pharmaceutical supply optimisation and redistribution mechanisms reduced losses and supported planning for 2026. Hospitalisation digital systems, including hospital management systems (HMS) and dashboards, improved monitoring, efficiency, and decision-making.

Looking to the future

UNRWA remains committed to sustaining quality health services across all fields, including during emergencies. The health department will continue to evaluate and draw lessons from the emergency response in the Gaza Strip and the occupied West Bank.

Future priorities for hospitalisation include better integration with primary health care, expanded preventive approaches, and strengthened financial sustainability. In the Gaza Strip, the focus will be on strengthening telemedicine, restoring health systems, and ensuring sustainable supply chains.

The introduction of a new electronic medical record system by 2027 will further enhance data quality and reporting accuracy, improve staff efficiency, and support evidence-based planning through real-world health data.



An UNRWA medical officer providing a medical consultation to a Palestine Refugee patient at Am'ari Camp Health Centre in the occupied West Bank, including East Jerusalem. ©2025 UNRWA photo.

health workforce and infrastructure

	HQ		Jordan		Lebanon		Syria		Gaza Strip		Occupied West Bank		Total	
	M	F	M	F	M	F	M	F	M	F	M	F	M	F
Doctors	6	2	78	14	46	25	44	26	111	107	49	18	334	192
Specialists	0	0	4	2	5	3	1	3	3	1	1	1	14	10
pharmacists	1	2	6	8	10	17	16	24	18	45	10	50	61	146
Dental Surgeons	0	0	17	12	9	10	13	12	15	17	11	4	65	55
Nurses	0	1	14	251	37	78	10	83	71	214	49	152	181	779
Paramedical	0	0	61	57	16	30	26	29	45	90	32	92	180	298
Admin/Support Staff	1	7	34	26	25	35	21	32	49	48	27	24	156	167
Cleaners	0	0	57	29	12	29	29	20	18	46	20	32	137	161
Sub-total	8	12	271	399	160	227	160	229	330	568	199	373	1,128	1,808

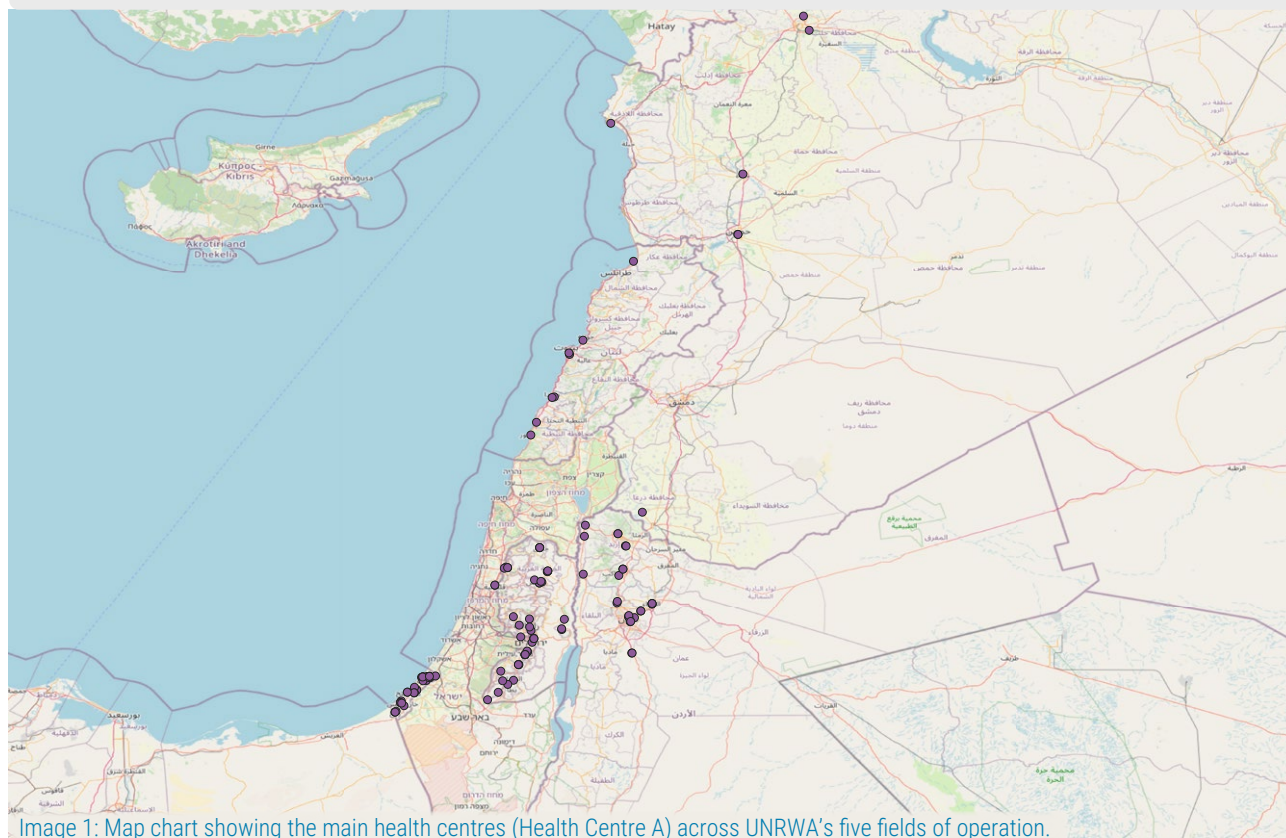
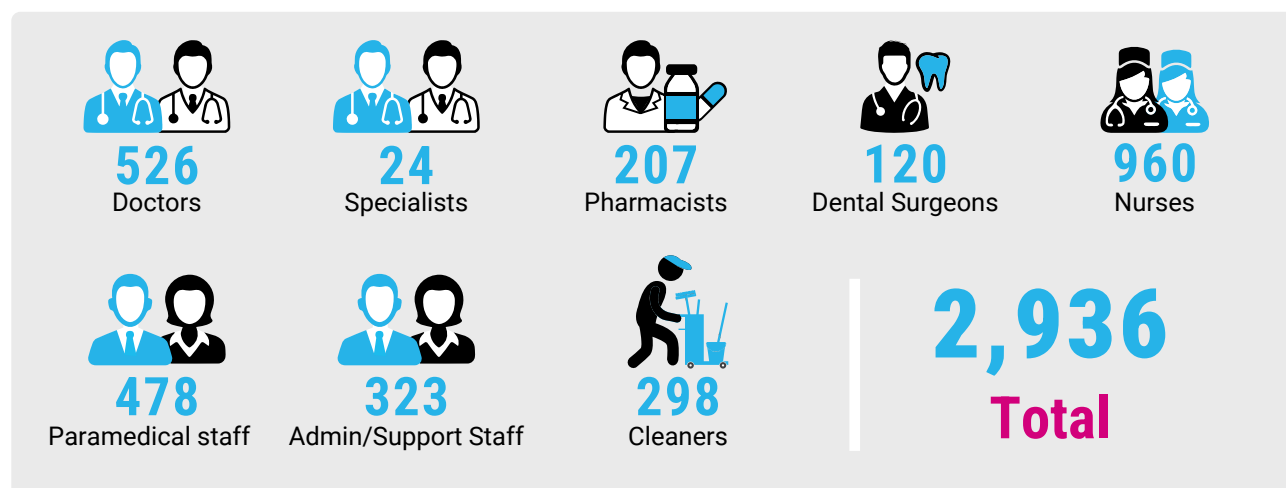
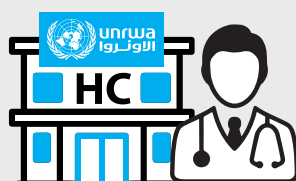


Image 1: Map chart showing the main health centres (Health Centre A) across UNRWA's five fields of operation.

(Source: The map was obtained from the department of digital impact, technology and innovation).



123

Total number of health centres (HCs)

Gaza Strip



8 HCs

Jordan



25 HCs

Lebanon



26 HCs

Syria



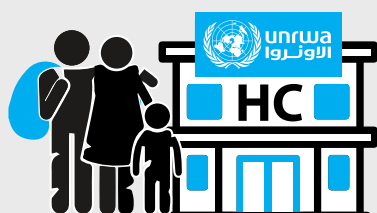
26 HCs

Occupied
West Bank



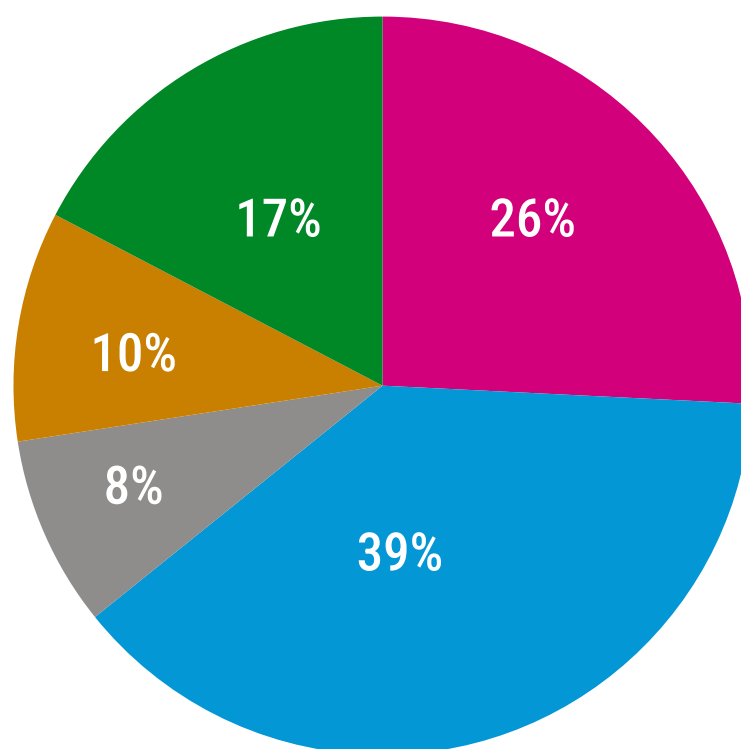
40 HCs

UNRWA-registered Palestine Refugees



3,423,392
Females

3,379,633
Males



■ Gaza Strip ■ Jordan ■ Lebanon ■ Syria ■ Occupied West Bank

Figure 1: Distribution (%) of registered Palestine Refugees by field in 2025, including the Gaza Strip.



registered population

Gaza Strip



↑ 878,948

↓ 874,987

Jordan



↑ 1,304,230

↓ 1,310,370

Lebanon



↑ 283,319

↓ 285,356

Syria



↑ 334,017

↓ 351,965

Occupied
West Bank



↑ 579,044

↓ 600,642

(For age distribution please see Annex 1).



A Palestine Refugee patient registering for an appointment with the receptionist at the Japanese Health Centre in the Khan Younis area, the Gaza Strip. ©2025 UNRWA photo.

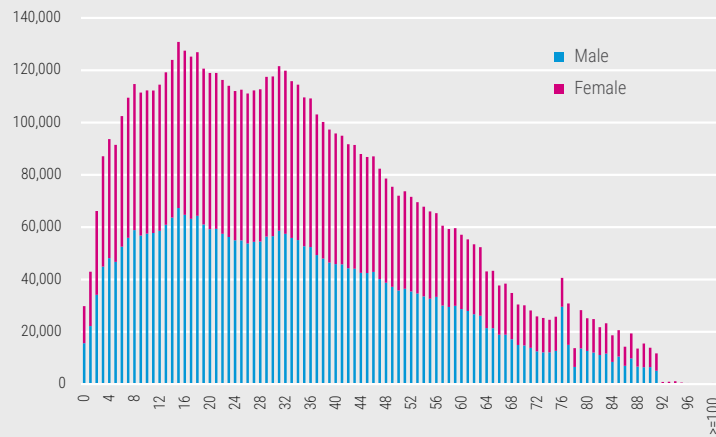


Figure 2: Age and sex distribution of registered Palestine Refugees in the five fields in 2025.

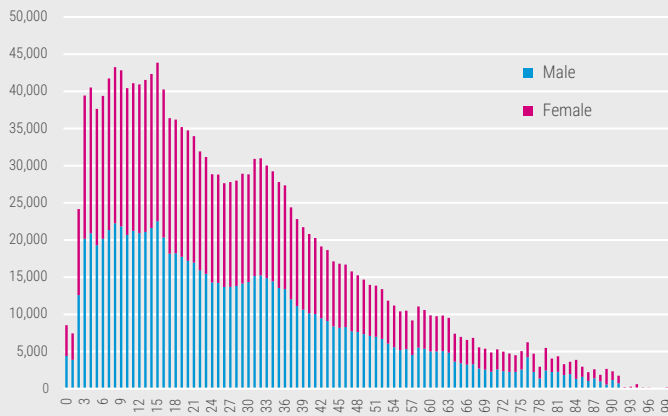


Figure 3: Age and sex distribution of registered Palestine Refugees in the Gaza Strip in 2025.

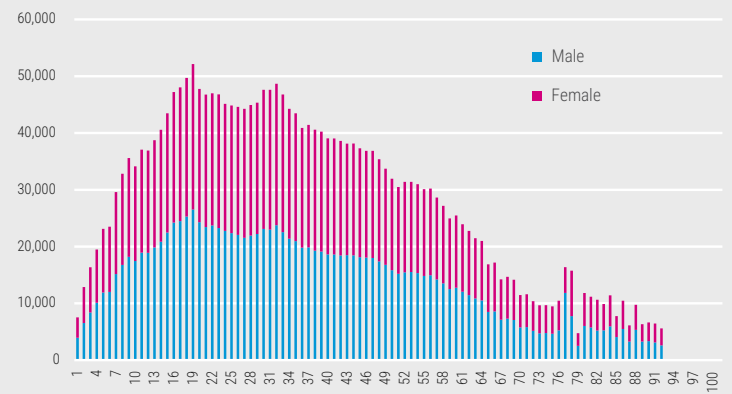


Figure 4: Age and sex distribution of registered Palestine Refugees in Jordan in 2025.

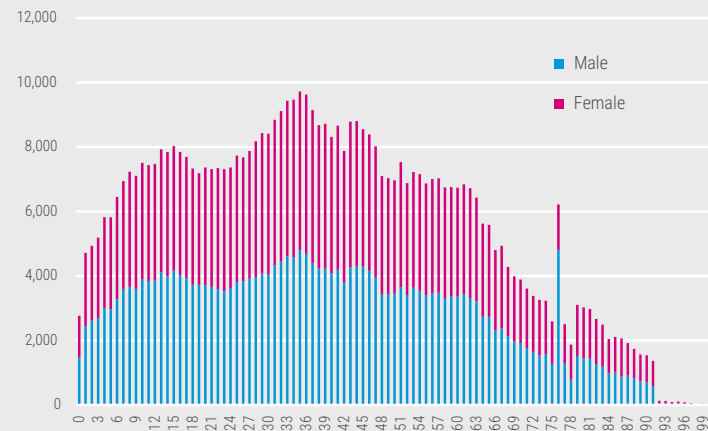


Figure 5: Age and sex distribution of registered Palestine Refugees in Lebanon in 2025.

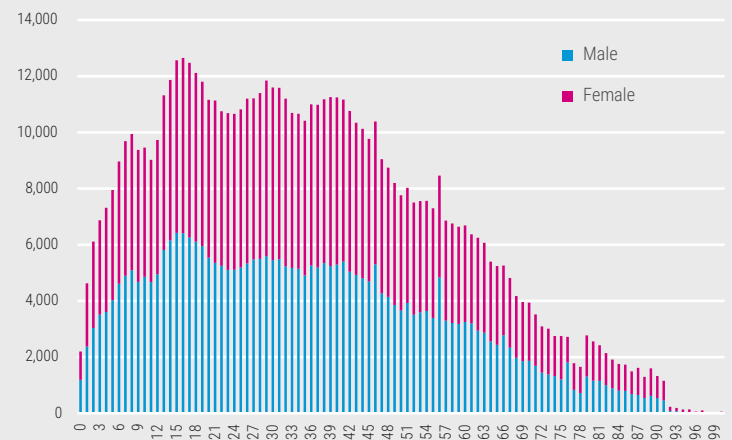


Figure 6: Age and sex distribution of registered Palestine Refugees in Syria in 2025.

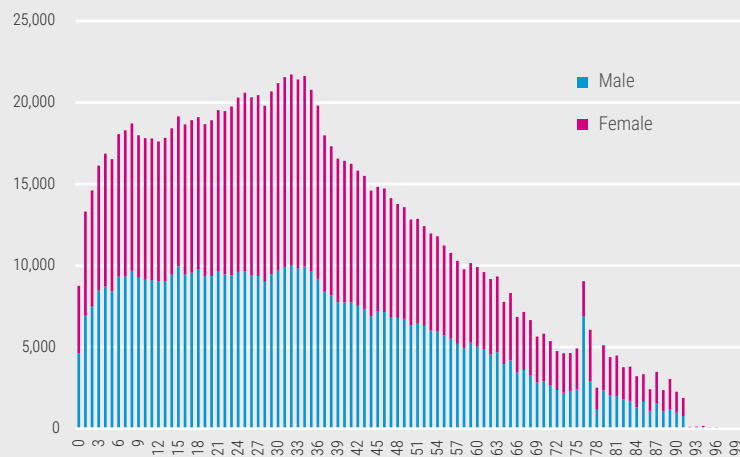


Figure 7: Age and sex distribution of registered refugees in the occupied West Bank, including East Jerusalem, in 2025.

serviced population

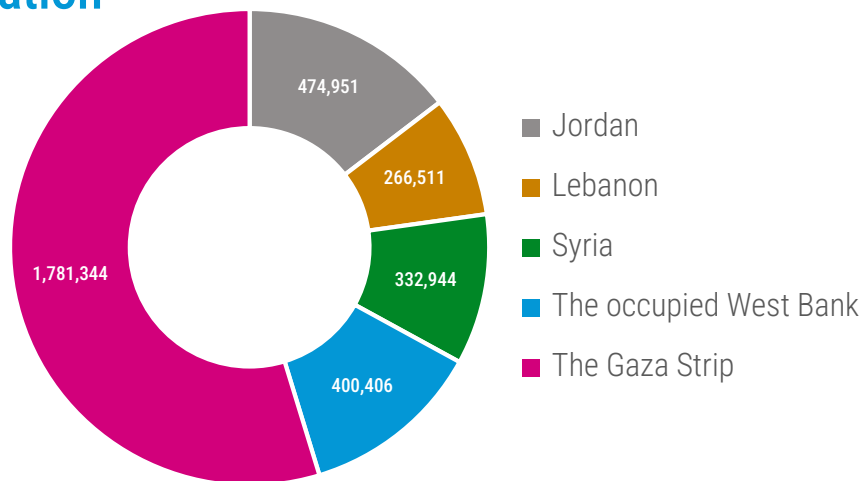


Figure 8: Number of individuals serviced by UNRWA health centres in its five fields, in 2025.

(Total = 3,256,156). Serviced population in the Gaza Strip is an estimate).

(For family files and potential population, please see Annex 2.)

workload

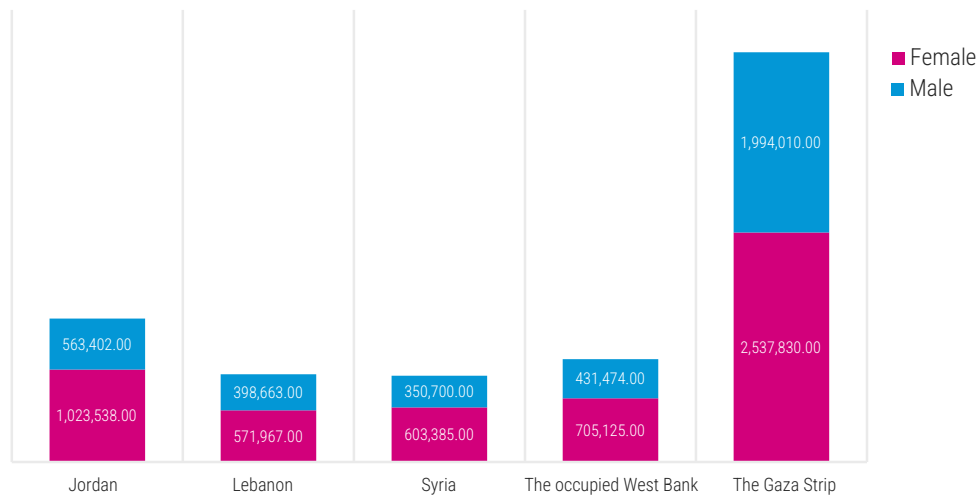


Figure 9: Number of medical consultations by sex in UNRWA's five fields, in 2025.

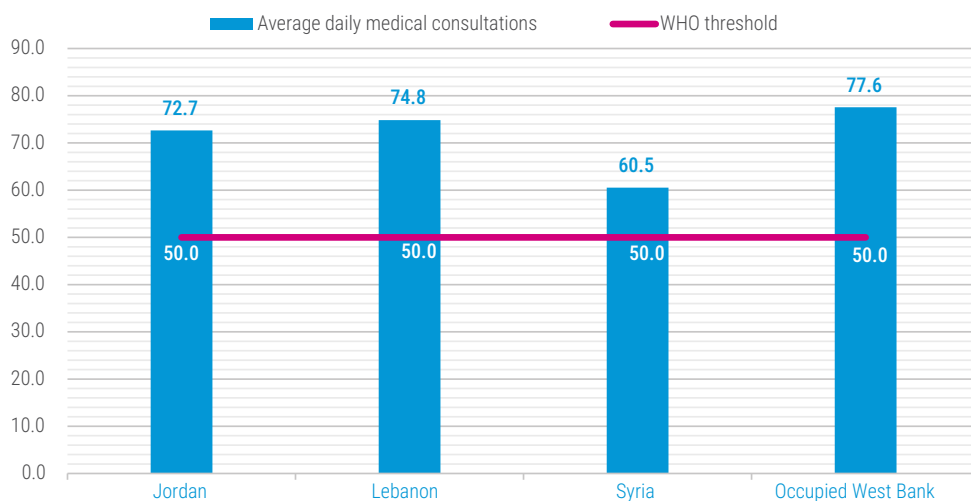


Figure 10: Average daily medical consultations per medical officer in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.

(For disaggregation per modalities of consultations and contact time, please see Annex 3.)

Figure 10 shows that across the four fields, excluding the Gaza Strip, a medical doctor provides medical consultations at a significantly higher rate than the standard set by the World Health Organization (WHO). It is highly likely that the workload of medical officers is higher in the Gaza Strip. However, it could not be calculated due to the absence of e-health systems there during the crisis. The financial situation of UNRWA and the region overall has hindered the recruitment of more doctors to meet this challenge. More daily pressure on medical doctors not only negatively affects the quality of the consultation and overall services, but also affects the performance, mental health, and well-being of staff.

contexts

The operating environment of the UNRWA Health Programme in 2025 was shaped by rapidly evolving political, security, and socio-economic dynamics across its fields of operation. In the Gaza Strip, the protracted war and its aftermath have fundamentally altered the public health landscape. Widespread infrastructure damage, repeated massive population displacement, and severe restrictions on the entry of essential supplies have contributed to a sustained deterioration in health determinants. The collapse of basic services including water, sanitation, and electricity, has increased the risk of communicable disease outbreaks, while disruptions in continuity of care have exacerbated non-communicable diseases and maternal and child health risks. At the same time, the psychological toll of prolonged exposure to violence and displacement has led to an unprecedented burden of mental health conditions, particularly among children and vulnerable populations. These factors have required continuous adaptation of service delivery modalities, including flexibility, dynamic planning, decentralisation of care and expanded outreach mechanisms.

In the northern areas of the occupied West Bank, escalating security operations, movement restrictions, and displacement from refugee camps have increasingly constrained access to health services. Frequent incursions and closures have disrupted routine service provision, hindered patient mobility, and limited the ability of health staff to operate safely and effectively. These dynamics have contributed to delayed care-seeking, interruptions in treatment for chronic conditions, and growing protection concerns, all of which have direct implications for population health outcomes.

In Lebanon, the ongoing economic crisis has further deepened vulnerabilities among Palestine Refugees. At the same time, socio-economic hardship has intensified food insecurity and negatively affected health-seeking behaviours, particularly among the most vulnerable groups, including women, children, and older persons.

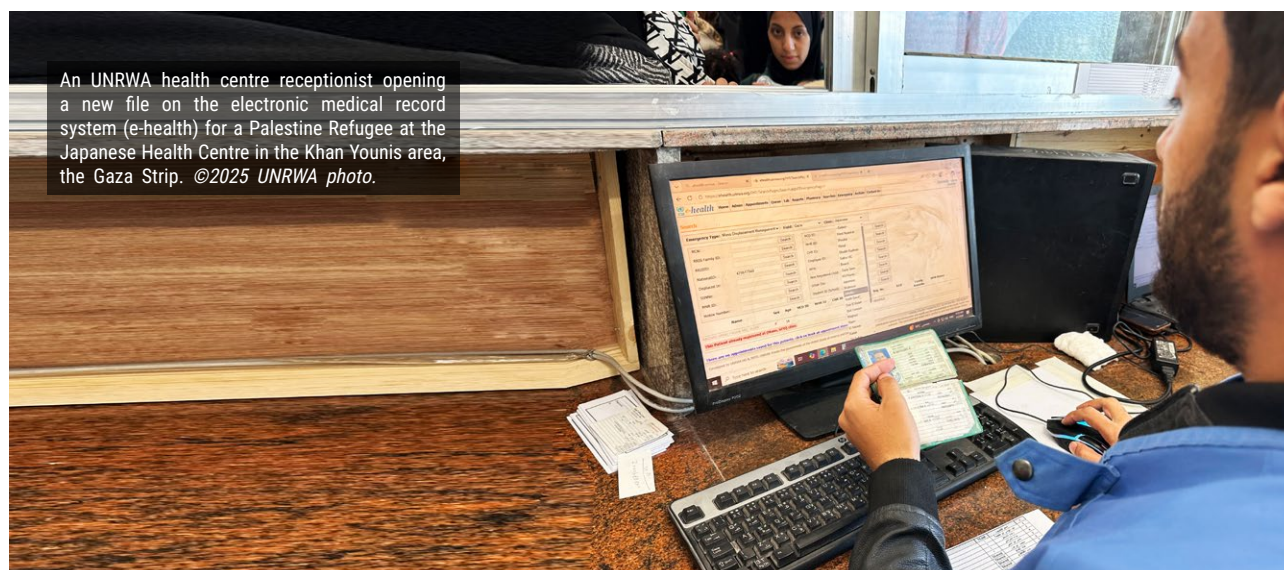
Across all the five fields, these crises have underscored the imperative of sustaining a resilient, adaptable, and equity-focused primary healthcare approach. In line with the overarching principle of “Stay and Deliver,” the UNRWA Health Programme has continued to operate under severe and protracted constraints, ensuring the continuity of essential services, prioritising the most vulnerable populations, and upholding standards of quality and integrity of care. This commitment to remain present and responsive in the most challenging environments defined the programme’s operational posture in 2025. These contextual realities are fundamental to interpreting programme performance and shaping strategic priorities for the period ahead.

data management, digitalisation for monitoring, support, and credibility: the e-health software

UNRWA’s adoption of the e-Health electronic system represents a critical advancement in the digital transformation of primary healthcare delivery and monitoring. By enabling real-time data entry at the health centre level, the system ensures that patient information, service utilisation, and key health indicators are captured accurately and consistently at the point of care. This immediate digitisation minimises transcription errors associated with paper-based systems, enhances data completeness, and supports standardised reporting across all fields of operation.

A central strength of the e-health system lies in its integration with a centralised data warehouse, where information from all health centres is pooled, validated, and stored securely. This architecture facilitates efficient data aggregation and longitudinal tracking of health trends, enabling evidence-based decision-making at both operational and strategic levels. The linkage of this warehouse to interactive dashboards further amplifies its utility. Health managers and UNRWA officers can access up-to-date, visualised data through user-friendly interfaces, allowing for rapid interpretation of trends, identification of service gaps, and timely programmatic adjustments. This significantly enhances responsiveness, accountability, and performance monitoring across the health programme.

Importantly, the system’s flexibility has proven essential in humanitarian contexts. During emergencies and crises, including situations of displacement or restricted access, UNRWA has dynamically extended access to the e-Health platform to emergency units and mobile clinics. This ensures



An UNRWA health centre receptionist opening a new file on the electronic medical record system (e-health) for a Palestine Refugee at the Japanese Health Centre in the Khan Younis area, the Gaza Strip. ©2025 UNRWA photo.

continuity of data reporting even in unstable environments, preserving the integrity of health information systems when it is most needed. Mobile teams can input and retrieve patient data in real time, supporting continuity of care, avoiding duplication of services, and enabling effective follow-up for vulnerable populations. An example is using e-health to track pregnant women and NCD patients in the northern occupied West Bank after during the escalations early in 2025. Please see pages 74-78.

Overall, the digitalisation of health data and the establishment of integrated dashboards have strengthened UNRWA's capacity to deliver timely, efficient, and equitable health services. These systems not only enhance routine service delivery but also ensure resilience and adaptability in the face of complex humanitarian emergencies.

health communication

UNRWA's health communication remains a critical pillar for engaging Palestine Refugees, staff, and partners, ensuring the timely dissemination of essential health information and supporting service delivery, public health awareness, and emergency response.

A multi-channel approach continues to maximise outreach and accessibility. Printed materials are made available at health centres, while digital platforms, including social media, multimedia content, and the UNRWA website, serve as key channels for sharing programme updates, health education messages, and advocacy content.

Communication efforts focused on the development and dissemination of a wide range of information, education, and communication (IEC) materials, covering maternal and child health, oral health, nutrition, gender-based violence,

and noncommunicable diseases. New advocacy and visibility materials were also developed, including a video highlighting over 75 years of UNRWA primary healthcare services and materials supporting the rollout of centralised telemedicine services.

Health promotion remained a key focus, with targeted campaigns and observances reinforcing public health messaging. A preventive health awareness campaign during the holy month of Ramadan promoted healthy fasting practices, management of chronic conditions, and maternal and child health. Global health days, including World Health Day, Breast Cancer Awareness Month, World Mental Health Day, Menstrual Hygiene Day, and Nurses and Midwives Day, were marked by coordinated multimedia campaigns that amplified key health priorities and highlighted patient and frontline worker experiences.

Internal communication continued to prioritise staff well-being, in line with SDG #3 on promoting good health and well-being. Monthly health tips are disseminated to UNRWA staff as part of workplace wellbeing efforts, offering evidence-based guidance on maintaining healthy habits, preventing illness, and promoting overall wellbeing. The monthly health tip initiative was sustained and complemented by tailored workplace awareness materials, including screen savers on heat stress, influenza vaccination, respiratory infection prevention, and tobacco control. In collaboration with the Staff Care and Wellbeing Division, dedicated wellbeing sessions were also delivered for health department colleagues.

Health communication also played a central role in evidence generation, research dissemination, and programme improvement. UNRWA contributed to several high-impact publications, including a cross-regional study on tobacco



Communication and knowledge products of UNRWA Department of Health, 2025.

and e-cigarette use among adolescent Palestine Refugees⁴, based on the Global Youth Tobacco Survey conducted in collaboration with WHO and CDC. Additional publications addressed the deterioration of health outcomes in the Gaza Strip and the rebuilding of the health information system⁵, reinforcing UNRWA's role in generating evidence from protracted crisis settings.

Operational research and monitoring efforts were further supported through follow-up on the Patient Satisfaction Survey action plan, including workshops across fields to strengthen service quality, accessibility, and patient experience.

In emergency contexts, health communication remains essential for risk communication and community engagement. In 2025, UNRWA's health team continued to support the emergency response in the Gaza Strip, including by disseminating findings on nutrition and health conditions, developing advocacy materials that highlight staff and patient experiences, and contributing to global platforms, such as the WHO dashboard on promising practices in migration and health. Advocacy and visibility were further strengthened through active participation in global and regional forums, including the World Health Assembly and international conferences. UNRWA's Department of Health conducted the UNRWA-Lancet

side event at the 78th World Health Assembly under the theme "Broken Systems, Unbroken Spirits," in collaboration with WHO and the International Federation of Medical Students' Associations (IFMSA). Additional engagements included the IFMSA regional meeting and international health conferences, supporting knowledge-sharing and strategic partnerships.

Transparency and institutional communication remained a priority. This included the development and dissemination of the 2024 Annual Health Report and its executive summary, the United Nations Office at Geneva (UNOG) press conference, and continuous updates to the health intranet and the new UNRWA website. Communication efforts also included the publication, branding, and dissemination of technical instructions, guidelines, fact sheets, and operational tools, including updates to laboratory services and maternal health and family planning.

Finally, UNRWA's health communication focused on strengthening external coordination and visibility, including facilitating media engagements, producing advocacy content, and commemorating key milestones—such as marking two years of war in the Gaza Strip through staff-focused storytelling and multimedia content on the impact of the war on health systems.

⁴ Vollebregt S, Arafah A, Fouad H "Prevalence of tobacco use among adolescent Palestine refugees: A cross-regional study in United Nations Relief Works Agency schools in Gaza, Jordan, Lebanon, Syria and the West Bank" Preventive Medicine Reports, Volume 58, 2025, 103190, 2211-3355. <https://www.sciencedirect.com/science/article/pii/S2211335525002293?via%3Dihub>.

⁵ Yaghmaei N, Rood E, Arafah A, Whaidi Z, Zeidan W, Spiegel PB, et al. "The Gaza health information system: Rebuilding for a resilient future", PLOS Glob Public Health, Volume 5, 2025, e0004700. <https://journals.plos.org/globalpublichealth/article?id=10.1371/journal.pgph.0004700>



A pregnant Palestine Refugee woman receiving ANC supplements counselling from an UNRWA midwife at Hamad Health Centre in the Khan Younis area, the Gaza Strip. ©2025 UNRWA photo.

outcome 1: palestine refugees maternal and child health

Maternal health

overview

UNRWA delivers a comprehensive and integrated package of health services designed to protect, promote, and sustain the health and well-being of Palestine Refugee women, children, and families across all fields of operation. These services include maternal healthcare, child health services, school health, disability care, nutrition interventions, gender-based violence (GBV) response, gender mainstreaming, and adolescent- and youth-friendly health services, delivered through UNRWA's primary healthcare (PHC) system. This integrated model reflects a life cycle approach, recognising the intergenerational determinants of health and the critical importance of continuity of care across key stages of life.

What are the maternal health services provided?

Maternal health remains a cornerstone of UNRWA's primary healthcare programme and a critical investment in reducing preventable maternal and neonatal morbidity and mortality among Palestine Refugees. UNRWA's maternal health services encompass family planning (FP), preconception care (PCC), antenatal care (ANC), delivery care through referral networks, and postnatal care (PNC). These services are delivered through UNRWA health centres and are designed to ensure early risk identification, timely intervention, and continuous follow-up, even in highly constrained and crisis-affected settings. Approximately 43,500 pregnant women were registered and have accessed antenatal care services at UNRWA health centres across the four fields of operation in 2025, excluding the

Gaza Strip. Additionally, around 22,000 pregnant women who were registered in 2024 have accessed maternal care in 2025 across the four fields of operation, excluding the Gaza Strip.

Family planning

UNRWA health centres ensure universal access to integrated family planning services, enabling women and couples to make informed decisions on fertility, birth spacing, and pregnancy timing. These services are delivered through counselling, modern contraceptive provision, and initiatives that promote male engagement, contributing directly to improved maternal and child health outcomes within primary healthcare. Around 92,000 women accessed UNRWA's family planning services in the four fields of operation last year, excluding the Gaza Strip. Approximately 11,000 of the women were newly enrolled in the programme.

Preconception care

Over the past decade, UNRWA has progressively strengthened preconception care (PCC) as a strategic, evidence-based intervention to reduce adverse pregnancy outcomes, maternal complications, and preventable maternal and neonatal mortality. Since its introduction as a standard service in 2011, PCC has become an essential pillar of maternal health services delivered across all UNRWA health centres, reflecting a shift towards upstream, preventive, and life cycle oriented maternal healthcare.

Preconception care aims to prepare women of reproductive age for pregnancy by optimising their health status prior to conception. Women attending PCC undergo systematic assessment for medical and psychosocial risk factors that may affect pregnancy and birth outcomes. This includes screening for hypertension, diabetes mellitus, anaemia, oral health conditions, and other chronic or acute illnesses, as well as targeted health counselling, lifestyle modification support, and health promotion interventions. Preventive measures include routine folic acid supplementation to reduce the risk of congenital anomalies, thereby improving maternal and newborn outcomes before pregnancy begins.

A key added value of preconception care lies in its contribution to early pregnancy detection and timely registration for antenatal care. Women who engage with PCC services are more likely to recognise pregnancy early and to register promptly for ANC, allowing the early initiation of evidence-based, quality antenatal care. Early registration remains a cornerstone of UNRWA's ANC programme, as it enables comprehensive risk stratification, personalised follow-up, and timely management throughout the continuum of pregnancy. Around 72.7 per cent of pregnant women registered- during the first trimester at UNRWA's ANC services and efforts are ongoing to increase the per cent across all fields.

Recognising its high impact, the Health Department placed renewed strategic emphasis on strengthening preconception care in 2025 as a core intervention to reduce preventable maternal morbidity and mortality. This prioritisation translated into measurable improvements across the Agency. The percentage of women newly registered in PCC services increased by 3.72 per cent Agency-wide, indicating enhanced uptake, awareness, and engagement with pre pregnancy services. In parallel, the strengthened focus on PCC contributed to a 3.42 per cent increase in the proportion of newly registered pregnant women, underscoring improved early pregnancy identification and stronger linkage between preconception and antenatal care services.

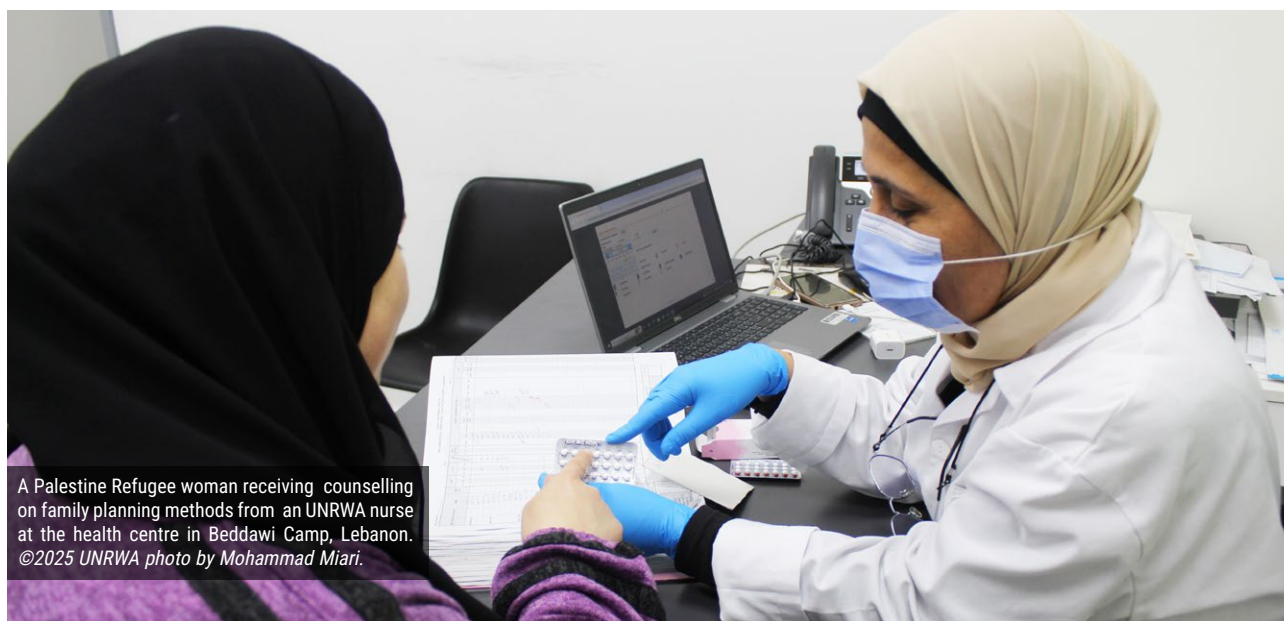
UNRWA registered 43,450 new pregnant women across the four fields (excluding the Gaza Strip) in 2025, compared to 42,206 in 2024, representing a 2.95 per cent year on year increase. These gains highlight the effectiveness of upstream investments in preconception and early pregnancy services and reinforce the role of PCC as a critical entry point into the maternal health continuum, particularly in fragile and crisis affected settings where timely access to care is essential for safeguarding maternal and newborn lives.

Strengthening quality, mental health, and protection in maternal care

UNRWA's Health Department advanced a major transformation of its maternal health programme last year, reinforcing an integrated, people centred model of care across all five fields of operation. A central achievement of this effort was the comprehensive revision of the maternal health framework, which now consolidates clinical quality, maternal mental health, and patient protection into interdependent, mutually reinforcing pillars of service delivery.

Updated maternal health technical instructions were developed through a collaborative, consultative process involving field teams and technical experts and were aligned with the latest World Health Organization (WHO) evidence based recommendations. These updates embedded explicit quality of care standards, strengthened referral pathways, and institutionalised routine maternal mental health screening within daily clinical practice. This integrated approach enables the early identification and management of psychosocial risks alongside biomedical care, ensuring more holistic and responsive services for pregnant and postpartum women.

To support consistent implementation across diverse and often challenging operational contexts, 15 Trainers of Trainers (TOTs) from the five fields were trained to lead cascade trainings at field level, aimed at strengthening frontline clinical competencies, psychosocial support skills, and adherence to updated protocols. This investment has promoted greater



A Palestine Refugee woman receiving counselling on family planning methods from an UNRWA nurse at the health centre in Beddawi Camp, Lebanon.
©2025 UNRWA photo by Mohammad Miari.

standardisation of care, reinforced quality assurance mechanisms, and enhanced the sustainability of the revised maternal health model at the point of service delivery.

The protection dimension of maternal health services was also significantly strengthened. The Medical Examination Requiring Chaperone Technical Instruction was updated to further safeguard privacy, dignity, and informed consent during clinical examinations. In parallel, maternal health services were more explicitly linked with gender based violence (GBV) response pathways. The introduction of a streamlined PRIMERO* based referral form enabled secure documentation, referral, and follow up of GBV cases, strengthening coordination with UNRWA's Relief and Social Services Department. Pilot implementation in the occupied West Bank, including East Jerusalem, and Jordan demonstrated faster, more accountable survivor centred

protection responses, further positioning maternal health services as trusted and safe entry points for comprehensive, rights based care.

Integrated services beyond maternal health

Beyond maternal health, the Health Department continued to advance evidence-based nutrition services, strengthen school health interventions, and promote disability inclusion and adolescent well-being. These efforts reinforce UNRWA's integrated PHC approach, addressing the interconnected health needs of women, children, and families across the lifecycle, and contributing to improved resilience and equity in refugee health outcomes.

(For the full maternal health report, please see Annex 4.)

Trend of mother's health statistics and indicators over the past years

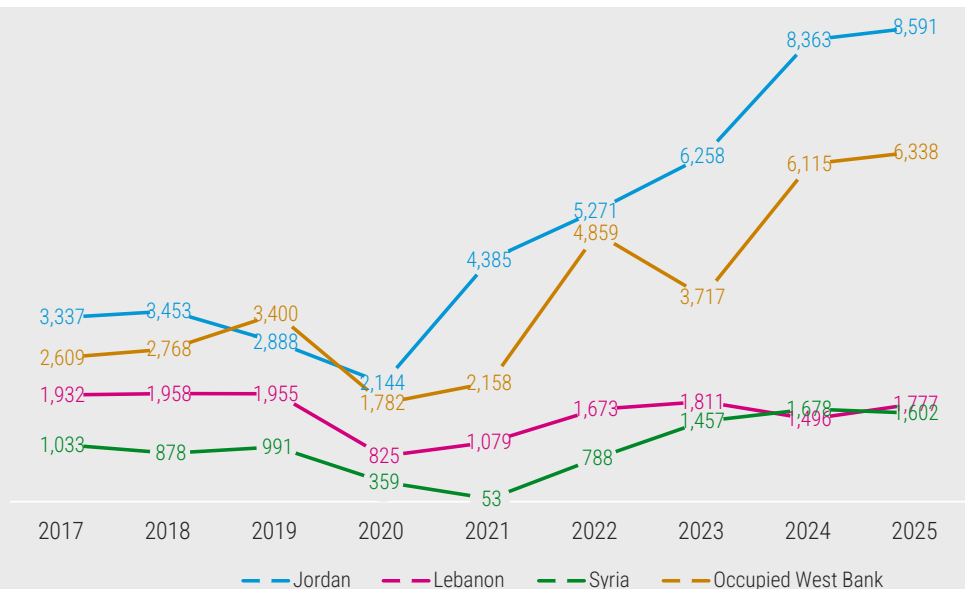


Figure 11: Number of women newly registered in preconception care service in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, from 2017 to 2025.

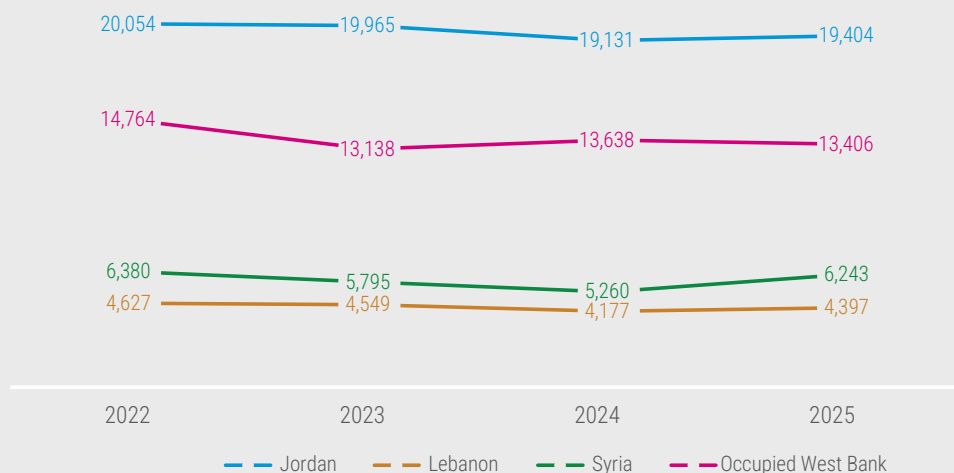


Figure 12: Number of newly registered pregnant women in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, from 2022 to 2025.

* PRIMERO is a secure, UNICEF-supported case management information system used to manage protection cases, including gender-based violence (GBV). It ensures strict data confidentiality through controlled access and protection protocols. Within UNRWA, it is used by the Relief and Social Services Department, with the Health Programme referring GBV cases through a standardized intake form linked to the system

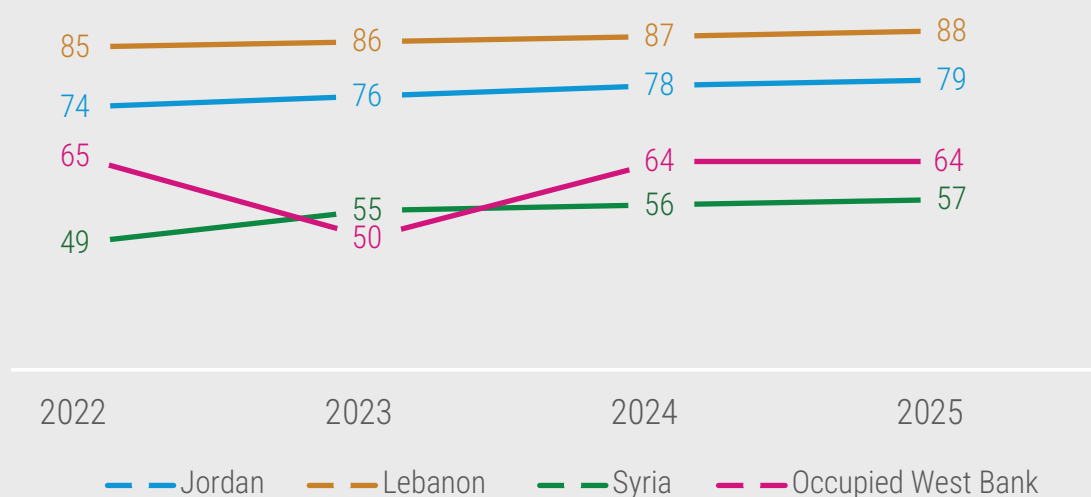


Figure 13: Percentage (%) of pregnant women registered in antenatal care service in the first trimester in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, from 2022 to 2025.

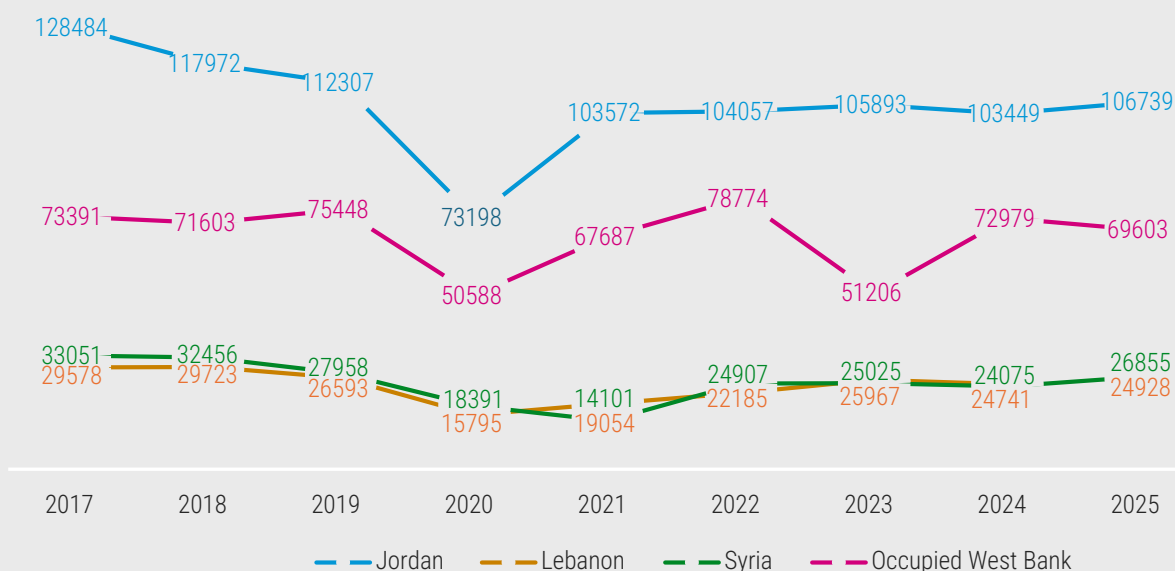


Figure 14: Number of pregnant women attending antenatal care services at any trimester in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, from 2017 to 2025.

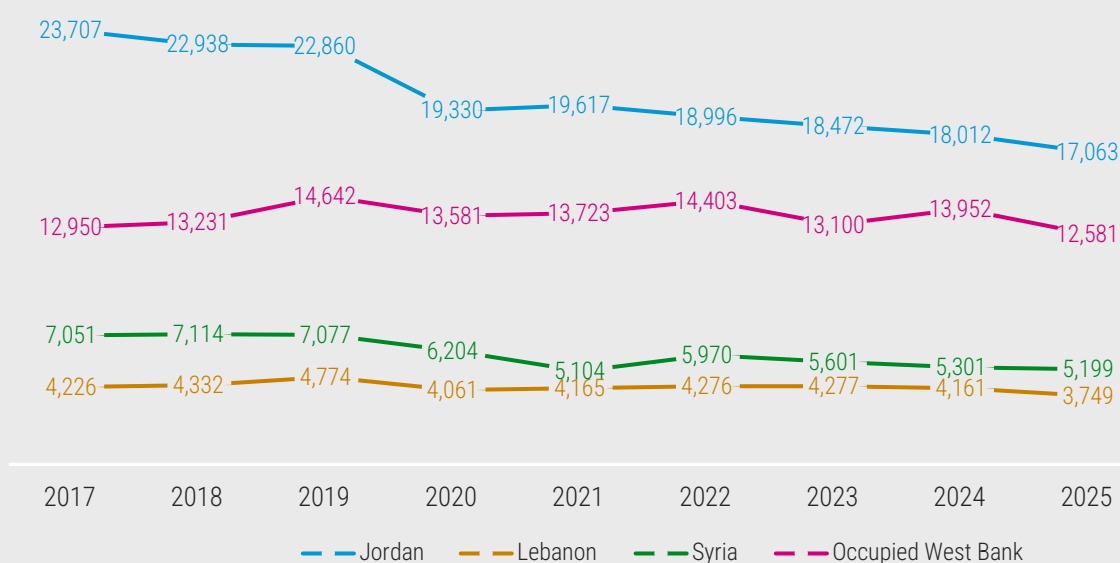


Figure 15: Number of pregnant women with live births in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, from 2017 to 2025.

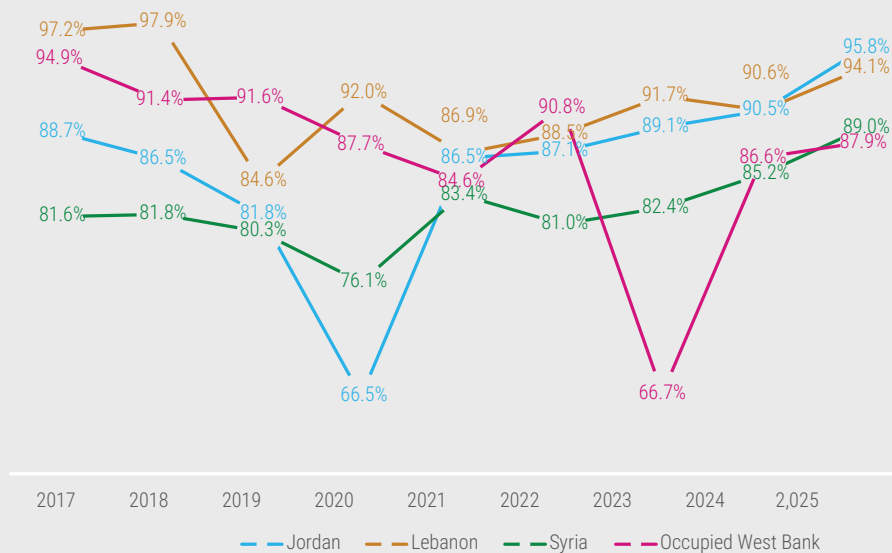


Figure 16: Percentage (%) of women covered by postnatal care services in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, from 2017 to 2025.

(The percentage of postnatal women who completed 6 postnatal visits was 91.6% in 2025 in the four fields excluding Gaza).

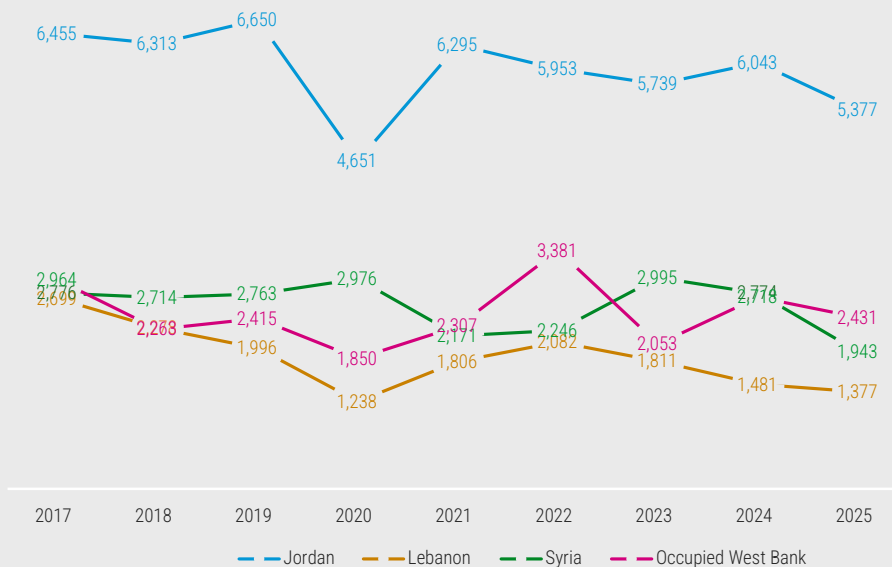


Figure 17: Number of new family planning acceptors in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, from 2017 to 2025.

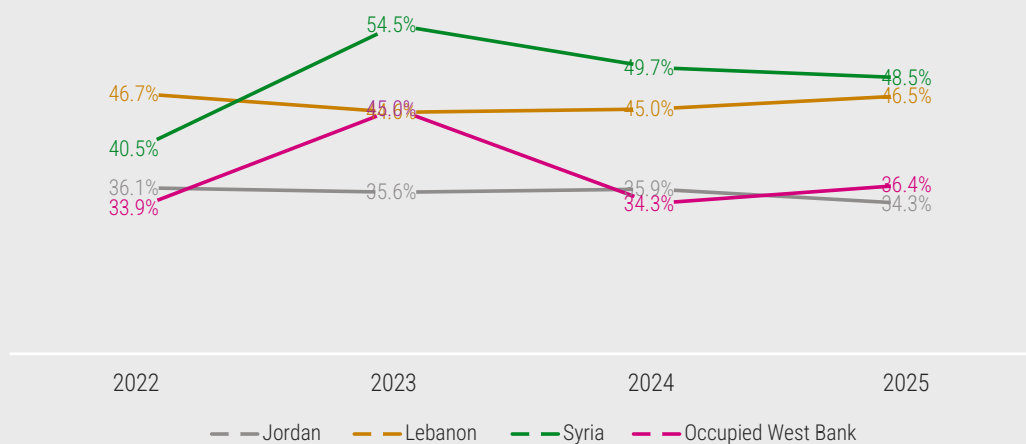


Figure 18: Percentage (%) of pregnant women with alert risk status in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, from 2022 to 2025.

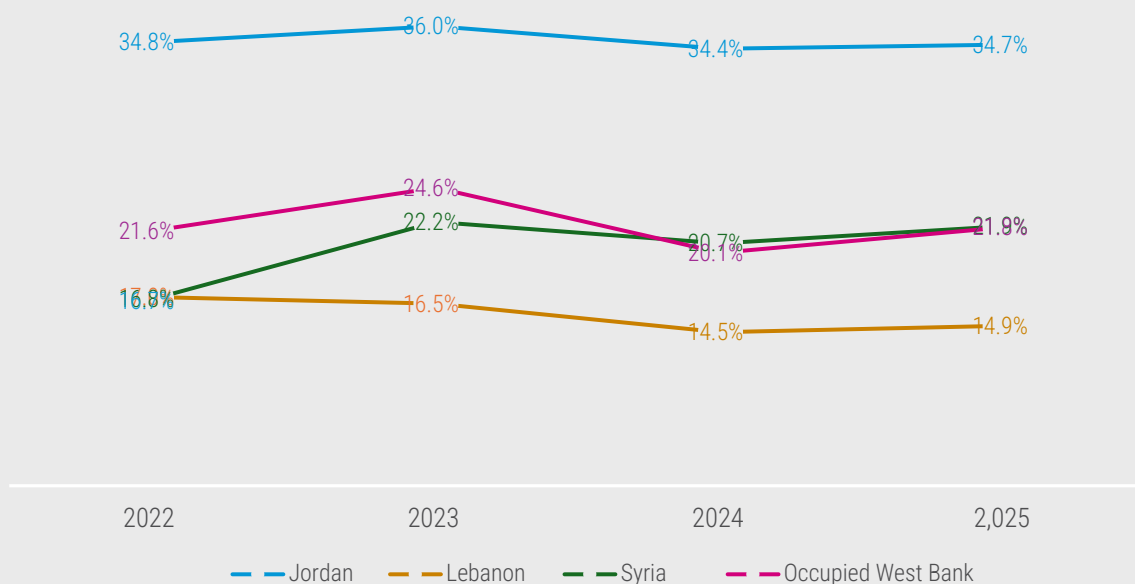


Figure 19: Percentage (%) of pregnant women with high risk status in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, from 2022 to 2025.

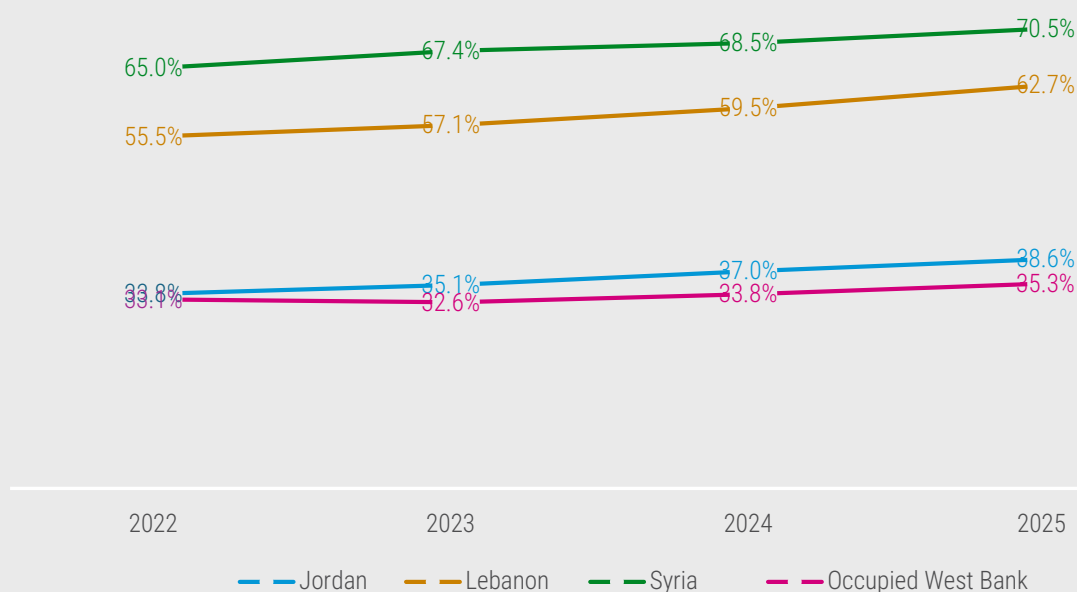
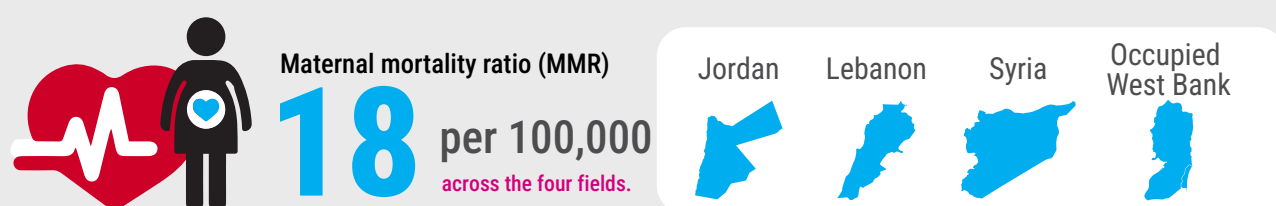


Figure 20: Percentage (%) of deliveries by caesarean section relative to total births in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, from 2022 to 2025.



Gender-based violence (GBV)

Gender-Based Violence (GBV) services within UNRWA's health programme are embedded within a broader framework of reproductive health, protection, and gender mainstreaming, reflecting a rights-based and survivor-centred approach. GBV response is integrated into maternal and reproductive health services, allowing women and girls to access care through familiar and less stigmatising entry points such as antenatal, postnatal, and family planning services. This integration facilitates early detection of GBV cases and ensures that survivors receive timely, confidential, and appropriate care. UNRWA's GBV services encompass clinical management of rape, psychosocial support, referral pathways to specialised services, and coordination with protection actors, including UNFPA and other partners. In crisis settings such as the Gaza Strip, where the breakdown of social systems and overcrowded shelters increase risks of violence, GBV services have been adapted to operate through

medical points and mobile teams, ensuring continued access despite infrastructure collapse. The Agency has also strengthened its collaboration with partners to provide essential supplies, including reproductive health kits and dignity kits, and to deploy trained midwives and health personnel capable of responding to GBV-related needs. Importantly, GBV interventions are linked with community awareness and risk mitigation strategies, aiming to reduce stigma and encourage reporting. The integration of GBV into health services also supports continuity of care, as survivors can receive medical, psychological, and social support within a coordinated system. Despite significant challenges such as resource constraints, insecurity, and access limitations, UNRWA has maintained a functional GBV response that prioritises dignity, confidentiality, and comprehensive care, reinforcing its commitment to protecting vulnerable populations in both stable and emergency contexts.

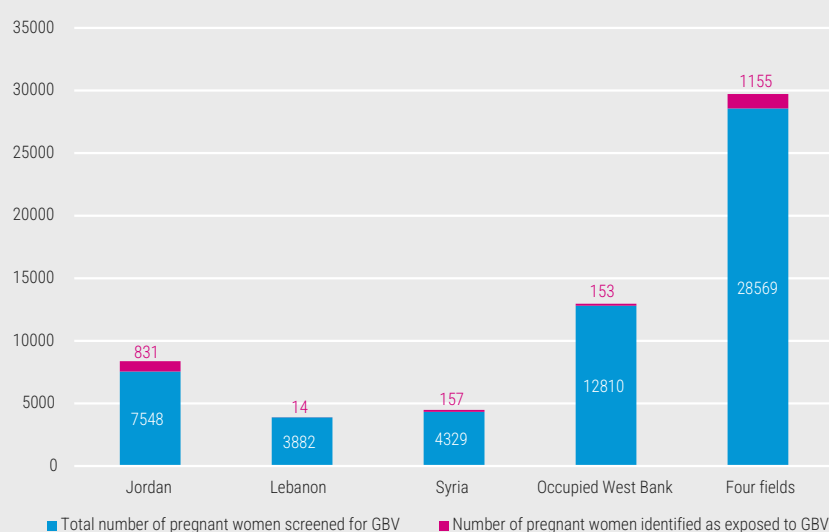


Figure 21: Numbers of pregnant women screened for gender-based violence and those identified with gender-based violence exposure in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025

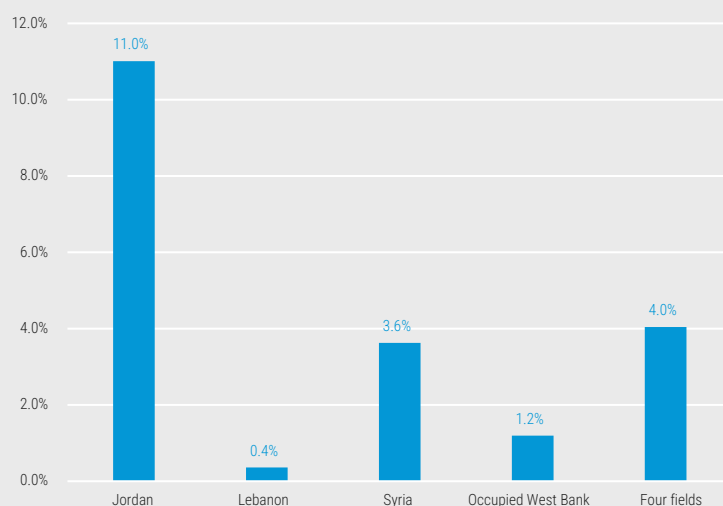


Figure 22: Percentage (%) of gender-based violence survivors identified out of the total women screened for gender-based violence in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.

(For detailed numbers per each field, please see Annex 5.)

Infant and child health

Infant and childcare

UNRWA continues to provide comprehensive child healthcare as a cornerstone of its primary healthcare programme, protecting and improving the health and well-being of Palestine Refugee children through a life-cycle, preventive approach. Delivered through the Family Health Team model, child health services span from maternal and newborn care to early childhood and adolescence, ensuring continuity and integration across critical stages of development. Core services include newborn assessments, periodic physical examinations, routine immunisation, growth monitoring and nutritional surveillance, micronutrient supplementation, preventive oral healthcare, and timely referral for specialised care.

Last year, UNRWA health centres provided preventive services to 198,159 children under the age of five.

Immunisation coverage remained high across fields, reaching 98.4 per cent among children aged 12 months and 97.7 per cent among those aged 18 months, reflecting strong service delivery and effective follow-up. Growth monitoring and nutritional surveillance, supported by the e-Health system and aligned with WHO standards, continued to identify the persistent double burden of malnutrition. In response, UNRWA intensified nutrition counselling, updated clinical guidelines and health education materials, and continued routine iron and vitamin A supplementation, anaemia screening, and referral for hereditary anaemias when indicated, reinforcing child health services as a lifelong investment in resilience, wellbeing, and human development. (For the infant and child healthcare report, please see Annex 6.)



A Palestine Refugee mother waiting for healthcare services for her child at the Japanese Health Centre in the Khan Younis area, the Gaza Strip. ©2025 UNRWA photo.

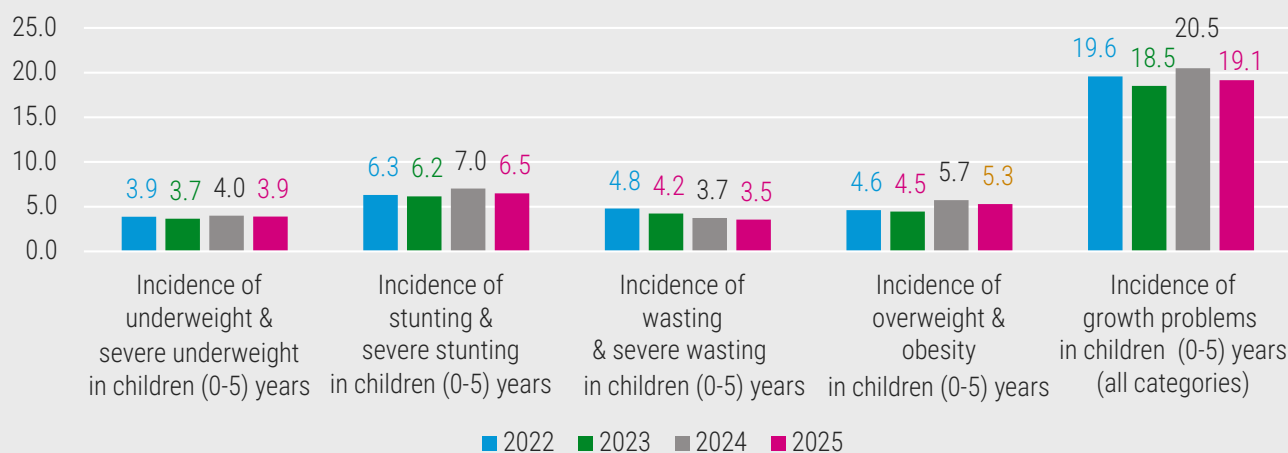


Figure 23: Proportion (%) of growth problems in children (<5 years), in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, from 2022 to 2025.

(For full growth problems report, please see Annex 7.)

Infant and child (<5 years) mortality

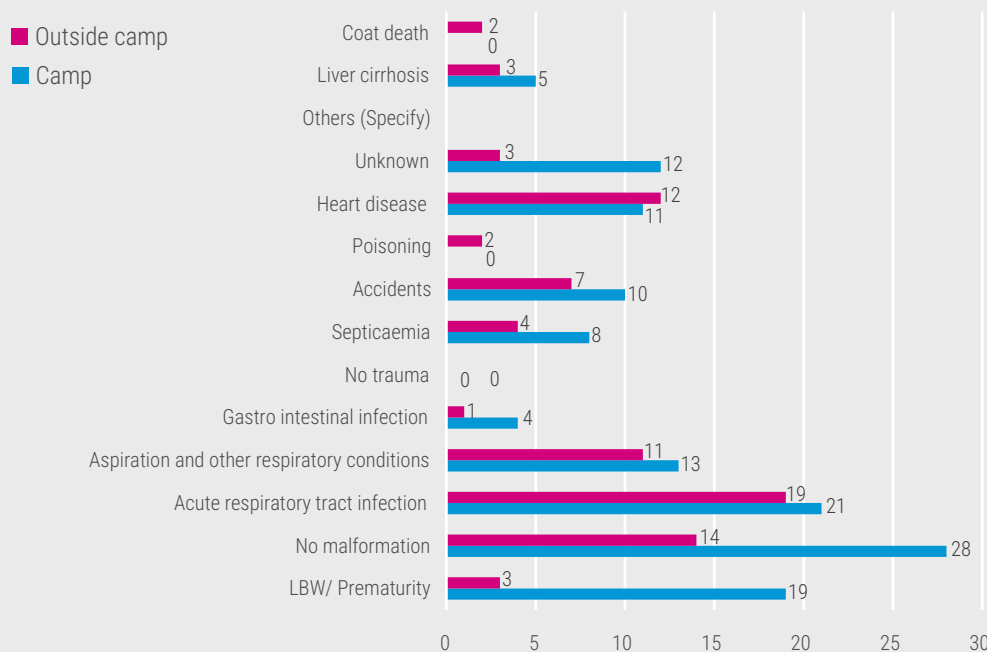


Figure 24: Distribution of causes of Infant and child (<5 years) mortality by living setting (In camp / Outside camp) in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.

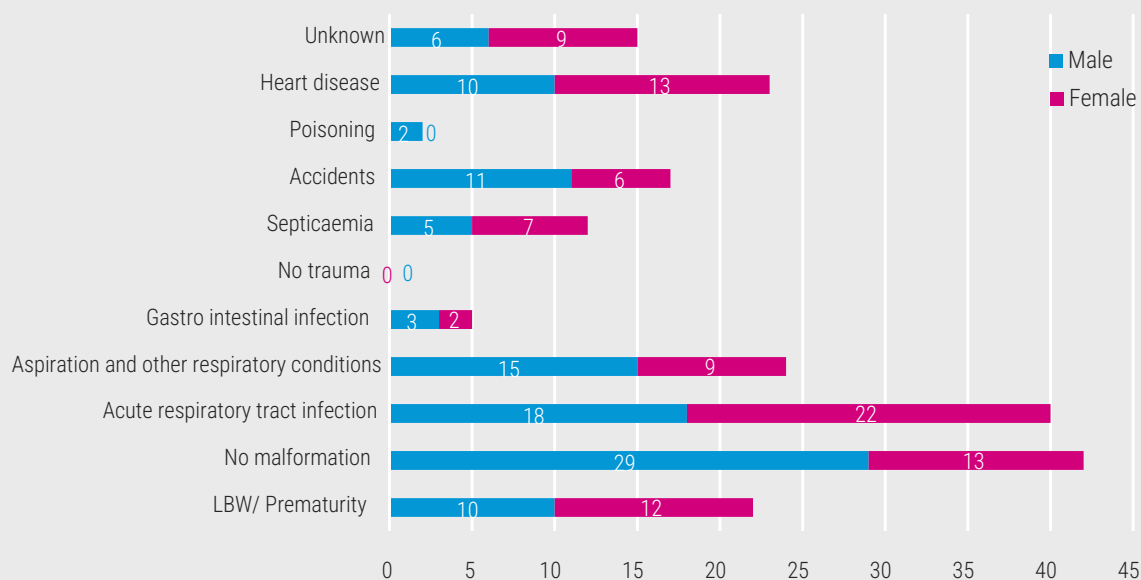


Figure 25: Distribution of causes of infant and child (<5 years) mortality by sex in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.

Table 1: Distribution of infant and child (<5 years) mortality by age, gender and place of death, in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.

Cause of death	Locality			Sex			Age in years			Place of death		
	Camp	Outside camp	Total	M	F	Total	<1	>1-<5	Total	Home	Hospital	Total
LBW/ Prematurity	19	3	22	10	12	22	22	0	22	0	22	22
Congenital malformation	28	14	42	29	13	42	28	14	42	5	36	41
Acute respiratory tract infection	21	19	40	18	22	40	34	6	40	8	32	40
Aspiration and other respiratory conditions	13	11	24	15	9	24	16	8	24	5	19	24
Gastrointestinal infection	4	1	5	3	2	5	4	1	5	0	5	5
Birth trauma	0	0	0	0	0	0	0	0	0	0	0	0
Septicaemia	8	4	12	5	7	12	9	3	12	0	12	12
Accidents	10	7	17	11	6	17	1	16	17	8	9	17
Poisoning	0	2	2	2	0	2	0	2	2	0	2	2
Heart disease	11	12	23	10	13	23	14	9	23	2	21	23
Unknown	12	3	15	6	9	15	12	3	15	5	10	15
Others												
Liver cirrhosis	5	3	8	4	4	8	7	1	8	1	7	8
Coat death	0	2	2	1	1	2	0	2	2	1	1	2
Leukaemia	0	0	0	0	0	0	0	0	0	0	0	0
Brain tumour	0	0	0	0	0	0	0	0	0	0	0	0
Fever	0	0	0	0	0	0	0	0	0	0	0	0
Meningitis	0	0	0	0	0	0	0	0	0	0	0	0
Spinal muscular atrophy	0	0	0	0	0	0	0	0	0	0	0	0
Epilepsy	0	0	0	0	0	0	0	0	0	0	0	0
Renal failure	0	0	0	0	0	0	0	0	0	0	0	0
Total	131	81	212	114	98	212	147	65	212	35	176	211
Total others	6	7	13	6	7	13	9	4	13	2	11	13

School health

UNRWA's School Health Programme delivers a comprehensive package of preventive, promotive, and protective health services to Palestine Refugee students through close collaboration between the Health and Education Departments and in line with the Primary Health Care life-cycle approach. Core programme components include medical examination of new school entrants, routine immunisation, systematic vision and hearing screening, oral health services, nutrition screening, deworming, and micronutrient supplementation, as well as follow-up of children with special health needs through

referral and continuity of care at UNRWA health centres. The programme also places strong emphasis on health education, psychosocial wellbeing, and the promotion of healthy behaviours, alongside regular inspection of school environments and canteens to ensure safe, hygienic and child friendly learning conditions. These services are delivered through scheduled school visits by multidisciplinary School Health Teams and are reinforced by School Health Committees and trained school staff, strengthening early detection of health risks and timely intervention among school age children.

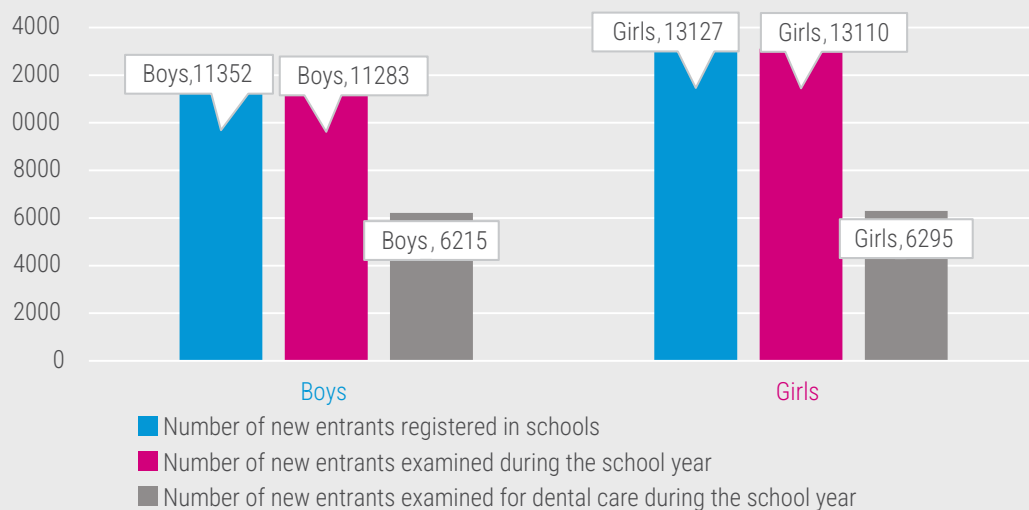
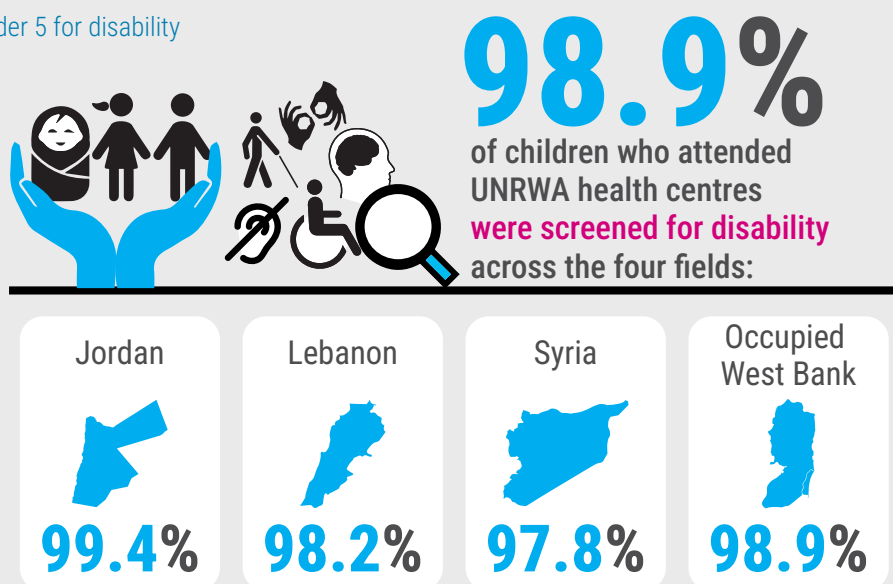


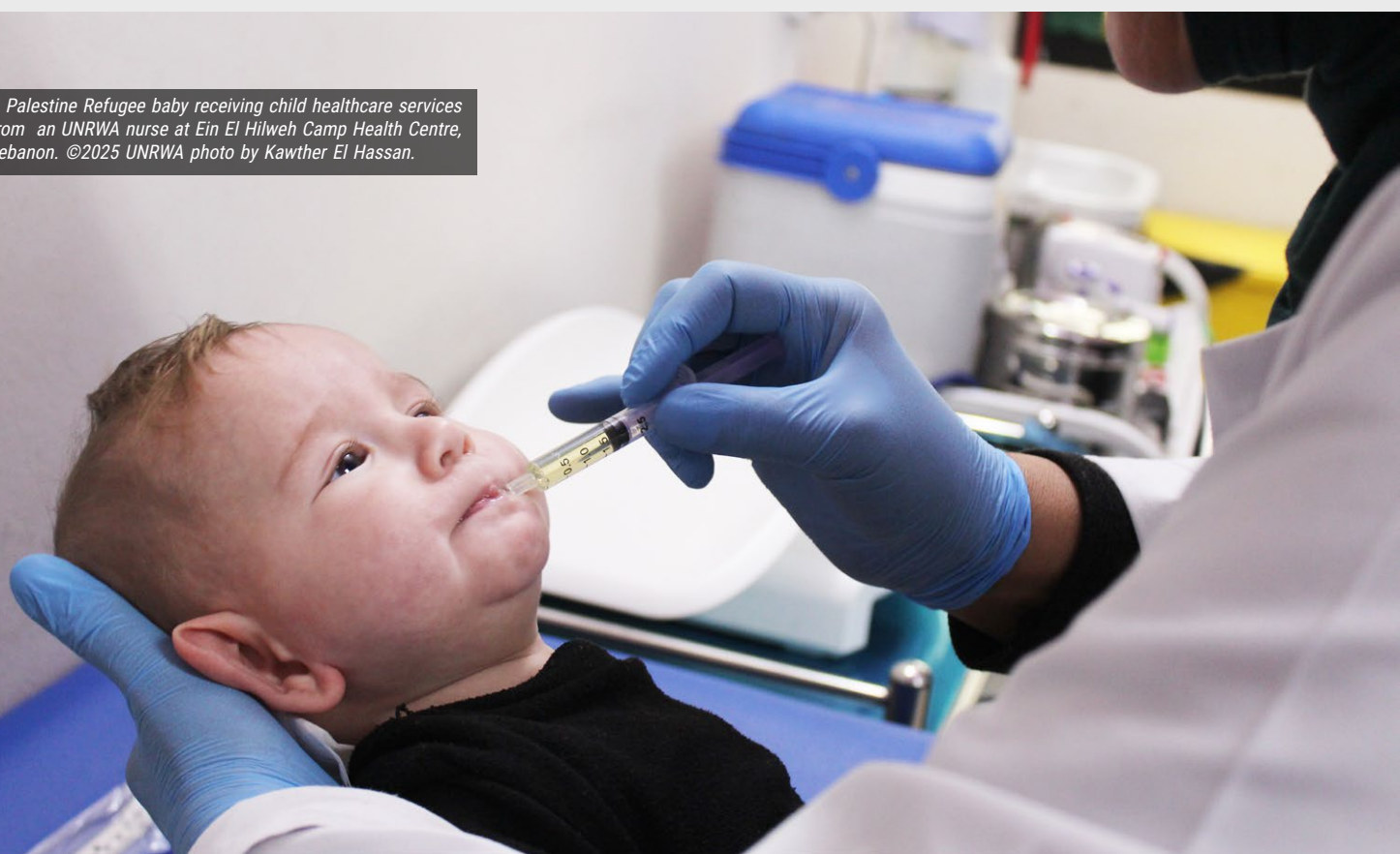
Figure 26: Number of children newly registered in schools and those medically examined in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.

(Please see the full school health report in Annex 8.)

Screening of children under 5 for disability



A Palestine Refugee baby receiving child healthcare services from an UNRWA nurse at Ein El Hilweh Camp Health Centre, Lebanon. ©2025 UNRWA photo by Kawther El Hassan.



Nutrition

Nutrition is a cross-cutting service integrated within UNRWA's primary healthcare programme, supporting Palestine Refugees across all life stages. It contributes to the prevention and management of both Non-Communicable Diseases (NCDs) and Maternal and Child Health (MCH) conditions. UNRWA continued efforts to strengthen early detection of malnutrition among children under five through growth monitoring and screening, while addressing nutritional deficiencies among pregnant and lactating women (PLW) and children. Nutrition services also included counseling on breastfeeding and complementary feeding, as well as individual nutrition consultations for patients with NCDs such as hypertension, diabetes mellitus (DM), and obesity. Additional details are presented in relevant sections of this report, including maternal health services, child health services, school health, and non-communicable diseases.

Nutrition needs and programme priorities vary across UNRWA fields of operation. In the Gaza Strip, the humanitarian disaster that began in October 2023 continued throughout 2025, severely affecting food security and access to essential services. Despite major operational constraints, UNRWA maintained life-saving nutrition services. As an active member of the UNICEF-led Nutrition Cluster and its Technical Working Groups, UNRWA supported screening for acute malnutrition among children aged 6–59 months using Mid-Upper Arm Circumference (MUAC) measurements in line with WHO and UNICEF guidance.

The situation deteriorated further in mid-2025. In August 2025, the Integrated Food Security Phase Classification (IPC) confirmed famine (Phase 5) "with reasonable evidence" in Gaza Governorate, with more than half a million people facing catastrophic food insecurity and severe hunger. "As of 15 August 2025, Famine (IPC AFI Phase 5)—with reasonable evidence—is confirmed in Gaza Governorate".

(IPC Global Initiative - Special Brief - Gaza Strip⁷) UNRWA continued to implement the Community Management of Acute Malnutrition (CMAM) approach for children aged 6–59 months, combining outpatient treatment with referrals to inpatient care when required. These activities continued despite damaged infrastructure, supply shortages, unstable communication systems, and severe restrictions on humanitarian aid, including prolonged limitations on food assistance entering the Gaza Strip during 2025. In response, UNRWA prioritised screening and treatment for children under five while expanding preventive and therapeutic nutrition support to other vulnerable groups, including pregnant and lactating women and school-aged children. Partnerships with humanitarian actors were essential to ensuring comprehensive healthcare and the availability of therapeutic and preventive supplements.

Nutrition surveillance system on child malnutrition in the Gaza Strip

During the two years of war in the Gaza Strip, child malnutrition became an increasing concern. In collaboration with UNICEF, WHO, and OCHA, UNRWA launched the Community Management of Acute Malnutrition (CMAM) programme across health centres and medical points in January 2024. In health centres, children aged 6-59 months visiting clinics for vaccination, growth monitoring, malnutrition screenings, or sick visits were screened for acute malnutrition using mid-upper arm circumference (MUAC) tapes donated by UNICEF. At Medical Points, which are primary care stations inside shelters and tented camps, UNRWA staff conducted outreach to inform families living in shelters and surrounding communities about the availability of nutrition services. Any children aged 6-9 months present at the medical points were screened for acute malnutrition using MUAC measurements. Children with MUAC results under 125 mm were enrolled in the therapeutic feeding programme and followed up periodically.



Palestine Refugees patients participating in a diabetes awareness session with UNRWA health staff at Camp No. 1 Health Centre in the occupied West Bank, including East Jerusalem. ©2025 UNRWA photo.

⁶ Integrated Food Security Phase Classification (IPC), Famine confirmed in Gaza Governorate.

⁷ https://www.ipcinfo.org/fileadmin/user_upload/ipcinfo/docs/IPC_Gaza_Strip_Acute_Food_Insecurity_Jul_Sep2025_Special_Report.pdf



As MUAC screening and CMAM therapeutic feeding services were scaled across the Gaza Strip, UNRWA's research team established a nutrition surveillance system to monitor and track the prevalence of child malnutrition. Data was collected and recorded by nurses and aggregated by UNRWA's Gaza Field Office on a biweekly basis and submitted to Headquarters for data quality checks, cleaning, re-organisation, and analysis. A standard operating procedure for MUAC screening and the CMAM programme was developed to standardise data collection, reporting, and submission processes across the Agency. Data processing procedures have been continuously reviewed and refined to improve data quality.

A new data management system was introduced in November 2025, and key staff in the Gaza Field Office, area offices, and clinics were trained to use it. Surveillance data were regularly shared with the UNICEF-led Nutrition Cluster and the Integrated Food Security Phase Classification (IPC). In 2025, 6-8 health centres and 48 medical points conducted 163,704 MUAC screenings among an estimated 131,800 children aged 6-59 months. Therapeutic feeding regimens were offered to more than 12,181 cases. The surveillance results of child malnutrition in the Gaza Strip during the war, from January 2024 to mid-August 2025, were published in *The Lancet*⁸ in October 2025.

Nutritionists work in the West Bank and Lebanon

Recruiting nutritionists within primary healthcare services in UNRWA clinics represents an important field initiative to strengthen nutrition care for Palestine Refugees. The initiative is currently implemented in the occupied West Bank and Lebanon.

In the occupied West Bank, nutritionists conducted rotational visits across UNRWA health centres to provide individual counselling and community-based nutrition education. Services included targeted dietary counselling for patients with diabetes and hypertension, as well as maternal nutrition support. Group education sessions addressed maternal and child nutrition,

prevention, and management of noncommunicable diseases, and seasonal health topics. Nutritionists also implemented school and community awareness activities promoting healthy diets, physical activity, and disease prevention, including sessions on breastfeeding, gestational diabetes, and healthy eating habits. Awareness campaigns were organised around key global health events such as World Diabetes Day, Breast Cancer Awareness Month, World Breastfeeding Week, 16 Days of Activism Against Gender-Based Violence, and International Day of Older Persons. Coordination with partners was strengthened through meetings and joint initiatives with the Ministry of Health, World Food Programme (WFP), UNICEF, Action Against Hunger, Medical Aid for Palestinians, and the Palestine Red Crescent Society, in addition to participation in nutrition cluster coordination meetings supporting emergency preparedness and response in vulnerable areas, including Jenin, Tulkarm, and Nur Shams when needed.

In Lebanon, nutritionists integrated into UNRWA primary healthcare centres have been providing comprehensive clinical and community-based nutrition services since August 2025, across the five operational areas: Beirut/Central Lebanon, North Lebanon, Saida (Sidon), Tyre, and Bekaa. On average, around 712 patient nutrition consultations per month were delivered, primarily supporting patients with noncommunicable diseases (approximately 447 consultations monthly), alongside maternal and child nutrition care. In addition to individual counselling, nutritionists conducted, on average, around 40 awareness and education sessions per month across clinics, schools, and communities, addressing key topics such as diabetes management, the Dietary Approaches to Stop Hypertension (DASH) diet and heart-healthy diets, breastfeeding, childhood obesity prevention, and food safety. These activities aimed to promote healthy dietary behaviours, strengthen disease prevention efforts, and support the early identification of nutrition-related risks among vulnerable groups. Nutritionists also contributed to capacity-building efforts, delivering more

⁸ Horino M, Al Najjar S, Tabaza A et al. "Assessment of malnutrition in preschool-aged children by mid-upper arm circumference in the Gaza Strip (January, 2024–August, 2025): a longitudinal, cross-sectional, surveillance study", *The Lancet*, 2025; 406, 1993-2002. <https://www.thelancet.com/action/showCitFormats?doi=10.1016%2FS0140-6736%2825%2901820-3&pii=S0140-6736%2825%2901820-3>

than 10 training sessions per month for clinic staff on topics including therapeutic supplements and diet, malnutrition in children under five and growth monitoring, food safety, and nutrition management for chronic diseases. Collaboration with partners and community institutions, including schools and kindergartens supported by UNICEF, further expanded outreach and strengthened early detection and prevention of malnutrition and obesity among children.

Nutrition education

Nutrition education programming in 2025 included electronic nutrition advice through the e-NCD and e-MCH mobile applications, which patients can access their health records, monitor their health conditions, receive appointment and medication reminders, and obtain health education, including nutrition advices, in addition to in-person educational sessions and counselling on specific topics, and the distribution of Information, Education, and Communication (IEC) materials in health centres to promote best practices. To align with recent global guidelines and improve service delivery, the Health Department at Headquarters updated key nutrition topics, such as management of diabetes in conflicts, nutrition management of anaemia, proper nutrition in pregnancy and others, in addition to modifications to the guidelines related to nutrition counselling within primary healthcare services for UNRWA staff.



Patient nutrition education and awareness brochure samples.

Multiple micronutrient supplementation (MMS)

Maternal anaemia and micronutrient deficiencies are a major public health burden among Palestine Refugees in the Middle East. Since the 1990s, UNRWA Health Programme has provided antenatal iron and folic acid (IFA) supplementation to address maternal anaemia. However, the effectiveness of the IFA supplementation remained uncertain. In alignment with the 2020 WHO recommendation to use MMS in the "context of rigorous research", UNRWA has advanced efforts to enable, plan, pilot and evaluate the replacement of the IFA regimen with the United Nations International Multiple Micronutrients Antenatal Preparation (UNIMMAP)-formulated MMS as the standard of care within its antenatal care

system. In collaboration with Vitamin Angel Alliance, Johns Hopkins Bloomberg School of Public Health, Sight and Life Foundation, and Kirk Humanitarian Foundation, UNRWA conducted 10-month implementation research in Jordan in 2023 to compare implementation outcomes between MMS delivery and the previous IFA regimen. The study documented improvements in MMS clinics in coverage, adherence, fewer side effects, greater acceptability, improved mid-pregnancy haemoglobin levels with a lower risk of anaemia, and some cost savings for patients and the health system. Based on the key findings, the Agency has decided to scale up the MMS system-wide. Since 2024, all 25 clinics in Jordan distribute UNIMMAP-formulated MMS at registration, reaching approximately 20,000 Palestine Refugee pregnant women annually. In 2025, the Agency worked on scaling up MMS across 26 UNRWA facilities in Lebanon and 26 UNRWA clinics in Syria, through pre-assessment, trainings of health officers and clinicians, and integration of MMS surveillance system. As of December 2025, all UNRWA health centres and mobile clinics in Lebanon and Syria provide a 180-count bottle of MMS to pregnant women at registration. Concurrently, the UNRWA Health Department has expanded the MMS surveillance system, which has enabled system-wide evaluation of inputs, throughputs, and outputs during MMS scale-up and implementation. Key findings on programme fidelity and sustainability are continuously communicated to the key programme staff to inform programmatic and policy adjustments.

Expanding health department staff capacity for nutrition knowledge and practices at UNRWA

The Agency enhanced efforts to strengthen nutrition knowledge and best practices among Health Department staff involved in NCD and MCH care services by offering an online course on the UNRWA Moodle platform. In 2025, 1,589 staff across the five fields were targeted to take the course, including medical officers, nurses, and midwives who were engaged in nutrition counselling and related services. Approximately 50 per cent of targeted staff across all five fields were enrolled in the course.

For instance, in the Gaza Strip, relevant health staff received both in-person and virtual training on emergency nutrition topics, including refresher training on the Community-Based Management of Acute Malnutrition (CMAM) and Infant and Young Child Feeding (IYCF) programmes, as well as training on reporting tools and programme documentation. In Lebanon, in-person training focused on strengthening nutrition services and introducing MMS during pregnancy, which has also been expanded to other fields, including Syria and Jordan. Multiple micronutrient supplementation during pregnancy is increasingly promoted in antenatal care programmes to address micronutrient deficiencies and improve maternal and birth outcomes.



A Palestine Refugee patient visiting the temporary Jenin medical point (at Jenin Club) in the occupied West Bank, including East Jerusalem. ©2025 UNRWA photo.

outcome 2: improved control status of non-communicable diseases

Non-communicable diseases (NCDs)

Overview of the programmes

Disease prevention within UNRWA's health programme is a core strategic priority aimed at reducing morbidity, mortality, and long-term health system burden among Palestine Refugees. The burden of non-communicable diseases (NCDs) continues to rise, driven by increased life expectancy, lifestyle changes, and disruptions in healthcare continuity caused by ongoing crises. Cardiovascular diseases, diabetes mellitus (DM), hypertension, chronic respiratory diseases, and cancer remain the leading causes of morbidity among Palestine Refugees. In 2025, UNRWA provided services to 216,622 NCD patients, distributed as follows: 87,135 for hypertension, 31,399 for DM, and 98,088 for both conditions across the four fields, excluding the Gaza Strip. Economic hardship has driven up risk factors such as malnutrition, obesity, and smoking, underscoring the importance of sustained prevention and management strategies. UNRWA's digital health initiatives, including its NCD mobile application, remain crucial for patient engagement and treatment adherence. The Agency adopts an integrated, primary healthcare-based approach that combines prevention, early detection, and continuous management of both noncommunicable diseases and mental health, along with psychosocial support services.

(For full NCD report please see Annex 9.)

The estimated Agency-wide (excluding Gaza) prevalence of DM and hypertension has slightly varied from the 2024 figures. Among patients aged 18 years and above, the prevalence of DM is 18.6 per cent, while the prevalence of hypertension is 26.7 per cent. Among those aged 40 years and above, the prevalence of having DM and HTN together reaches 31.3 per cent and 45.5 per cent, respectively. For individuals aged 60 years and above, the rates increased to 47.6 per cent for DM and decreased to 68.4 per cent for hypertension.

Noncommunicable disease (NCD) prevention focuses primarily on diabetes mellitus (DM) and hypertension (HTN), which represent a significant and growing burden. UNRWA implements standardised, WHO-aligned protocols emphasising risk factor modification, early diagnosis, regular monitoring, and continuity of care. Preventive efforts include lifestyle counselling, screening of at-risk populations, and structured follow-up systems to improve disease control and reduce complications.

Recognising the strong link between physical and mental health, UNRWA also integrates Mental Health and Psychosocial Support Services (MHPSS) into primary care. These services focus on early identification, basic mental healthcare, and referral pathways, particularly for vulnerable populations and patients with chronic conditions such as DM and HTN.

Through this comprehensive and integrated approach, UNRWA aims to enhance Palestine Refugees' health outcomes, strengthen resilience, and ensure equitable access to preventive and curative services under challenging operational contexts.

Measures, standards, interventions, and guidelines followed to ensure high-quality care

In 2025, UNRWA's Disease Prevention and Control (DPC) programme delivered quality care guided by a comprehensive set of evidence-based technical standards, operational guidelines, and quality-assurance mechanisms aligned with global public health norms and adapted to humanitarian settings. The programme was implemented within UNRWA's Primary HealthCare (PHC) model, ensuring continuity, equity, and person-centred care across all five fields of operation.

Core interventions were guided by UNRWA Technical Instructions and Guidelines on Disease Prevention and Control, which standardise prevention, early detection, treatment, and follow-up for both communicable and non-communicable diseases (NCDs). These guidelines are explicitly aligned with WHO normative guidance. The quality of care was further strengthened through the Family Health Team (FHT) approach, which promotes integrated, multidisciplinary, and longitudinal care across life stages.

To ensure clinical quality and accountability, UNRWA applied standardised screening protocols, risk stratification tools, and clinical audit mechanisms, particularly for priority NCDs such as diabetes and hypertension. Regular monitoring, use of indicator frameworks aligned with WHO and Sustainable Development Goal (SDG) targets, and structured referral and follow-up pathways supported continuous quality improvement across health centres.

Collectively, these measures enabled UNRWA to provide safe, effective, and equitable disease prevention services in 2025, even amid conflict, displacement, and resource constraints, while contributing practical lessons to global public health and humanitarian practice.

Outcomes achieved and expected impact of implemented outputs

In 2025, the systematic implementation of UNRWA's Disease Prevention and Control (DPC) interventions translated programmatic outputs into measurable health service outcomes and sustained population-level impact, despite severe operational constraints. Strengthened application of standardised technical guidelines, integrated service delivery, and continuous monitoring contributed to improved service continuity, early detection, and quality of non-communicable disease care.

Consistent application of screening, risk stratification, and follow-up protocols resulted in earlier diagnosis and improved clinical monitoring of patients with diabetes and hypertension. Integration within the Family Health Team approach supported continuity of care, adherence to treatment, and management of multimorbidity, contributing to improved control of disease progression and prevention of avoidable complications. Additionally, mental health integration strengthened health-seeking behaviour and risk awareness, particularly among vulnerable groups.

Despite all the challenges, including difficult security situations and restrictions on access to UNRWA health services in the Gaza Strip and the occupied West Bank, the screening for NCDs continued. The health programme across the Agency screened 160,000 patients with risk factors for diabetes in 2025. Six per cent of them were found to have diabetes, hypertension, or both (two per cent DM, three per cent HTN, and one per cent, both conditions). The detection rate remained the same as the previous year across the agency. Moreover, control status among patients with diabetes has improved compared with 2024 across the four fields, excluding the Gaza Strip (34 per cent vs 32 per cent).

Using the GHQ-12 tool, 139,000 Palestine Refugees were screened for mental health and psychosocial issues. Of those, 12 per cent were identified to have MHPSS issues (score >6), and 99 per cent were provided with assistance. This is a significant improvement compared to 9.85 per cent detection in 2024 and reflects the magnitude of the mental health and psychosocial issues among Palestine Refugees.

Collectively, these outcomes are likely to yield long-term impact through reduced avoidable morbidity and disability, preservation of the functionality of overstretched health services, and increased resilience of refugee communities. The 2025 experience further reinforced UNRWA's role in generating operational public-health evidence applicable to humanitarian and protracted crisis contexts worldwide.

Challenges and limitations hindered optimal outcomes and impact

Despite significant achievements in 2025, the scale and impact of disease prevention interventions were constrained by a complex set of contextual, operational, and systemic challenges. Protracted conflict, insecurity, and extensive infrastructure damage, particularly in the Gaza Strip and the northern part of the occupied West Bank, disrupted routine preventive services, referral pathways, and follow-up care, while repeated population displacement undermined continuity of screening and long-term management of non-communicable diseases.

Severe supply chain disruptions, driven by import restrictions, border closures, and global shortages, led to intermittent stockouts of essential medicines, diagnostics, and cold-chain items, necessitating therapeutic substitutions and prioritising life-saving interventions over comprehensive prevention. To read Pharmaceuticals section please go to page 51.

These programme-level challenges were compounded by wider, Agency-wide constraints, including funding reductions, persistent humanitarian access restrictions, and heightened political and security pressures across multiple fields. Chronic underfunding limited maintenance of health facilities, digital systems, and workforce surge capacity, forcing difficult tradeoffs between sustaining essential services and investing in upstream prevention.

In the occupied West Bank, including East Jerusalem, the implementation of restrictive legislative and administrative measures further constrained UNRWA's operations, translating into access limitations, administrative uncertainty, and operational disruptions. Movement restrictions, permit constraints, and increased security incidents affected both patient access and staff mobility, while high workloads, fatigue, and exposure to trauma reduced capacity for health promotion, MHPSS, and community-based prevention. Collectively, cross-cutting challenges increase population vulnerability while limiting the pace at which health gains could be consolidated.

Success stories demonstrate overcoming challenges and bottlenecks

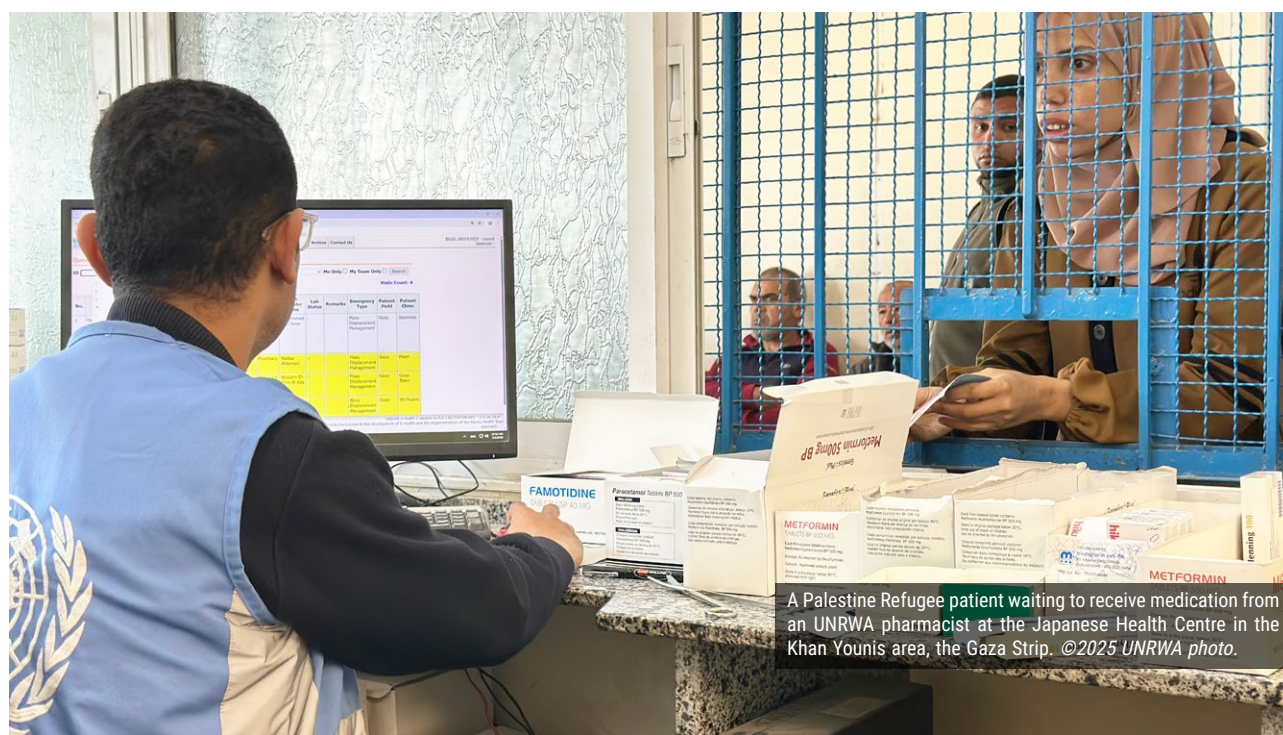
Despite operating in one of the most constrained humanitarian contexts globally, UNRWA demonstrated notable resilience and adaptability in overcoming major operational bottlenecks during 2025. In the

Gaza Strip, where extensive damage to health infrastructure severely limited access to fixed facilities, UNRWA successfully shifted service delivery to mobile medical teams, shelter-based health points, and temporary clinics, ensuring continuity of essential preventive and curative services. Health teams maintained disease surveillance, non-communicable disease (NCD) follow-up, and emergency triage despite repeated displacement and insecurity.

To address disruptions to digital health systems and patient records, UNRWA activated emergency e-health modalities, safeguarding continuity of care for patients with chronic diseases, and preserving core monitoring functions. Supply chain bottlenecks were mitigated through close coordination with WHO and partners, prioritisation of life-saving medicines, therapeutic interchange policies, and adaptive stock management.

In the occupied West Bank, despite movement restrictions and administrative constraints, UNRWA health staff maintained service delivery through emergency clinics, telemedicine, flexible scheduling, decentralised planning, and strengthened community engagement, enabling continued screening, follow-up, and the integration of mental health and psychosocial support (MHPSS). UNRWA tracked the continuity of care for diabetic patients based on their place of registration, following the closure of three UNRWA health centres, Jenin, Nur Shams, and Tulkarm, in the northern occupied West Bank, between December 2024 and February 2025. Of the 5,273 patients tracked, 93 per cent, corresponding to 4,902 patients, sought diabetes consultations at other UNRWA health centres in the occupied West Bank in 2025. The emergency health clinics provided diabetic care to 70.2 per cent of those who visited UNRWA health centres, corresponding to 3,443 patients.

Across all fields, the commitment and adaptability of frontline



A Palestine Refugee patient waiting to receive medication from an UNRWA pharmacist at the Japanese Health Centre in the Khan Younis area, the Gaza Strip. ©2025 UNRWA photo.

health workers, supported by strong interagency coordination and evidence-based operational adjustments, enabled UNRWA to sustain disease prevention services and protect health gains under exceptionally challenging conditions.

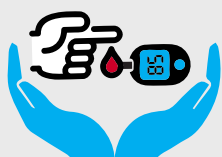
Future outlook, planned actions, and strategies for sustaining achievements and managing emergencies

The UNRWA NCD programme is increasingly oriented toward resilience, integration, and data-driven service delivery to sustain achievements and respond effectively to future challenges. Building on established strengths in primary healthcare and standardised protocols, the programme is advancing a forward-looking approach that balances routine care with emergency preparedness.

On the ground, the programme is prioritising the consolidation and scale-up of digital health systems, including enhanced use of e-health platforms – Electronic Medical Records (EMR) – for patient tracking, clinical decision support, and real-time performance monitoring. Strengthening data quality and analytics, through tools such as dashboards and business intelligence applications, will support timely, evidence-based decision-making across all fields. Continued implementation and updating of technical guidelines aligned with WHO's Package of Essential Noncommunicable Disease Interventions (PEN) and HEARTS technical package⁹ will ensure standardised, high-quality care. Service delivery

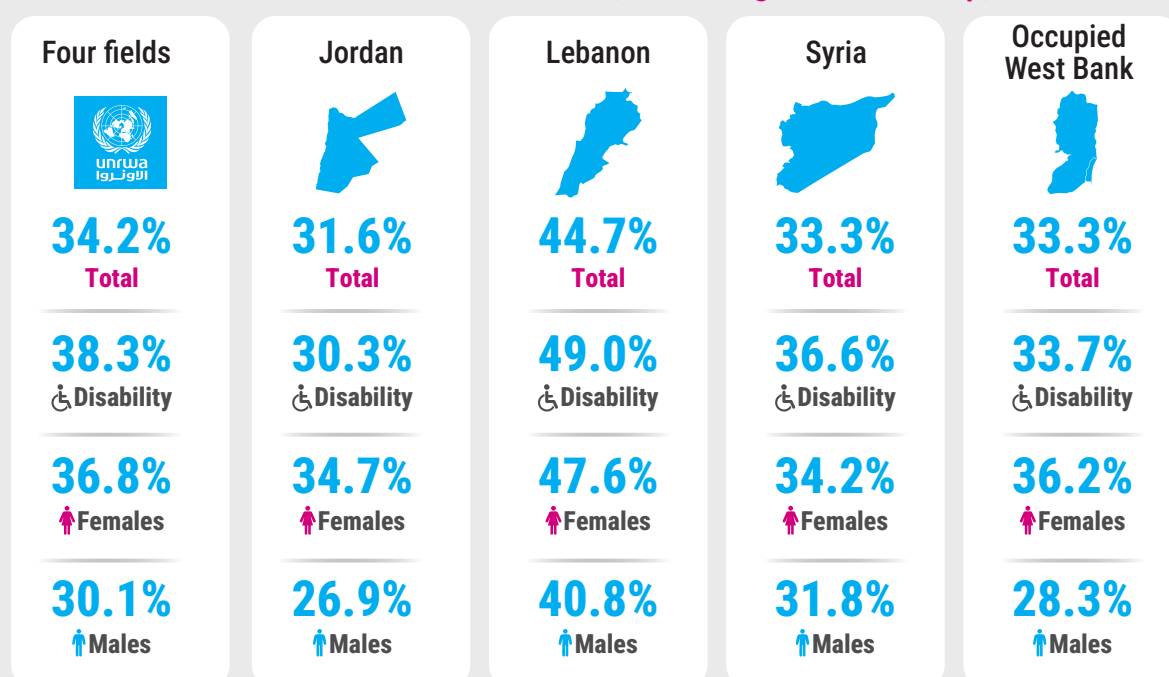
models are being adapted to improve efficiency and continuity, including differentiated care approaches, such as extended intervals for stable patients and intensified follow-up for high-risk groups, expanded telemedicine services, and better integration of MHPSS within NCD care. Workforce capacity-building remains central, with targeted training to enhance clinical management, counselling, and the use of digital tools. To maintain achievements and anticipate emergencies, the programme is strengthening supply chain resilience for essential medicines and diagnostics and embedding contingency planning within routine operations. This includes flexible service delivery modalities, mobile outreach where feasible, and prioritisation frameworks during crises.

Further efforts are needed to deepen integration between NCD, MHPSS, and communicable disease services, enhance community engagement and patient self-management, and institutionalise digital clinical audits. Strengthening partnerships with WHO, other UN agencies and other stakeholders will be critical to mobilise resources and align interventions. Strengthening partnerships with WHO, other UN agencies, and other stakeholders will be critical to mobilise resources and align interventions. By combining system strengthening, innovation, and adaptive planning, the UNRWA NCD programme aims to sustain service delivery, protect gains in disease control, and remain responsive to evolving humanitarian and public health challenges.



Diabetes Mellitus (DM)

percentage of Diabetes Mellitus (DM) patients, under control agency-wide and in the four fields, excluding the Gaza Strip, in 2025.



⁹ The WHO-developed HEARTS technical package aims to improve cardiovascular disease management in primary healthcare. HEARTS stands for healthy-lifestyle counselling, evidence-based treatment protocols, access to essential medicines and technology, risk-based CVD management, team-based care and systems for monitoring.

Table 2: Age- and sex-specific screening and detection of Diabetes Mellitus (DM) and Hypertension (HTN) in the four fields, excluding the Gaza Strip, in 2025.

Indicator	Age Group	Male	Female	Total
Screened cases	< 40 years	8,177	31,677	39,854
	≥ 40 years	43,019	77,027	120,046
Newly detected DM	< 40 years	141	190	331
	≥ 40 years	1,079	1,484	2,563
Newly detected HTN	< 40 years	147	294	441
	≥ 40 years	1,939	2,629	4,568
Newly detected DM & HTN	< 40 years	33	26	59
	≥ 40 years	456	471	927

(Please see Annex 10 for full report).

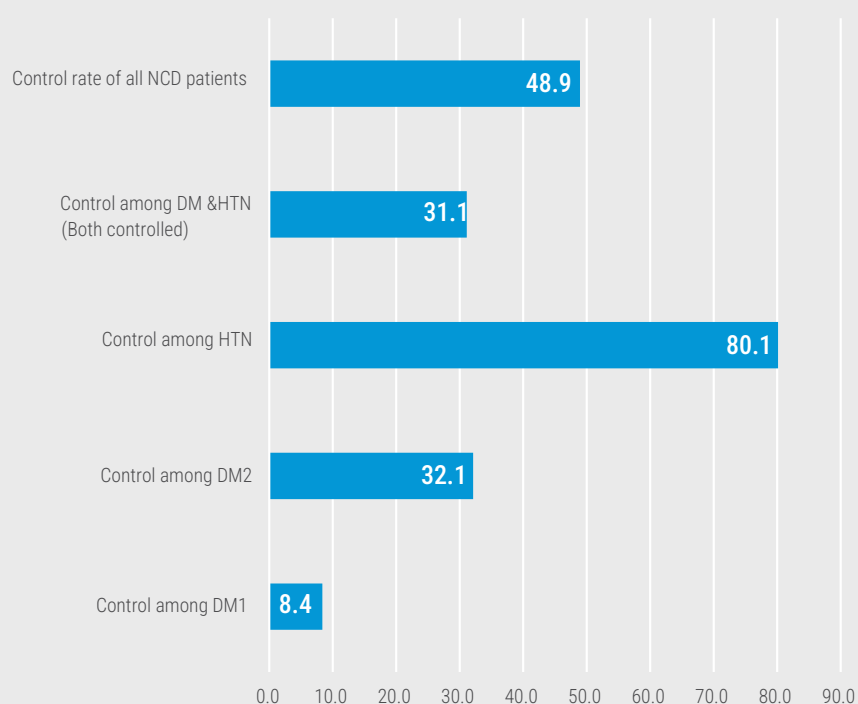


Figure 27: Proportion (%) of patients with noncommunicable diseases with controlled conditions in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.

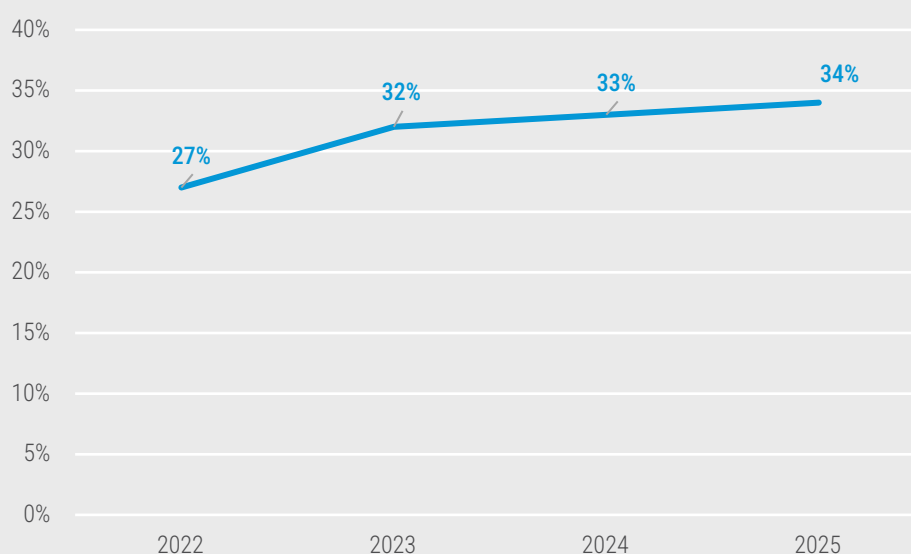


Figure 28: Percentage of diabetes patients with controlled HbA1c in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, from 2022 to 2025.

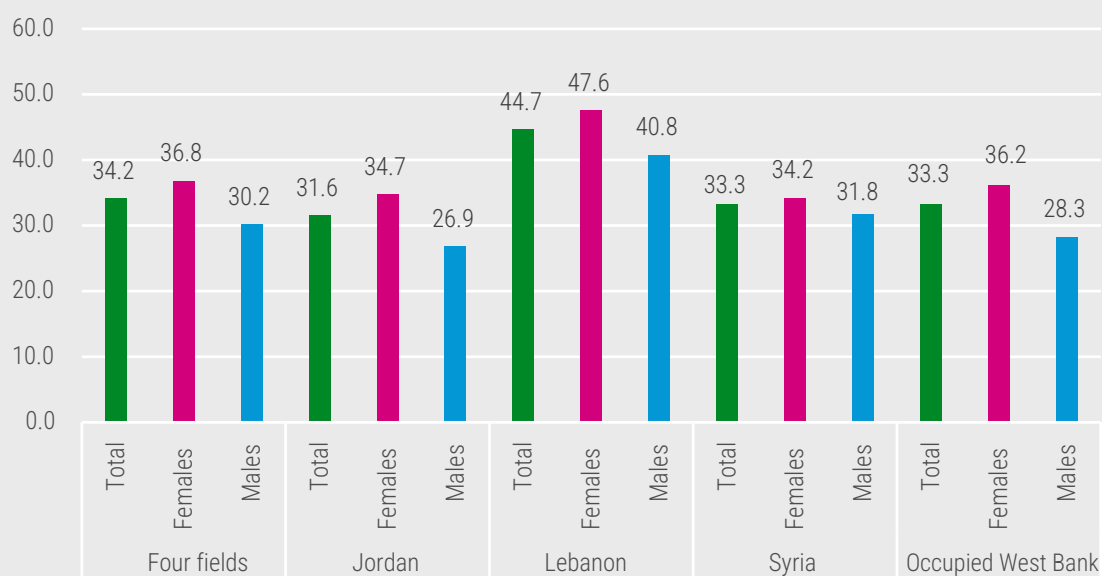


Figure 29: Proportion (%) of patients with diabetes mellitus controlled by sex in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.

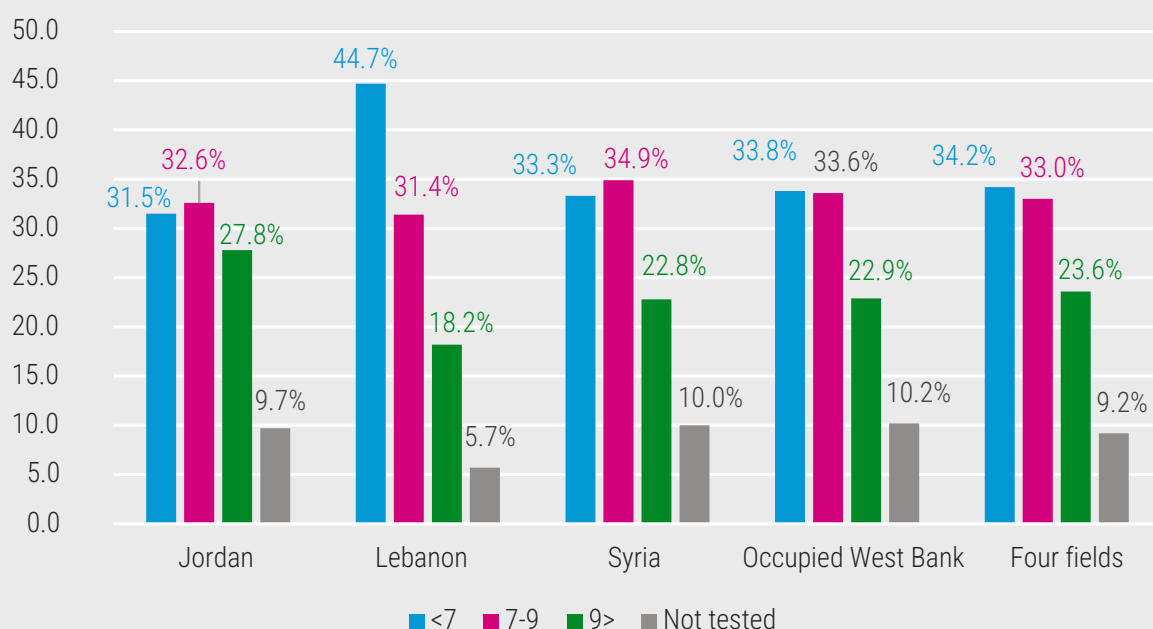


Figure 30: Proportion of diabetes mellitus patients by glycated haemoglobin level in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.

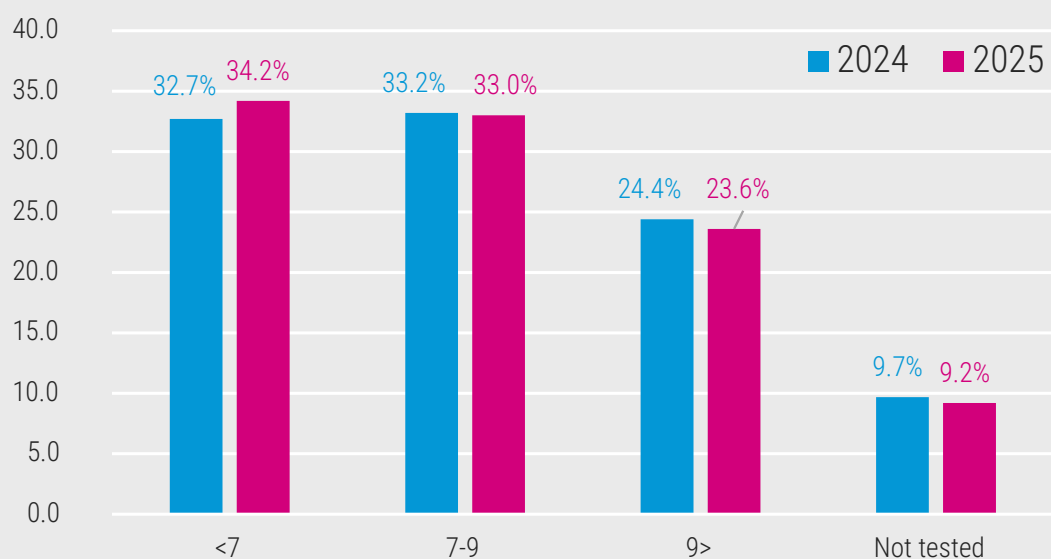


Figure 31: Distribution of diabetes mellitus patients by HbA1c level in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2024 and 2025.

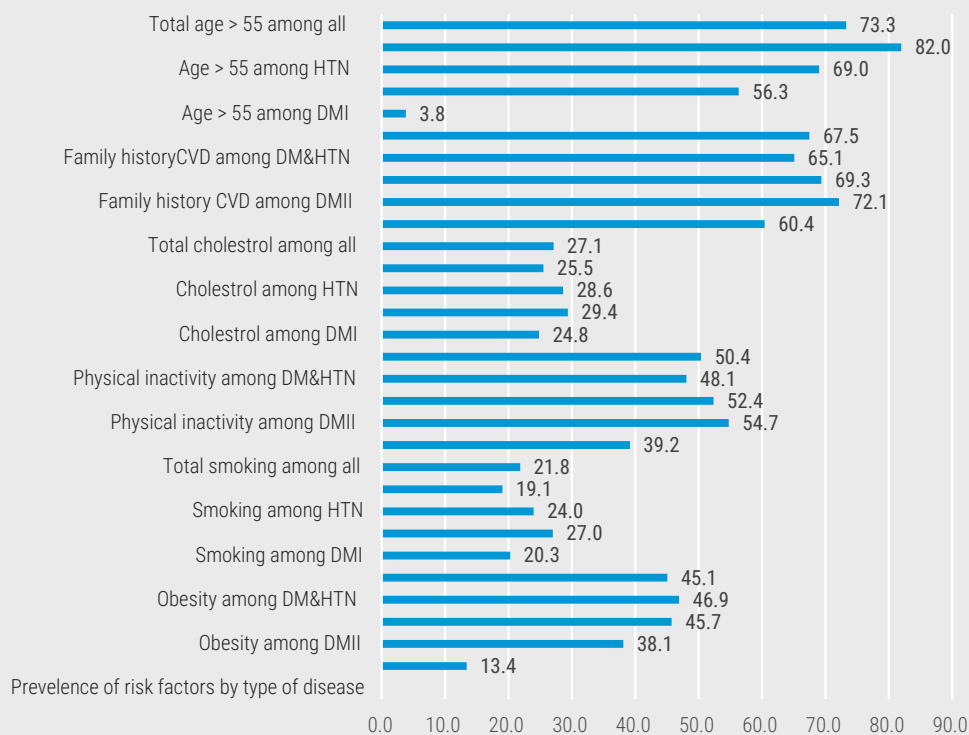


Figure 32: Prevalence (%) of risk factors among patients with noncommunicable diseases by disease type in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.

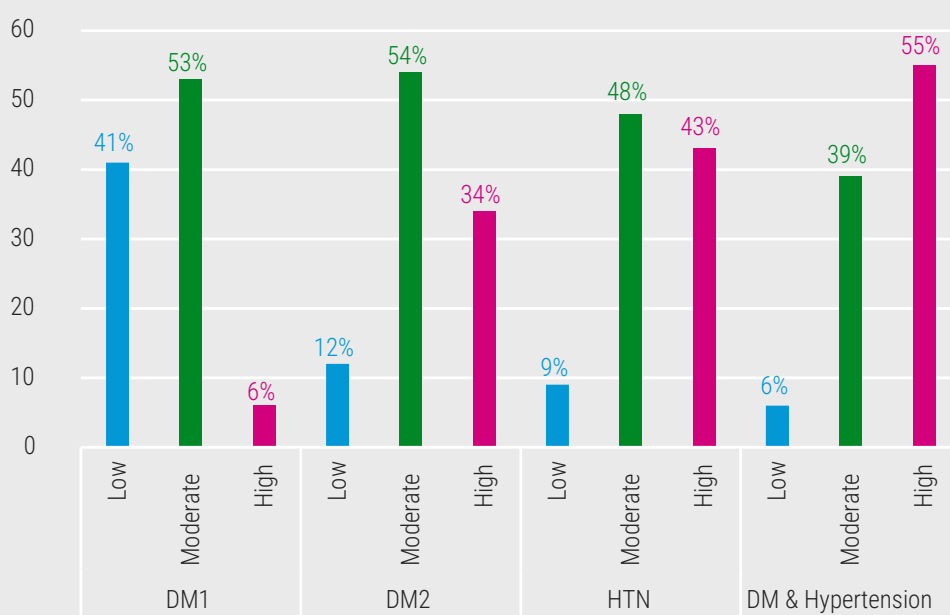


Figure 33: Distribution of NCD patients by their risk score in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.

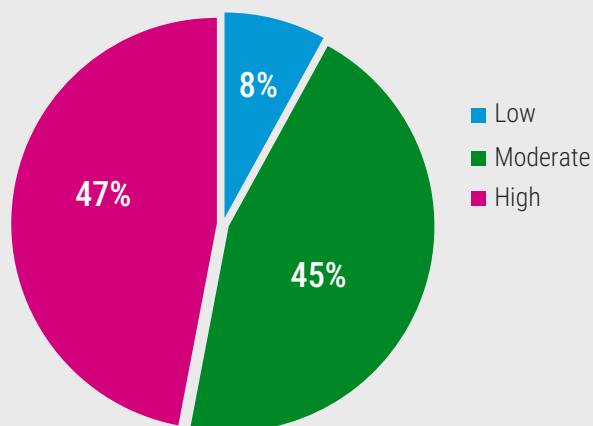


Figure 34: Distribution of overall noncommunicable disease patients by their risk score in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.

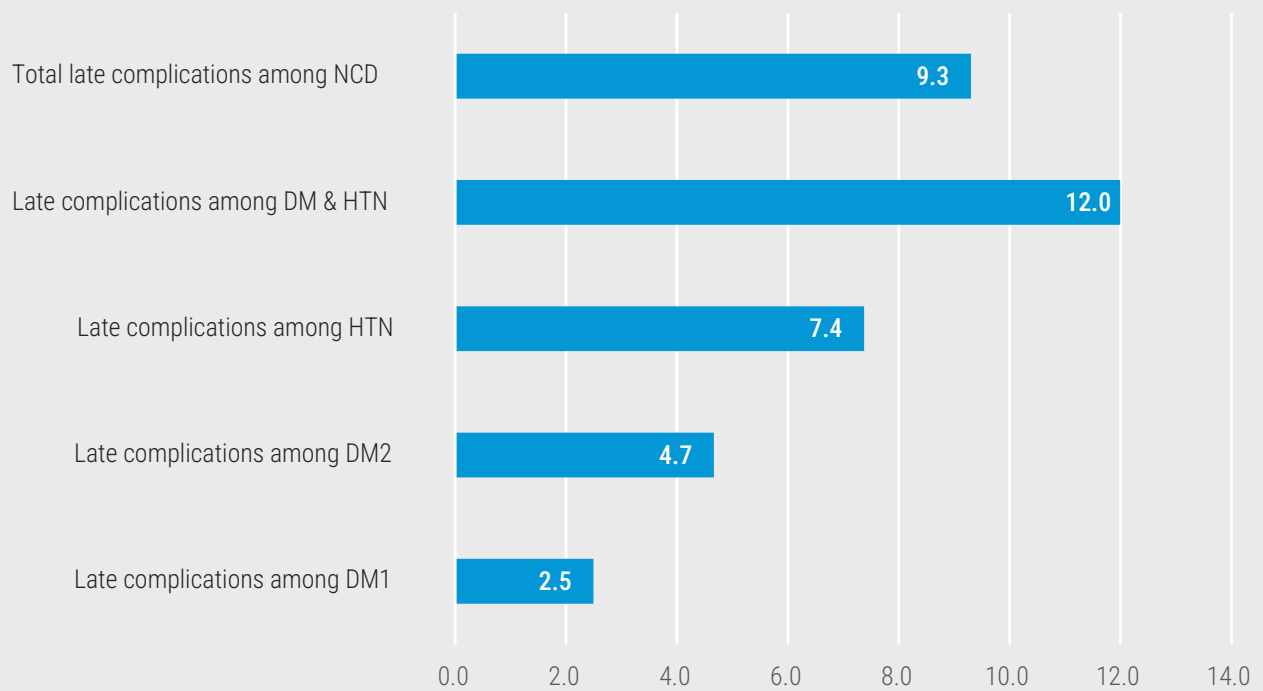


Figure 35: Prevalence (%) of late complications among patients with Diabetes Mellitus and Hypertension in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.

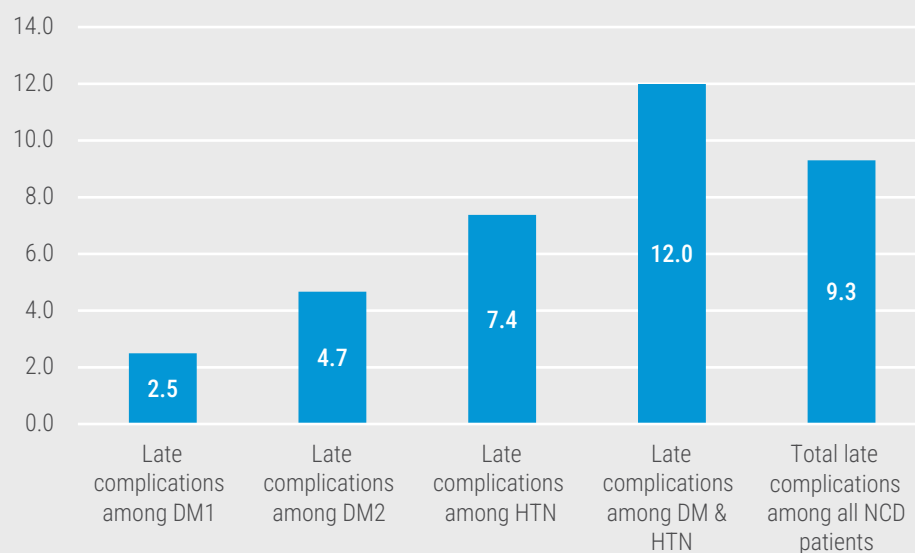


Figure 36: Prevalence (%) of late complications among noncommunicable disease patients by morbidity in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.

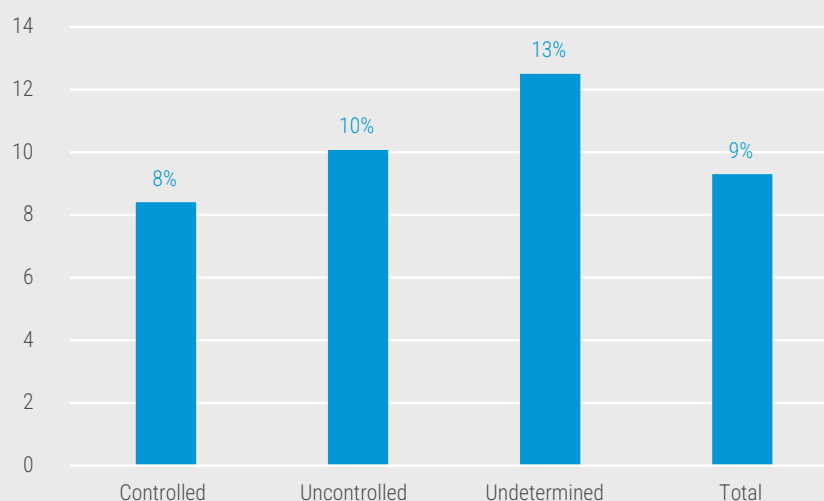


Figure 37: Prevalence of late complications among noncommunicable disease patients by control status in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.

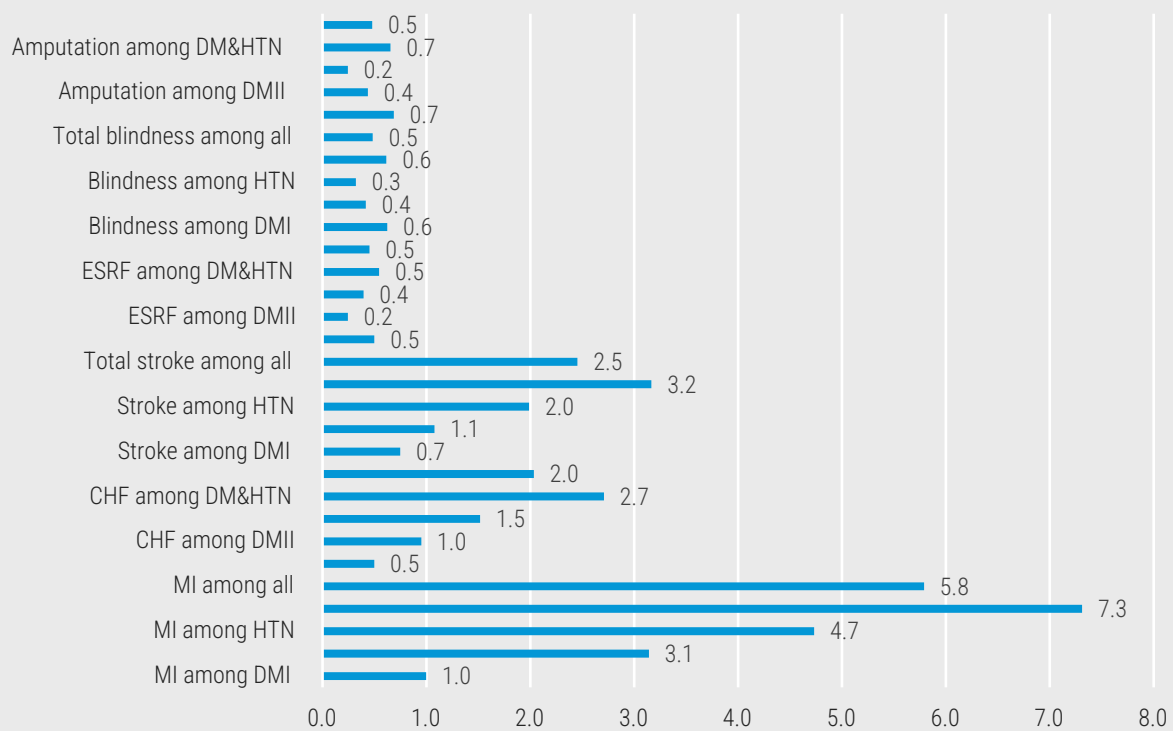


Figure 38: Distribution (%) of types of late complications among patients with noncommunicable diseases in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025

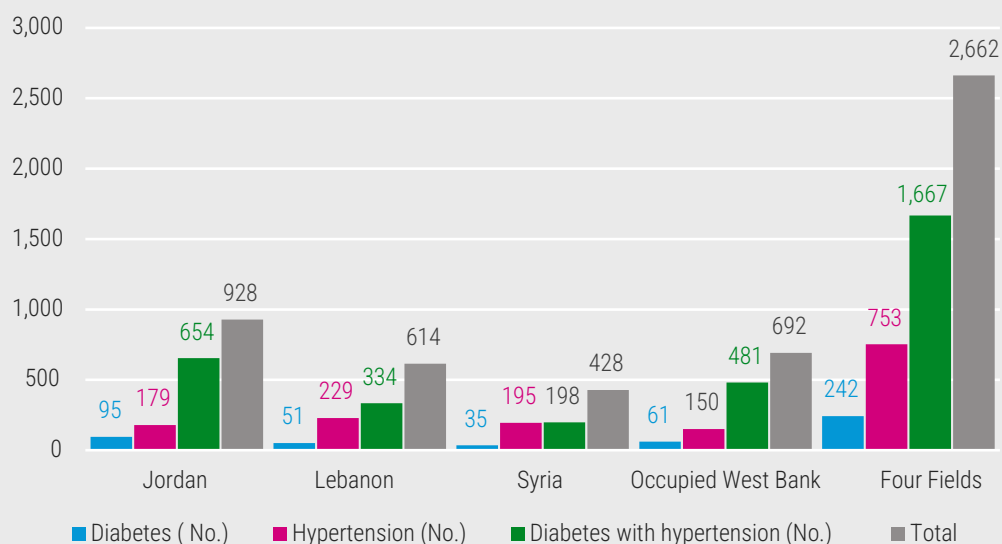


Figure 39: Number of disease-specific deaths in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.

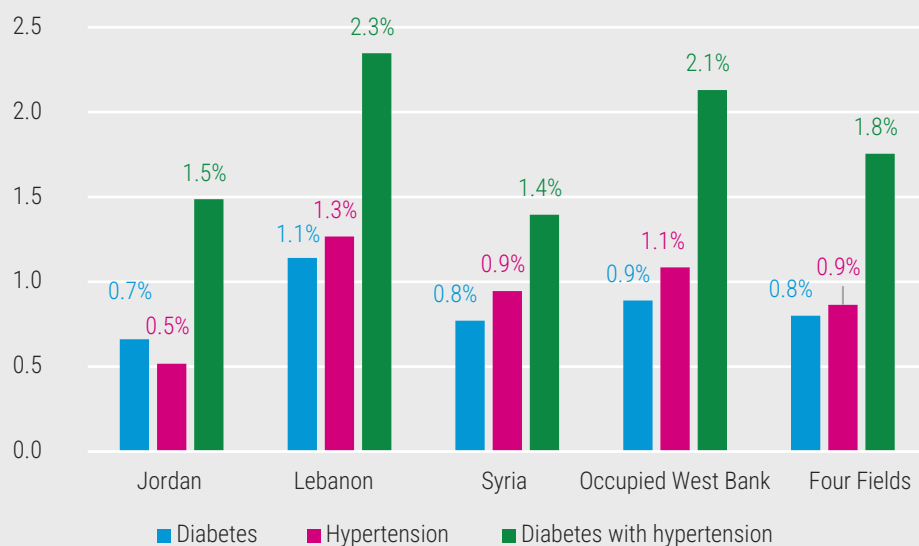


Figure 40: Distribution of disease-specific mortality in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.

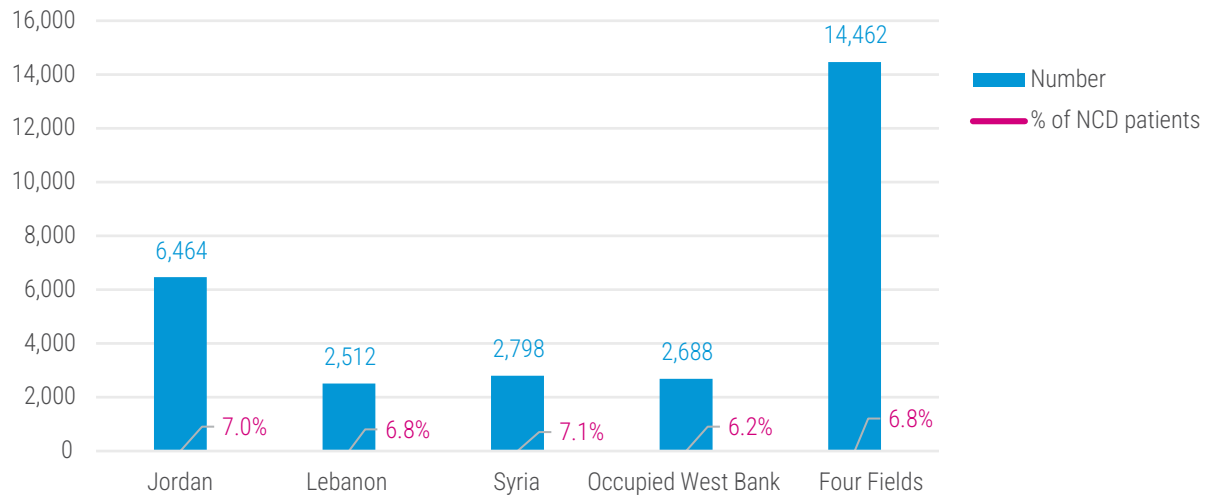


Figure 41: Distribution of NCD defaulters in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025. (For full NCD risk status report please see Annex 11), (Please see more details about the NCD risk scoring in Annex 12).

Prevention of Diabetic Foot and Retinopathy among Palestine refugees in Syria and Lebanon (World Diabetes Foundation (WDF) Project)

During 2025, the three-year World Diabetes Foundation (WDF) project, which began in October 2023 and targets Palestine Refugees in Syria and Lebanon, expanded its coverage by equipping an additional 21 health centres with the necessary furniture and medical equipment, thereby ensuring that all 52 targeted health centres are now fully supported under the initiative. Capacity-building efforts were strengthened through the completion of in-service training on diabetic retinopathy in both Lebanon and Syria, benefiting a total of 50 medical officers and 80 nurses. In parallel, on-the-job training on diabetic foot prevention was sustained across all 52 health centres, reinforcing clinical competencies and standardising care practices.

Procurement activities were also completed during the year, including the acquisition of 76 monofilaments and 48-foot Doppler devices to enhance screening and diagnostic capacity. To support patient awareness and self-care,

approximately 70,000 health education materials on diabetic retinopathy and diabetic foot prevention were produced and distributed. Screening activities yielded significant outputs, with 9,768 patients in Lebanon and 7,730 patients in Syria assessed for diabetic foot complications. Approximately 16 per cent of them were identified as having abnormalities and were subsequently provided with appropriate health education, clinical interventions, and referrals, as needed. Furthermore, screening for diabetic retinopathy was initiated using portable fundoscopic cameras, implemented on a rotational basis across the targeted facilities.

Throughout the reporting period, project monitoring was maintained through regular online meetings and supervisory field visits, ensuring continuous oversight and quality assurance. Updates on progress and achievements were systematically compiled and communicated to the donor.



Patient education and awareness materials for prevention of diabetic foot and retinopathy.



A Palestine Refugee patient undergoing a retinopathy examination by an UNRWA medical officer at Latakia Health Centre, Syria. ©2024 UNRWA photo by Taghreed Mohammad.

Mental health

UNRWA's Mental Health and Psychosocial Support (MHPSS) services are a core component of its primary health care approach, particularly in contexts of protracted conflict, displacement, and socioeconomic hardship affecting Palestine Refugees. Integrated within the Family Health Team (FHT) model, MHPSS is embedded into routine primary care to enable early identification, referral, and management of common mental health conditions, while addressing psychosocial stressors linked to conflict and poverty.

In 2025, despite severe disruptions, especially in the Gaza Strip and the northern occupied West Bank, UNRWA sustained MHPSS delivery through 123 health centres across the four

fields, excluding the Gaza Strip, and adapted services in high-intensity settings via shelters, mobile teams, and remaining operational facilities. In the Gaza Strip, services continued under extreme conditions through emergency shelters and medical points serving internally displaced populations, including psychological first aid, basic counselling, and targeted support for vulnerable groups. Aligned with WHO's mhGAP framework, UNRWA's approach emphasises task-shifting and capacity-building among non-specialist providers, reflecting a resilient and adaptive model that ensures continuity and accessibility of mental healthcare within primary health systems.



Positive MHPSS cases identified through the GHQ-12 screening tool

11.7%

Total (Four fields excluding the Gaza Strip)

19.2%

**Persons
with disabilities**

11.9%

Females

10.4%

Males



Individuals identified with MHPSS needs provided with assistance

98.7%

Total (Four fields excluding the Gaza Strip)

99.7%

**Persons
with disabilities**

98.7%

Females

98.5%

Males

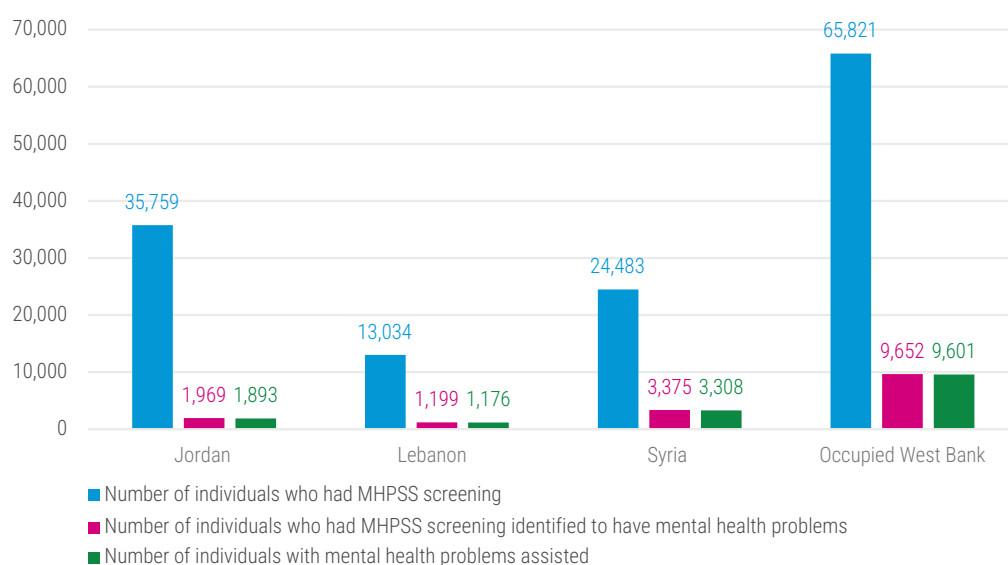


Figure 42: Number of individuals screened, identified, and assisted with general health questionnaire-12 (GHQ-12) screening and mental health and psychosocial support programme, in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.

Table 3: Prevalence of MHPSS positive cases (GHQ-12) and service coverage among identified cases by sex, disability status in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.

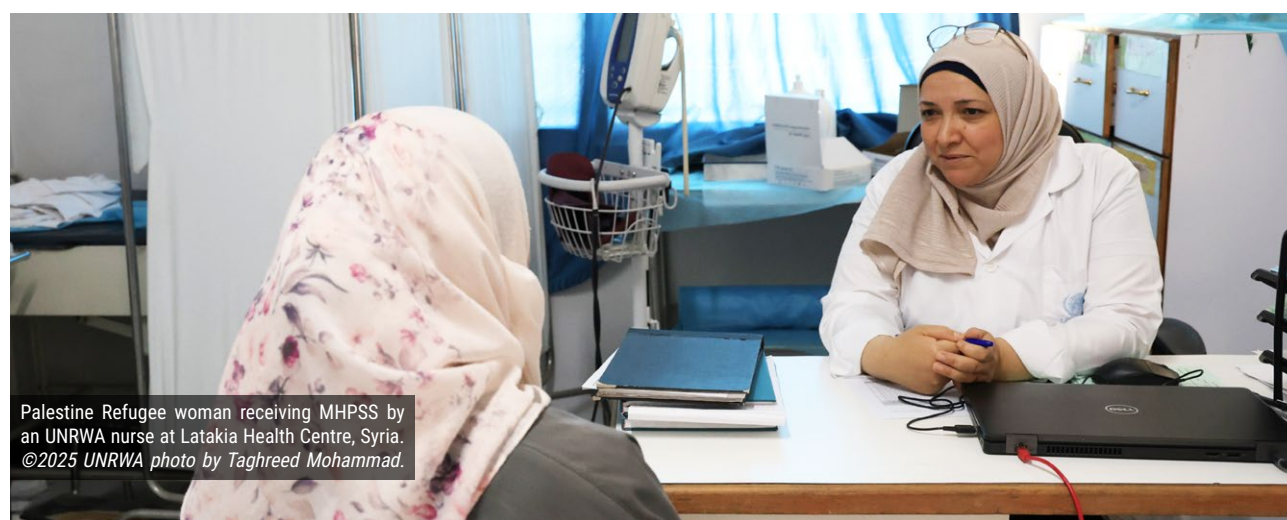
	Percentage of MHPSS positive cases identified through screening (GHQ-12) screening tool				Percentage of MHPSS positive cases assisted			
	Male	Female	Total	Disability	Male	Female	Total	Disability
Jordan	5%	6%	6%	11%	96%	96%	96%	100%
Lebanon	10%	9%	9%	16%	98%	98%	98%	100%
Syria	12%	14%	14%	26%	99%	98%	98%	100%
Occupied West Bank	13%	15%	15%	21%	99%	100%	99%	100%
Agency-wide	10%	12%	12%	19%	98%	99%	99%	100%

(Please see the detailed mental health report in Annex 13.)

(For more information about the mental health screening process please see Annex 14)

Table 4: Identified mental health gap problems in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.

Field	Depression			Epilepsy			Psychosis			Dementia		
	M	F	T	M	F	T	M	F	T	M	F	T
Jordan	272	1447	1719	8	9	17	10	20	30	2	4	6
Lebanon	147	442	589	64	42	106	34	27	61	0	3	3
Syria	83	457	540	12	12	24	27	17	44	0	0	0
Occupied West Bank	121	695	816	24	33	57	8	13	21	0	4	4
Total	623	3041	3664	108	96	204	79	77	156	2	11	13





outcome 3: improved status of communicable diseases and infectious disease prevention and control

Infectious disease surveillance

Infectious disease surveillance is a core public health function that enables the timely detection, reporting, analysis, and response to communicable diseases of public health importance. It supports early outbreak detection, disease trend monitoring, targeted prevention and control measures, and protection of population health through evidence-based action. UNRWA maintains an infectious disease surveillance system supported by its electronic platform, with a weekly reporting system and an infectious disease dashboard.

UNRWA continues its cooperation with host authorities and

WHO and participates in immunisation campaigns across all fields where required. UNRWA's focus on strengthening surveillance of emerging and re-emerging diseases remains active. Close coordination with the host-country Ministries of Health continues, including surveillance of communicable diseases, outbreak investigation and response, supply of vaccines, and exchange of information.

(Please see the full infectious diseases report in Annex 15).

(More details about infectious diseases surveillance are found in Annex 16)

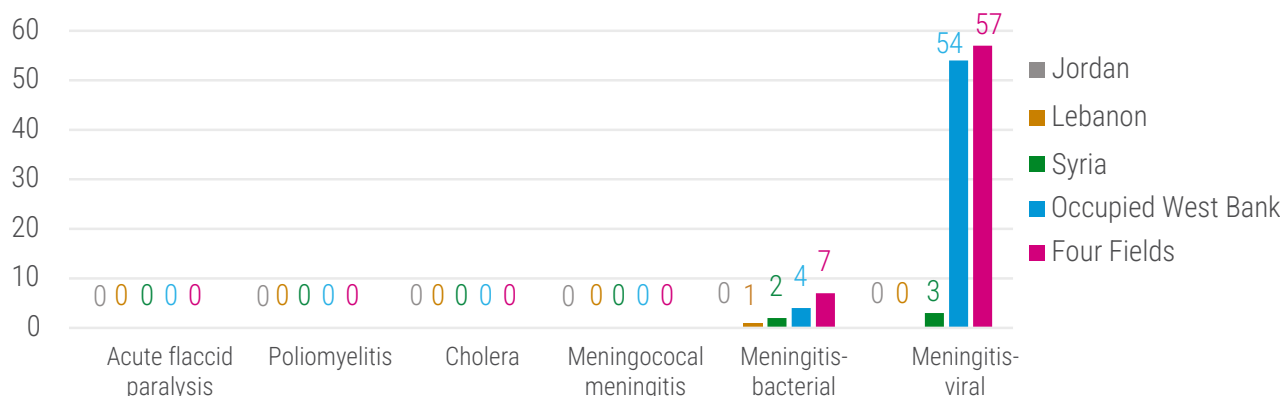


Figure 43: Number of expanded programmes on immunisation (EPI) targeted disease cases in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.

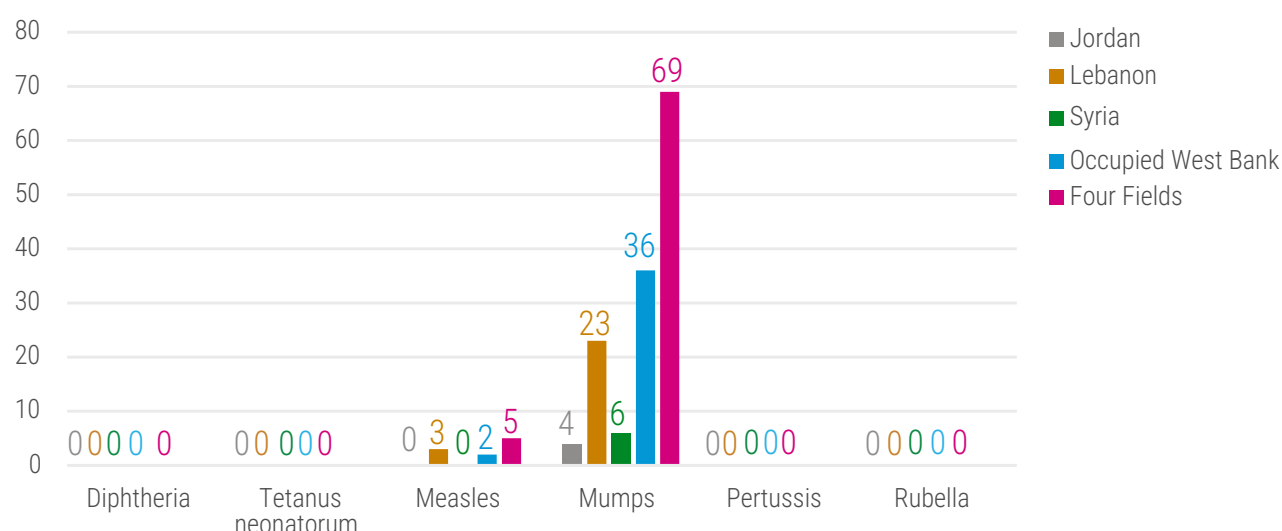


Figure 44: Number of expanded programmes on immunisation (EPI) targeted disease cases in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.

Immunisation

Child immunisation remains the most effective primary prevention method. UNRWA health services provide immunisation against several vaccine-preventable diseases in alignment with host country national vaccination programmes. UNRWA offers vaccines for tetanus-diphtheria (DT/Td), pertussis, tuberculosis (TB), measles, rubella, mumps, polio, haemophilus influenza type B (Hib), and hepatitis B across all fields. Additionally, the rotavirus vaccine is provided in all fields except Syria, while the pneumococcal vaccine is available in the West Bank, the Gaza Strip, and Lebanon, and was added to the national immunisation programme in Jordan in September 2025. Since 2020, UNRWA has also introduced the Hepatitis A vaccine in Jordan as part of the country's national vaccination programme. In 2025, the occupied

West Bank experienced a shortage of BCG, MMR, OPV, and Rota vaccines. Immunisation coverage varied across fields, ranging from 96.5 per cent in the occupied West Bank to 99.9 per cent in Jordan, bringing Agency-wide coverage, excluding the Gaza Strip, for the aforementioned diseases to 98.4 per cent for children aged 12 months, which is above the WHO-referenced target for high population immunity. Coverage for booster doses among children aged 18 months was 97.7 per cent. Vitamin A coverage remained high at 97.7 per cent. The use of the e-MCH application for mothers has played a key role in maintaining high vaccination coverage. This digital tool sends timely reminders to mothers based on each child's vaccination schedule, helping reduce the number of missed doses and minimising the need for nurse follow-ups.

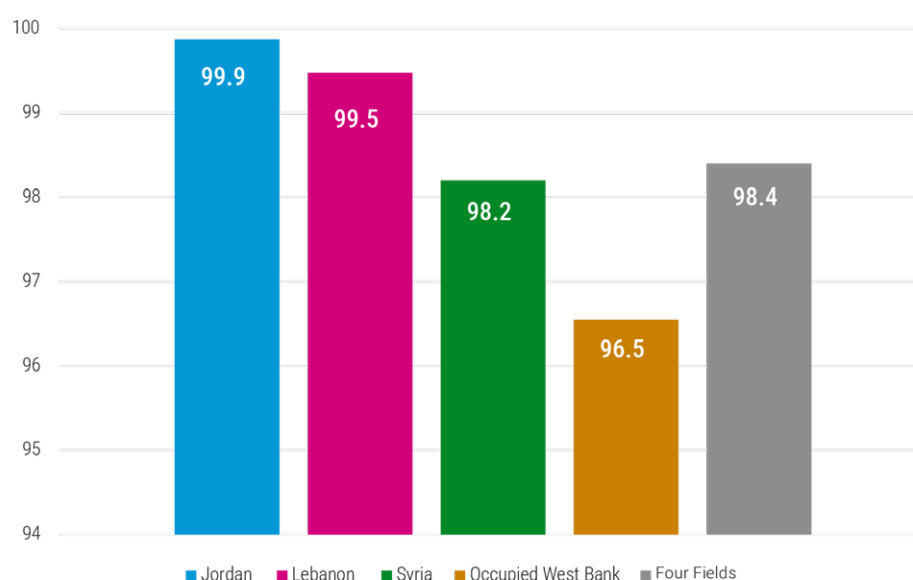


Figure 45: Vaccination coverage (%) of infants (<1 year) in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.

(Please see Annex 17 for detailed numbers).

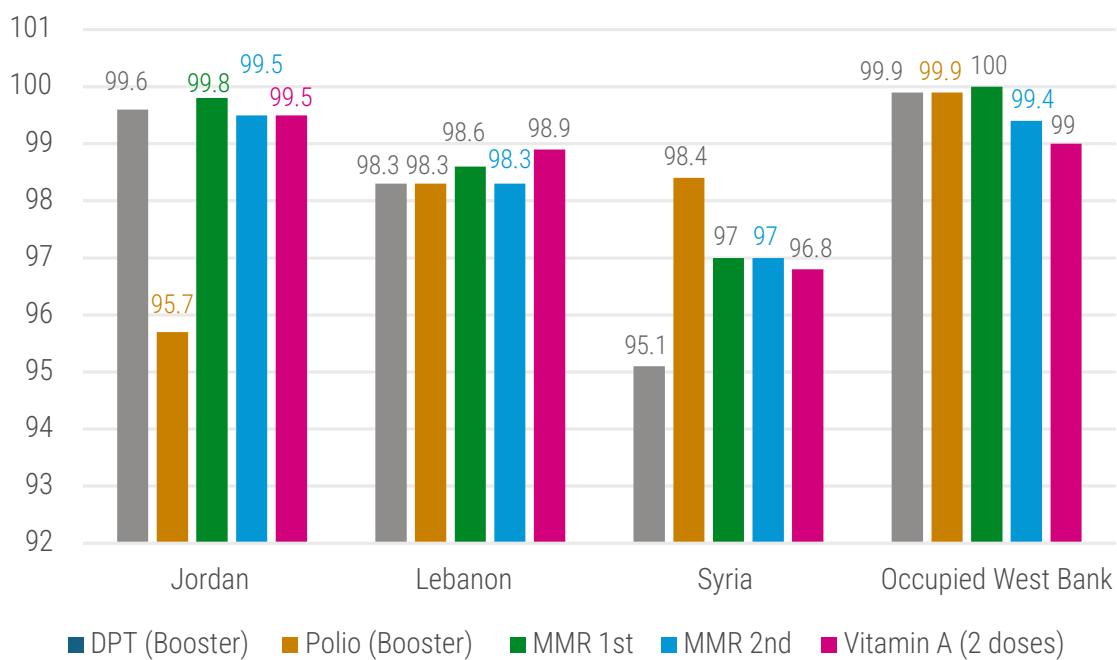
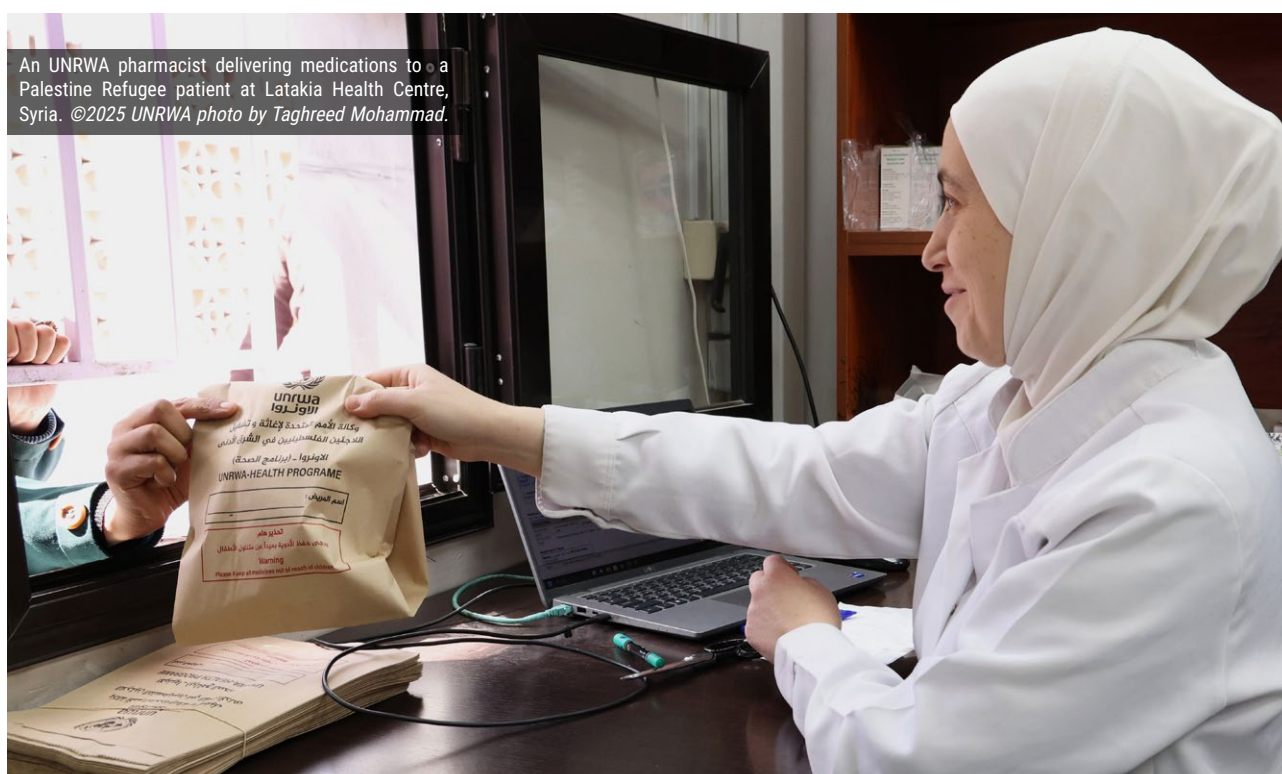


Figure 46: Coverage (%) of vaccine booster doses and vitamin A in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025.

(Please see Annex 18 for detailed numbers).



A Palestine Refugee baby receiving child healthcare services from an UNRWA nurse in Nuseirat Health Centre, the Gaza Strip. ©2025 UNRWA photo.



An UNRWA pharmacist delivering medications to a Palestine Refugee patient at Latakia Health Centre, Syria. ©2025 UNRWA photo by Taghreed Mohammad.

outcome 4: palestine refugees access quality and safe health services

Pharmaceutical services

In 2025, UNRWA continued to provide essential medicines and medical supplies to Palestine Refugees across all fields of operation, maintaining a central role in supporting primary healthcare services despite a highly constrained environment. Through adaptive supply chain management and strong partnerships, the availability of medicines remained relatively stable across several fields.

Relatively stable supply conditions were maintained in Jordan and Lebanon, where no prolonged stockouts of critical medicines were reported. This was supported by stable procurement channels and effective coordination mechanisms. In Lebanon, this outcome was achieved despite a deteriorating economic context that continues to place pressure on health systems. In Syria, improvements were observed following the introduction of more efficient regulatory processes in the host country, which significantly reduced approval timelines and enabled faster clearance of medical supplies.

Partnerships remained a cornerstone of pharmaceutical service delivery throughout the year. Contributions from WHO, UNICEF, UNFPA, and other humanitarian partners supported the provision of essential medicines, vaccines,

micronutrients, and reproductive health commodities. In particular, WHO support played a critical role in sustaining medicine availability, through the procurement of substantial volume of essential medicines that were procured and distributed across multiple fields of operation in 2025.

In parallel, supply optimisation measures were implemented across the Agency, including the redistribution of medicines that could not enter the Gaza Strip before expiry to other fields of operation, in line with operational guidance and programme needs. This approach helped mitigate losses associated with delivery delays and supported the planned requirements for 2026 particularly in Syria, Lebanon, and Jordan.

Overall, these combined efforts contributed to maintaining a generally stable stock status across most fields, with minimal and short-term stockouts of critical medicines reported.

The most severe disruptions were observed in the Gaza Strip, where the restrictions affecting crossings and the entry of supplies led to widespread shortages of medicines and medical supplies. In some months, up to 58 per cent of essential medicines were out of stock, alongside significant

shortages in laboratories, dental, and other medical supplies. Delays in the entry of shipments resulted in the accumulation of medical supplies outside the Gaza Strip, including in Egypt and Jordan, where some items were damaged or expired while needs inside the Gaza Strip remained critical.

During the temporary ceasefire in early 2025, UNRWA was able to facilitate the entry of approximately 520 pallets of medical supplies into the Gaza Strip. However, the resumption of access restrictions led to renewed stockouts, including shortages of insulin and other life-saving medicines, requiring emergency support from WHO and other partners. By December 2025, WHO deliveries of approximately 720 pallets of medicines significantly improved the supply situation, reducing stockouts to around 27 per cent of essential medicines and restoring partial availability across key treatment categories.

For the way forward, UNRWA will continue to strengthen its pharmaceutical services with a greater focus on improving clinical outcomes and cost-efficiency, particularly in the management of NCD. A key priority will be the gradual introduction of clinically appropriate newer-generation medicines for NCDs, subject to clinical guidance, procurement feasibility and available funding, especially for Type 2 Diabetes Mellitus (T2DM), that offer improved glycemic control while remaining cost-effective within the Agency's resource-constrained context.

Training

UNRWA recognises the importance of providing continuous professional development for all health staff working in UNRWA health centres. Ongoing training not only strengthens professional competencies but also ensures the maintenance and continuous improvement of quality healthcare services provided to Palestine Refugees. In

collaboration with the Jordanian Nursing Council (JNC), two specialised training programmes were developed and implemented.

The Professional Diploma Programme in Primary Healthcare and Reproductive Health for Staff Nurses and Midwives is a 10-month programme designed to strengthen the capacity of primary healthcare nurses and midwives in addressing key public health priorities. The programme adopts a blended learning approach that combines online learning with on-the-job training, enabling participants to integrate theoretical knowledge with practical experience in their workplace. It specifically targets registered nurses and midwives and aims to enhance their competencies in surveillance, prevention, and control of priority public health issues within primary healthcare systems. The programme is accredited by Al-Hussein Bin Talal University and approved by the Ministry of Higher Education and Scientific Research. To support the delivery of this programme, the JNC developed comprehensive training materials tailored for primary healthcare providers, ensuring that the curriculum aligns with national standards and reflects evidence-based practice.

In parallel, the Online Courses Programme in Primary Health Care and Reproductive Health for Practical Nurses offers a six-month learning opportunity specifically designed for practical nurses. Delivered entirely online, the programme aims to provide participants with a deeper understanding of evidence-based and translational primary healthcare practices. Through structured online modules, the programme supports practical nurses in strengthening their theoretical knowledge and enhancing their ability to apply contemporary primary healthcare principles in their professional practice.

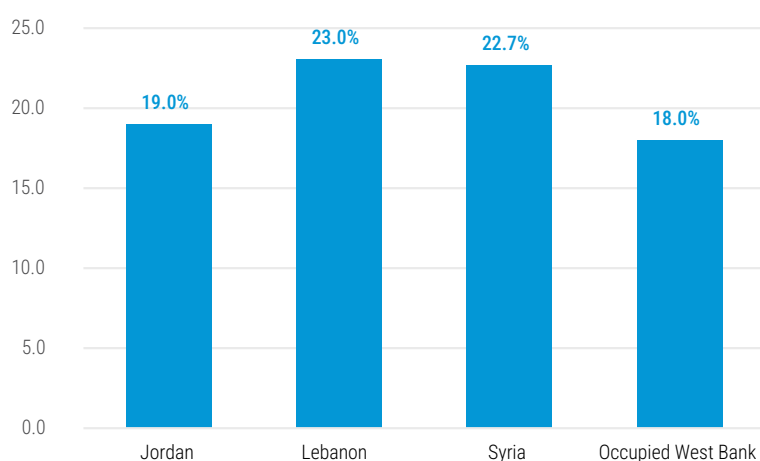


Figure 47: Antibiotic prescription rate (%) in Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem, in 2025. (Please see Annex 19 for detailed numbers).

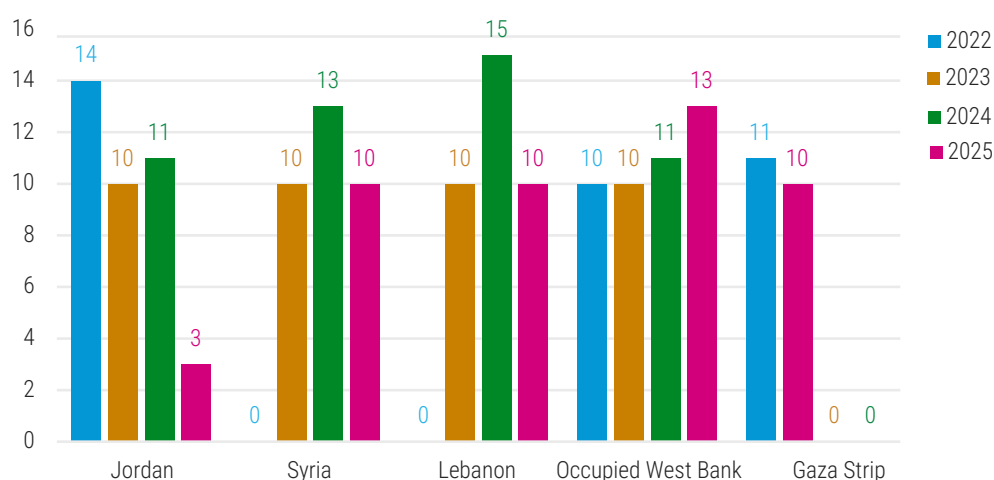


Figure 48: Number of healthcare workers who earned the professional diploma (course for primary care nurses/midwives) across UNRWA's five fields of operation, from 2022 to 2025.

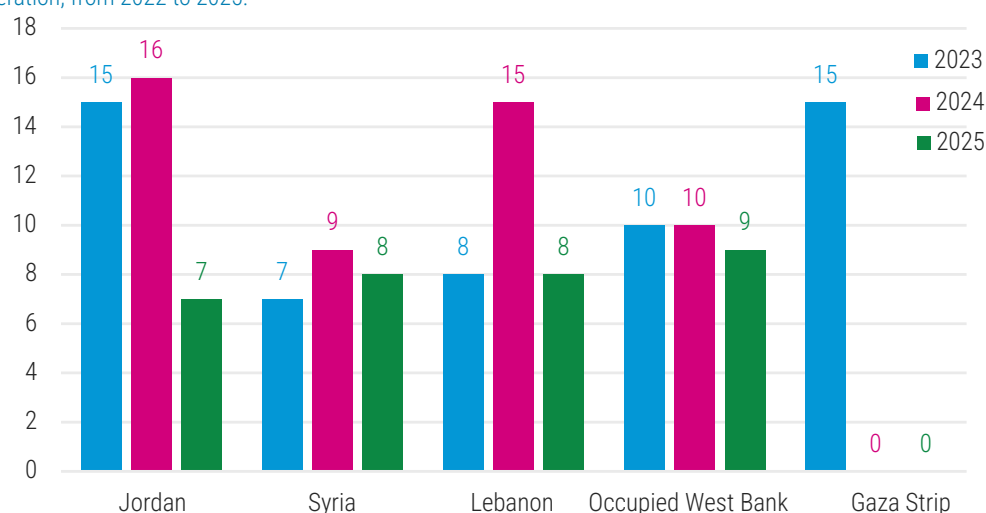


Figure 49: Number of practical nurses who completed the online course in primary healthcare and reproductive health across UNRWA's five fields of operation, from 2023 to 2025

Hospitalisation

Introduction

In 2025, UNRWA facilitated access to secondary and tertiary hospital care for Palestine Refugees who would have otherwise faced prohibitive financial barriers. Support is provided based on medical urgency, availability of alternative services, and economic vulnerability. Cases are prioritised based on clinical severity and household vulnerability to ensure that limited resources reach those most in need. These services complement Primary Health Care (PHC) services provided at UNRWA health centres. The Hospitalisation Support Programme (HSP) is designed to alleviate overwhelming health expenditures and support access to quality care, especially when local systems are under strain. Through strategic partnerships with contracted hospitals and robust support mechanisms for urgent cases, our efforts are dedicated to protecting human dignity and preserving lives. UNRWA's HSP facilitates access of Palestine Refugees to critical secondary and tertiary healthcare services across the Gaza Strip (during the first quarter of 2025 only), the occupied West Bank, Syria, Lebanon, and Jordan. Despite challenges in 2025, including

conflict and financial constraints, our interventions enabled timely referrals for lifesaving treatments, safe deliveries, medically urgent cases, and essential outpatient services such as care for cancer patients.

The programme provides comprehensive secondary, tertiary, and mental health services, including emergency care, surgeries, intensive care, complex treatments, such as cardiovascular, neurological, paediatric, and oncological, as well as specialised outpatient services not available at UNRWA health centres. Equity is maintained by prioritising vulnerable groups, including Social Safety Net Programme (SSNP) beneficiaries, ex-Gazans, and patients without alternative insurance coverage. Services are delivered effectively through contracted hospitals selected for quality, accessibility, and affordability, while accountability is ensured through monitoring of referrals, claims, expenditures, annual audits, patient satisfaction surveys, and adherence to technical instructions.

Services are delivered through governmental, private, and PRCS hospitals, with pricing and reimbursement agreements ensuring predictable cost coverage. Cost-sharing varies by field and patient category. For instance, in Lebanon, secondary care coverage ranges from 90 to 100 per cent of hospital invoices, tertiary care 60 per cent, and outpatient services 50 per cent, with additional support for SSNP patients and catastrophic cases through the Medical Hardship Fund (MHF). The Hospitalisation Management System supports comprehensive tracking of patients, expenditures, and outcomes. This support includes quarterly and annual reporting to analyse trends and inform programmatic decisions.

Programme outcomes

The Hospitalisation Support Programme (HSP) has led to improvements in access to care and is expected to support health outcomes across all fields. In terms of health outcomes, the programme has contributed to reducing complications among patients with chronic diseases, improving maternal and neonatal outcomes for women delivering in supported hospitals, and enabling early treatment and continuity of care for cancer patients and other vulnerable populations. The programme also improved access to hospitals that would otherwise be out of reach due to financial or geographic barriers, ensuring timely care for critical conditions. Additionally, coverage under the Social Safety Net Programme (SSNP) and for other high-risk groups has been maintained, strengthening equitable and prioritised access to services.

Inpatient hospitalization

In 2025, 84,189 patients received hospitalisation support across UNRWA's five fields of operation, in addition to 6,553 patients admitted to UNRWA's Qalqilya hospital in the occupied West Bank, compared with 104,008 patients in 2024. This 19.1 per cent decrease was primarily due to the suspension of hospitalisation services in the Gaza Strip from

April 2025, which substantially reduced the overall number of supported hospitalisations (by at least 25,077 patients). Excluding the Gaza Strip, hospitalisations in the remaining fields, Jordan, Lebanon, Syria, and the occupied West Bank, increased from 75,016 to 80,274 patients, an approximate 7 per cent year-on-year increase.

Table 5: Annual trend of reported cases across UNRWA's five fields of operation, from 2021 to 2025

Year	Jordan	Lebanon	Syria	Gaza Strip	Occupied West ank	Agency
2021	2,470	19,729	15,855	14,502	20,708*	73,264*
2022	2,310	23,861	16,110	14,743	20,720*	77,744*
2023	5,371	25,113	24,702	7,872**	18,634*	81,692*
2024	3,218	22,333	23,347	28,992	19,612*	97,502*
2025	4,342	25,091	24,691	3,915	26,150*	90,742

*Exclude patients admitted to Qalqilya Hospital;

**Until the end of September 2023

In Jordan, hospital admissions increased from 3,218 in 2024 to 4,342 in 2025, representing a 34.9 per cent increase, indicating both growing demand and improved reporting. In Lebanon, admissions rose from 22,333 to 25,091, a 12.3 per cent increase, reflecting stable demand for chronic and specialised care. In Syria, there was a modest increase from 23,347 to 24,691, representing a 5.8 per cent rise, indicating continued reliance on UNRWA for secondary and tertiary care amid ongoing economic challenges. In the Gaza Strip, hospitalisations declined sharply from 28,992 to 3,915, an 86.5 per cent decrease, following the suspension of services after Q1. In the occupied West Bank, admissions rose from 19,612 to 26,150, an increase of 33.3 per cent, demonstrating consistent use of referral pathways and increased demand for services outside Qalqilya Hospital. Overall, agency-wide hospitalisations decreased by 7 per cent, from 97,502 to 90,742, while all fields outside the Gaza Strip continued to show upward trends.

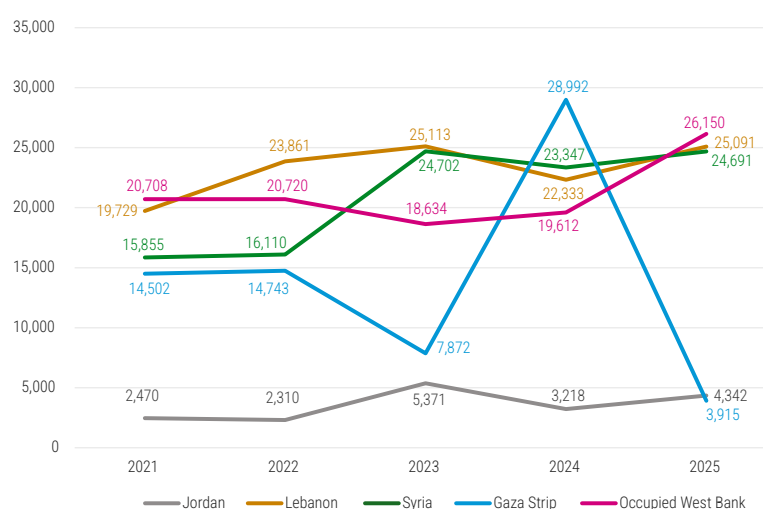


Figure 50: UNRWA inpatient hospitalisations across UNRWA's five fields of operation, from 2021 to 2025.

Gender Disaggregation: Female hospitalisations accounted for 54,419 in 2025, compared with 66,808 in 2024, reflecting an 18.5 per cent decrease, as most of the supported patients in the Gaza Strip were pregnant women. Male hospitalisations decreased to 36,323 in 2025 from 37,200 in 2024, showing a 2.3 per cent decrease overall. Field-specific female and male distributions follow similar trends to the total admissions, with the occupied West Bank showing a notable rise in male patients due to increased reliance on UNRWA's hospitalisation programme.

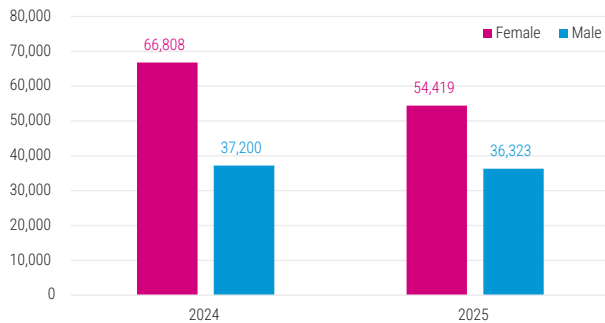


Figure 51: UNRWA inpatient hospitalisation by gender, across UNRWA's five fields of operation, from 2024 to 2025.

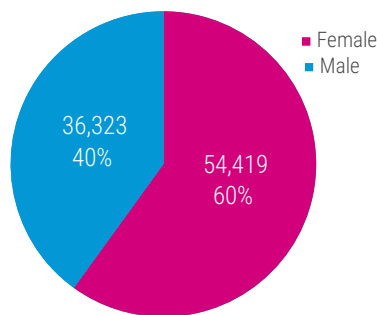


Figure 52: UNRWA inpatient hospitalisation by gender, across UNRWA's five fields of operation, in 2025.

Outpatient services and chronic disease support

In 2025, UNRWA's outpatient support focused on essential diagnostics, laboratory services, prosthetics, and cancer care. In Lebanon, there were 31,935 outpatient visits, with 35.79 per cent of patients covered under the SSNP. The majority of these visits supported cancer and chronic disease treatment, including medications beyond the essential drug list, such as those for thalassemia and multiple sclerosis. In Syria, 9,926 outpatient visits were recorded, with 27.87 per cent SSNP beneficiaries, primarily for essential investigations required for proper diagnosis that are not available within UNRWA primary healthcare (PHCs) or Ministry of Health facilities. In Jordan, patients accessed medications, laboratory services, diagnostics, and specialised care, with priority given to vulnerable groups. Services were carefully prioritised for life-saving and urgent interventions, while elective procedures were selectively limited to maintain financial sustainability.

UNRWA continued to provide targeted support for chronic and high-cost care through the Medical Hardship Fund (MHF). In Lebanon, support provided in 2025 included 76 patients with Thalassemia (38 females, 38 males), 70 patients with Multiple Sclerosis (47 females, 23 males), 98 patients with other non-catalogue conditions (47 females, 51 males), and 877 cancer patients receiving a 75 per cent subsidy (498 females, 379 males). Additionally, 360 new cancer cases were identified and 114 cancer deaths recorded, with 75 per cent reduction in reported mortality among supported cases following the increased coverage ceiling. Overall, the total number of supported patients reached 1,121 in 2025, compared with 922 in 2024, reflecting comprehensive MHF support.

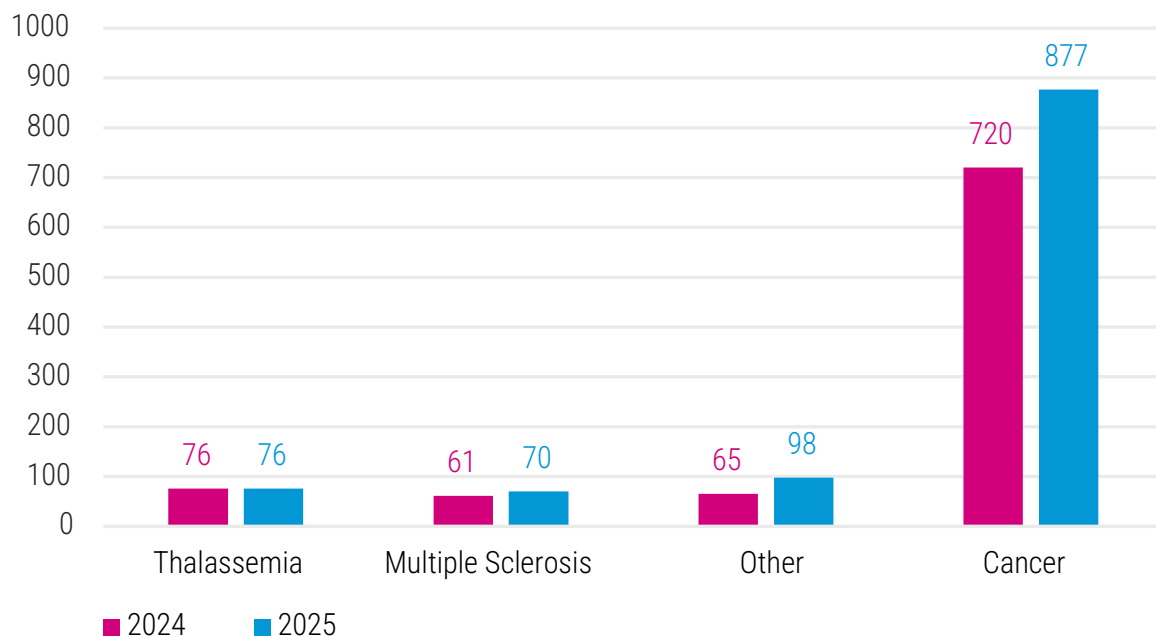


Figure 53: Chronic disease and cancer patient supported across UNRWA's five fields of operation, 2024 vs 2025.

UNRWA's MHF interventions ensured continuity of care through chemotherapy, radiotherapy, PET scans, and other high-cost interventions, significantly improving adherence and outcomes for chronic and cancer patients.

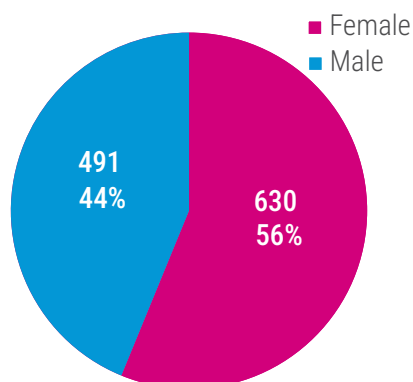


Figure 54: Chronic disease and cancer patient supported, disaggregated by gender across UNRWA's five fields of operation, in 2025.

Qalqilya hospital annual report – 2025

Qalqilya hospital continues to provide critical secondary healthcare services to Palestine Refugees in Qalqilya city and surrounding communities, reducing the financial and health barriers to hospital care. In 2025, the hospital supported 6,553 admitted patients, contributing to improved health outcomes for urgent, elective, and maternity cases, while also ensuring equitable access for vulnerable groups, including social cases and patients with very limited resources, referred by municipalities.

The hospital operated across surgical, maternity, paediatric, and medical wards, with dedicated staffing for each speciality. Bed management and monitoring were conducted daily, tracking occupancy, length of stay, and discharges to optimise resource use. Emergency readiness included additional ER and delivery room beds to handle patient surges. Outpatient services continued across all specialities, with a focus on paediatric and maternal care. Data management and reporting were conducted monthly, monitoring admissions, discharges, mortality, referrals, and outpatient visits to support planning and optimise patient flow.

Access and utilisation

The hospital maintained high access to inpatient care, with refugee patients constituting the majority of admissions (6,031), supplemented by targeted social cases and municipality referrals. Bed utilisation remained efficient, with an average bed occupancy rate of 61.7 per cent, compared with previous average daily bed occupancy of 35.8 per cent, and an average length of stay of 1.99 days. Services were delivered across surgical, medical, paediatric, and maternity wards, ensuring comprehensive speciality coverage.

Inputs and resources

UNRWA's hospitalisation programme operates through a coordinated mix of financial, human, and material resources to ensure equitable access and effective service delivery. Financial inputs include agency budget allocations, comprising programme budget and donor contributions, and Medical Hardship Fund (MHF) funding for catastrophic care, covering high-cost treatments such as cancer, Multiple Sclerosis, Thalassemia, and other non-catalogue medications. Human resources consist of medical officers, area health officers, administrative clerks, and hospitalisation monitoring and assessment officers (HMAO), supervised by the CFHP, responsible for daily oversight of admissions, invoice verification, data entry, and patient follow-up. Material resources include contracts with local hospitals, comprising governmental facilities, NGO facilities including PRCS, and private providers, as well as diagnostic equipment, medical supplies, prosthetics, chemotherapy, and radiotherapy.

Staff training and capacity building are conducted regularly for medical officers and hospitalisation staff, with standard operating procedures (SOPs) updated based on evolving field contexts. In Lebanon, SOPs were upgraded in line with findings and recommendations from an external assessment of the Hospitalisation Programme. These efforts, combined with the assessment of hospitalisation needs and upgrades to the Hospitalisation Management System (HMS), which is integrated with MHF electronic telemedicine medical files, enhance operational efficiency. Dashboards were introduced to monitor patient flow, resource utilisation, costs, and outcomes effectively.



A Palestine Refugee patient having vital signs recorded by an UNRWA nurse at the Seham medical point (shelter) in the Khan Younis area, the Gaza Strip. ©2025 UNRWA photo.

System strengthening

Programme monitoring has been strengthened through rigorous data management, regular audits, and patient satisfaction assessments, enhancing overall effectiveness. Health staff trained in Hospitalisation Support Programme (HSP) SOPs ensure adherence to admission criteria, accurate documentation, and efficient coordination with contracted hospitals. Real-time data and quarterly reports facilitate evidence-based decision-making and policy adjustments. Planned improvements include expanding preventive and early-intervention programmes for cancer and chronic NCDs, integrating services with primary health care to strengthen referral pathways, continuously assessing cost ceilings and co-payment structures to maintain financial sustainability, and enhancing supervision and audits to ensure clinical and financial integrity.

Monitoring

Monitoring and evaluation activities consist of monthly audits, review of referral pathways for both inpatient and outpatient services, identification of gaps in access for PRs, and ongoing data quality assessment using dashboard indicators for timely decision-making. Budget and resource allocation integrate funding for in-patient and specialised out-patient services, supported by programme budget projects and emergency funds, prioritising maternal care, chronic disease management, cancer care, and diagnostics.

Challenges

Key challenges include the suspension of services in the Gaza Strip, limited access to government hospitals in some fields, high demand for chronic and cancer care, and the need for coordination across multiple fields, such as Lebanon, the occupied West Bank and Syria.



Recommendations

Recommendations emphasise continued training and capacity-building for staff, expansion of inpatient and outpatient support for high-priority cases, strengthening dashboard monitoring and HMS reporting, and ensuring regular audits and follow-up on referrals.

Table 6: Distribution of ward admissions by specialty and total number of beds, across UNRWA's five fields of operation, in 2025

Ward	Surgical	Medical	OB/GYN	Pediatric	Total
Number of beds	14	15	14	15	58
Number of admitted patients	1,876	1,446	1,792	1,459	6,573

Demographically, the majority of admitted patients resided in Qalqilya City (5,315), with smaller numbers from surrounding villages (390), Tulkarem (268), Jenin (116), Nablus (252), and other areas in the occupied West Bank areas (212).

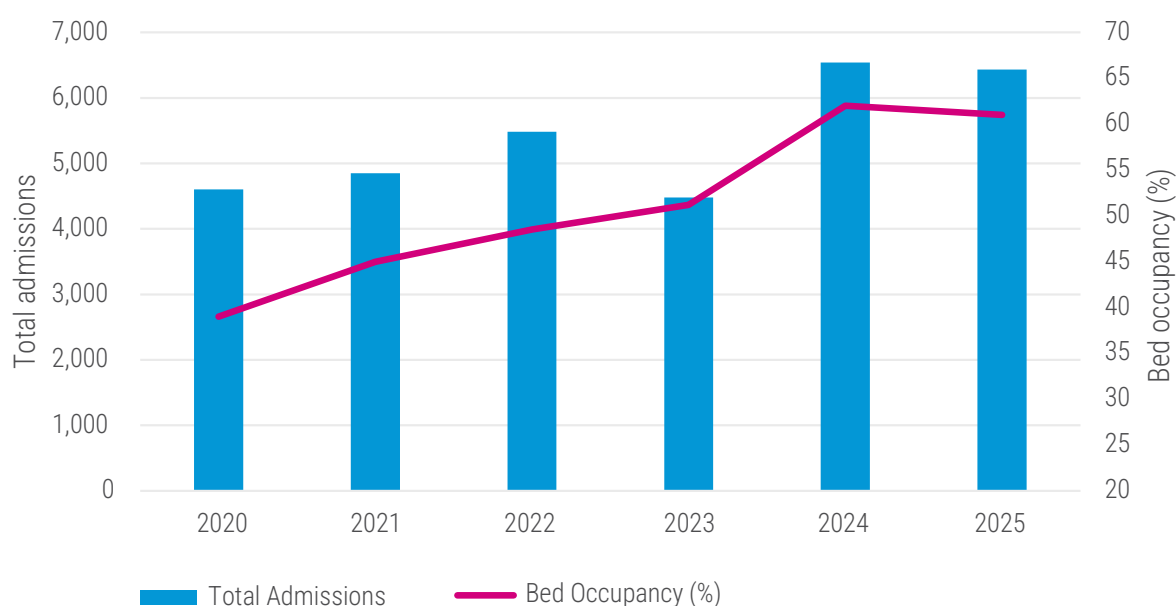


Figure 55: Total admissions and bed occupancy trends in Qalqilya Hospital in the occupied West Bank, across UNRWA's five fields of operation, from 2020 to 2025

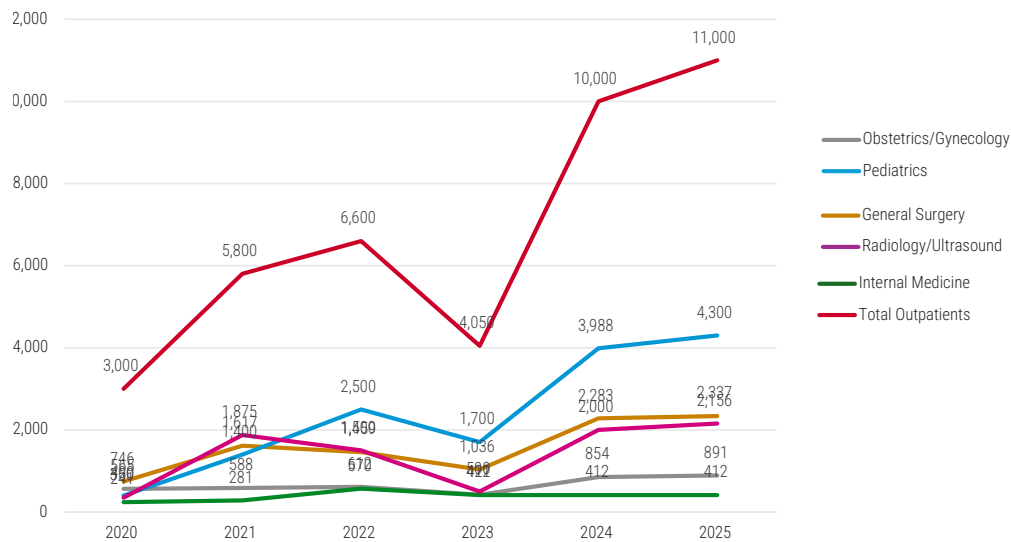


Figure 56: Outpatient growth (number of cases), by specialty across UNRWA's five fields of operation, from 2020 to 2025.

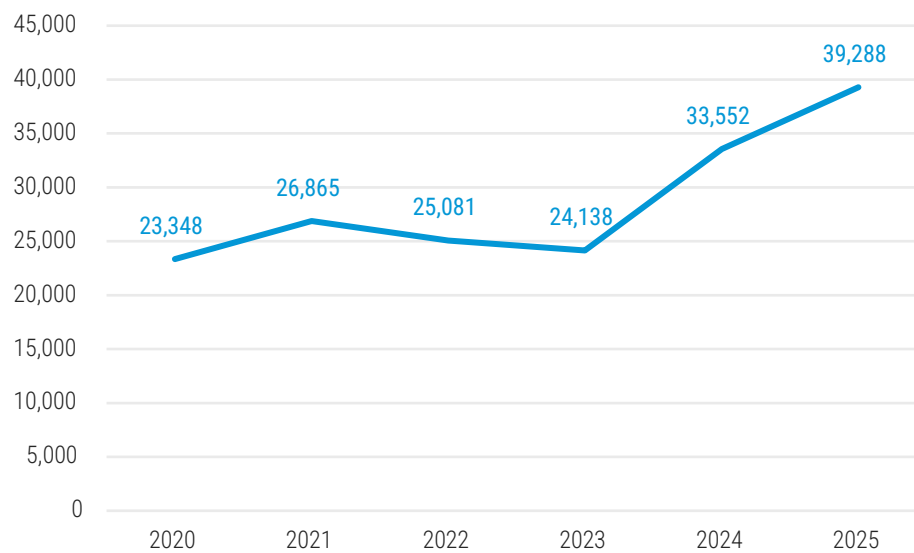


Figure 57: Total emergency room consultations across UNRWA's five fields of operation, from 2020 to 2025.

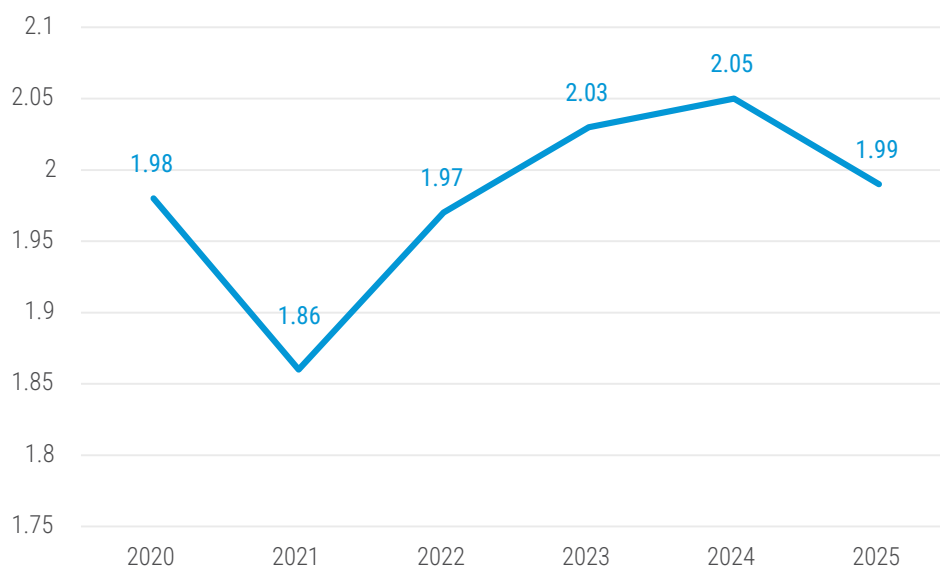


Figure 58: Average length of hospital stay (days) across UNRWA's five fields of operation, from 2020 to 2025.

Outpatients

UNRWA continued to provide comprehensive primary health care services across its five fields of operation, ensuring sustained access to essential healthcare for Palestine Refugees despite a highly constrained operating environment. Services were delivered through 123 health centres, including eight in the Gaza Strip, 25 in Jordan, 26 in Lebanon, 24 in Syria, and 40 in the occupied West Bank, including East Jerusalem.

UNRWA recorded 9,180,094 medical consultations across its five fields of operation last year, with 4,531,840 consultations in the Gaza Strip alone, and 4,648,254 consultations in the occupied West Bank, including East Jerusalem, Lebanon, Syria, and Jordan.

Across the four fields, excluding the Gaza Strip, face-to-face consultations reached 4,560,503, while telemedicine consultations reached 87,751. Jordan recorded the highest number of consultations among the four fields, with 1,586,940, followed by the occupied West Bank, including East Jerusalem, with 1,136,599, Lebanon with 970,630, and Syria with 954,085.

By sex, females accounted for most consultations across the four fields excluding the Gaza Strip. Face-to-face consultations included 1,711,117 male consultations and 2,833,386 female consultations, while telemedicine consultations included 33,122 male consultations and 54,629 female consultations.

The average daily medical consultations per doctor remained high across the four fields excluding the Gaza Strip. The occupied West Bank, including East Jerusalem, recorded the highest workload at 77.6 consultations per doctor per day, followed by Lebanon at 74.8, Jordan at 72.7, and Syria at 60.5. This continued to exceed the workload benchmark used in the report, reflecting sustained pressure on medical officers.

The average medical consultation time remained short across fields, reaching 2.26 minutes in Jordan, 2.43 minutes in Lebanon, 2.52 minutes in Syria, and 2.33 minutes in the occupied West Bank, including East Jerusalem. These figures reflect the ongoing operational pressure on outpatient services and the need to further strengthen workload management, appointment systems, and task sharing under the Family Health Team approach.

In terms of service utilisation patterns, the separate first, repeat, and specialist datasets showed 4,613,227 outpatient consultations across the four fields (excluding the Gaza Strip), including 1,113,620 first visits, 3,425,601 repeat visits, and 66,667 specialist consultations. The Agency-wide, excluding the Gaza Strip, repeat-to-first visit ratio was 3.1, indicating continued reliance on UNRWA services and sustained follow-up care.

Overall, outpatient care in 2025 demonstrated continuity and adaptability across all fields including resuming outpatient care in the Gaza Strip. (Please see Annex 20 for detailed numbers).



A Palestine Refugee patient receiving oral health services from an UNRWA dental surgeon at Zarqa Camp Health Centre in the Zarqa area, Jordan ©2025 UNRWA photo by Yasser Abdulfattah.

Oral and Supportive Health Services: Expanding access to quality care

UNRWA provides a comprehensive package of primary care supported by a range of diagnostic and supportive health services, including laboratory services, radiology, physiotherapy, and oral health. These services are essential for accurate diagnosis, effective treatment, and long-term health outcomes, ensuring that care is not only accessible but also safe, timely, and of good quality. Services are provided through a network of health centres located both inside and outside camps, in addition to mobile teams that reach populations in hard-to-access areas through both face-to-face consultations and telemedicine. This combined approach has been particularly important during crises, allowing patients to access care even when movement is restricted or health facilities are damaged.

Radiology services

Radiology services play a critical role in supporting diagnosis and guiding clinical decision-making. However, radiology capacity remains limited and is not available in all fields. In 2025, services were provided through functional units in Jordan (one unit), the occupied West Bank (five units), and Lebanon (four units), where X-ray procedures were conducted within UNRWA facilities. In the occupied West Bank, the number of operational units

decreased from eight due to conflict-related damage, reflecting the direct impact of instability on the availability of essential diagnostic services. In the Gaza Strip, seven radiology units were operational in 2023: however, all were reported damaged or non-functional following the escalation that began in October 2023.

Physical rehabilitation

Physiotherapy services continued to include a range of therapeutic interventions, such as manual therapy, thermotherapy, electrotherapy, and therapeutic exercises, in addition to patient and family education to support long-term rehabilitation and management of chronic conditions and disabilities.

In the Gaza Strip, 10 out of 11 physiotherapy units were reported damaged or non-functional due to conflict-related damage and access constraints. In the occupied West Bank, including East Jerusalem, physiotherapy services were provided through seven centres, with support from 11 physiotherapists and assistant staff. 975 new cases were registered. In Jordan, 430 new patients were registered and received therapy.



A Palestine Refugee adolescent receiving physiotherapy services from an UNRWA physiotherapist at Jaramana Camp Health Centre, Syria. 2025 UNRWA photo.

Oral health

Oral health services are an integral part of UNRWA's health programme, contributing to the overall health and well-being of Palestine Refugees. UNRWA provides oral health services through a network of dental clinics across its fields of operation. The programme focuses not only on treatment but also on prevention, aiming to reduce the burden of oral diseases, particularly among vulnerable groups.

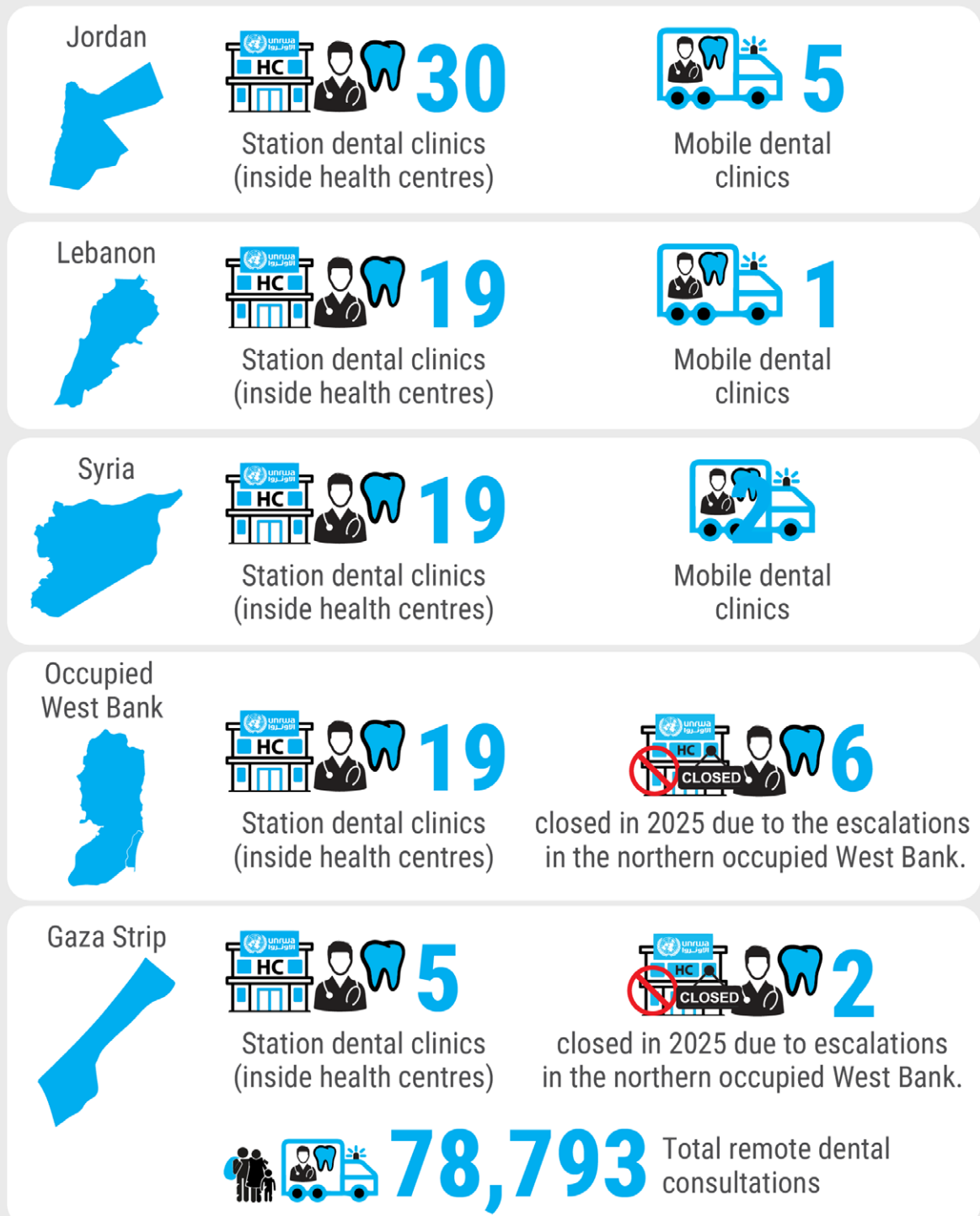
Across the four fields, excluding the Gaza Strip, a total of 469,842 dental consultations were conducted.

Of these, 311,588 were curative consultations, and 158,254 were preventive consultations.

- Jordan = 199,730 dental consultations.
- Syria = 123,852 dental consultations.
- Lebanon = 83,064 dental consultations.
- The occupied West Bank, including East Jerusalem = 63,196 dental consultations.

(For full report please see Annex 21).

Number of dental clinics



Service Delivery in Crisis Contexts

All diagnostic and supportive health services have been delivered under highly complex and constrained conditions. In the Gaza Strip, in particular, widespread destruction of health infrastructure, population displacement, and loss of health personnel have significantly affected service capacity. Despite these challenges, UNRWA has continued to provide essential services, demonstrating resilience and commitment to maintaining access to care. These efforts highlight the critical role of diagnostic and supportive services in sustaining health systems and ensuring that Palestine Refugees continue to receive the care they need, even in the most difficult circumstances.

Strengthening Laboratory Quality and Capacity

UNRWA previously operated laboratory services across all five fields of operation. Due to the ongoing conflict in the Gaza Strip and disruptions to service delivery, laboratory services were severely constrained, operating only basic point-of-care testing across nine health centres and seven medical points. Despite these challenges, UNRWA's laboratory services remain a critical pillar of primary healthcare, supporting early diagnosis, disease monitoring, and patient management.

Across the four operational fields, excluding the Gaza Strip, UNRWA conducted a total of 3,071,341 laboratory tests in 2025.

- In Jordan, 1,192,606 laboratory tests were conducted in 2025 across 25 health centres.
- In Lebanon, 478,778 laboratory tests were conducted across 17 laboratories, serving 187,754 patients.
- In Syria, 547,550 laboratory tests were conducted across 21 laboratories and two medical points.
- In the occupied West Bank, including East Jerusalem, 852,407 laboratory tests were conducted across 42 health centres, comprising 453,776 automated tests and 267,983 manual tests.

(Please see the full Laboratory report in Annex 22).

Programmatic initiatives: Promoting spinal health among UNRWA students in the occupied West Bank

In October, the UNRWA Physiotherapy Programme implemented a targeted spinal health awareness campaign across intermediate basic schools in the occupied West Bank, including East Jerusalem, aiming to promote early prevention of spinal problems among school-aged children. Through engaging, interactive sessions, physiotherapists introduced students to the structure and function of the spine and raised awareness of common spinal deformities and the factors contributing to their development. The sessions emphasised practical preventive behaviours, including correct techniques for carrying school bags, maintaining proper sitting posture during classroom and home study, and safely lifting and handling heavy objects.

The activity reached 991 students across UNRWA schools in Jerusalem (180), Hebron (315), and Nablus (496), reflecting broad geographical coverage and strong engagement across the occupied West Bank. A key component of the sessions included guided therapeutic exercises designed to strengthen postural muscles and help students maintain healthy spinal alignment.

By combining awareness with hands-on demonstrations and exercises, the programme supported students with practical knowledge to protect their spinal health in their daily routines. This initiative highlights UNRWA's commitment to preventive health education in schools, supporting children to develop lifelong habits that safeguard musculoskeletal health and reduce the risk of future spinal problems.

An UNRWA laboratory technician conducting blood sample testing at Bar Elias Health Centre in Bekaa area, Lebanon. © 2025 UNRWA photo by Bilal Alhaj Khalil.





An UNRWA medical officer providing a remote health consultation (telemedicine) to a Palestine Refugee patient via digital communication technologies at Deir Al- Balah Health Centre, the Gaza Strip. ©2025 UNRWA photo.

Telemedicine

Telemedicine is a critical component of UNRWA's healthcare strategy, particularly in humanitarian and emergency contexts. Its origins trace back to the COVID-19 pandemic in 2020, when UNRWA first institutionalised telemedicine to maintain continuity of care amid lockdowns, movement restrictions, and social distancing requirements. During the pandemic, remote consultations proved essential for managing non-communicable diseases (NCDs), maternal and child health (MCH), and psychosocial support, establishing the foundation for the Agency's current telemedicine framework.

Building on this foundation, telemedicine became a central lifeline during the 2023-2025 Gaza emergency, when health facilities were overwhelmed, movement was restricted, and vulnerable populations, including pregnant women, children, the elderly, and NCD patients, faced significant barriers to care.

Key impacts include:

- Ensuring continuity of care during conflict and displacement.
- Reducing patient exposure to movement-related security risks.
- Alleviating pressure on overstretched health centres and healthcare staff.
- Enabling timely clinical decision-making and early identification of high-risk cases.
- Supporting equitable access to healthcare across the Gaza Strip, the occupied West Bank, including East Jerusalem, Jordan, and Syria.

Telemedicine addresses logistical barriers, limited mobility, and access constraints, and supports continuity of care while reinforcing health system resilience in crises.

How Telemedicine Improves Health and Service

In 2025, UNRWA's telemedicine services played a substantial role in patient care and health system performance across the Gaza Strip, the occupied West Bank, Jordan, and Syria. A total of 563,938 consultations were delivered, accounting for approximately 30 per cent of all health centre visits in the Gaza Strip, underscoring the critical role of telemedicine in maintaining continuity of care despite conflict-related disruptions and restricted mobility. By the end of the year, telemedicine had been expanded to 25 health centres across the occupied West Bank, Syria, and Jordan, extending equitable access to healthcare for vulnerable populations, including children, elderly patients, and those with limited physical access to health facilities.

The quality and safety of services were reinforced by implementing standardised clinical protocols, ensuring consistency in diagnosis, treatment planning, and follow-up, and enabling timely identification of high-risk cases. This structured approach contributed to improved patient safety and more efficient use of clinical resources. Telemedicine also enhanced chronic disease management, enabling regular monitoring and follow-up for patients with diabetes, hypertension, asthma, and other non-communicable diseases. By providing these continuous, remote services, the programme helped prevent disease complications, improve treatment adherence, and promote better long-term health outcomes.

The quality and safety of services were reinforced by implementing standardised clinical protocols, ensuring consistency in diagnosis, treatment planning, and follow-up, and enabling timely identification of high-risk cases. This structured approach contributed to improved patient safety and more efficient use of clinical resources. Telemedicine also enhanced chronic disease management, enabling regular monitoring and follow-up for patients with diabetes, hypertension, asthma, and other non-communicable diseases. By providing these continuous, remote services, the programme helped reduce the risk of disease complications, supported treatment adherence, and contributed to better health outcomes.

Achievement During 2025

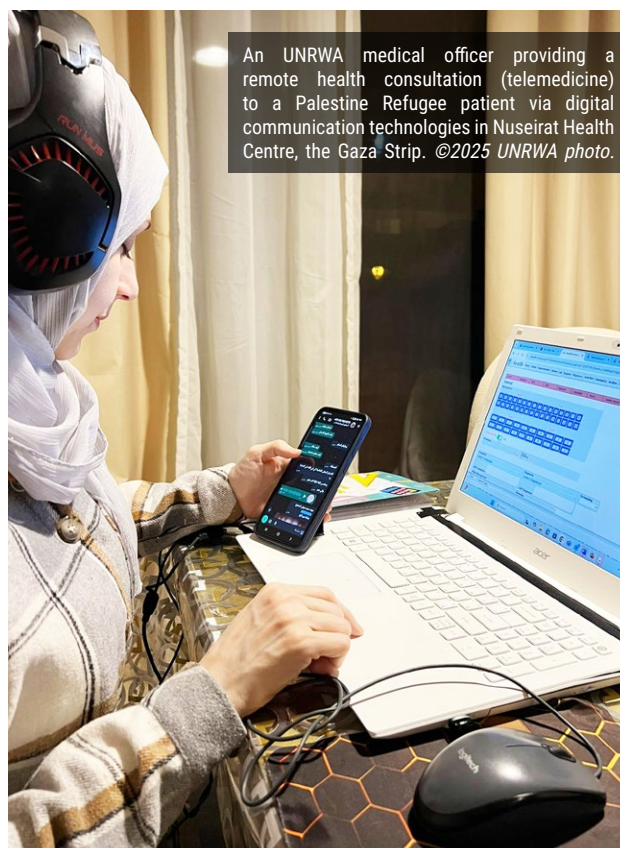
UNRWA's telemedicine programme delivers a wide range of essential health services across multiple fields, ensuring that vulnerable populations receive care despite the challenges of conflict, displacement, and limited access to physical health facilities. The services were structured to provide continuity, accessibility, and quality, while leveraging remote technologies to overcome logistical barriers.

Medical officers and specialists play a central role in service delivery, providing general medical consultations, chronic disease management, maternal and child health services, and specialist cardiac care. Remote consultations included comprehensive patient assessments, development of individualised management plans, and issuance of electronic prescriptions when indicated. Patients submitted their medical histories, laboratory results, and prescriptions via digital photographs or scanned documents, enabling remote evaluation and decision-making. Follow-up teleconsultations ensured adherence to treatment plans and the early identification of patients requiring urgent in-person care. Throughout 2025, 55 medical officers and four specialists conducted 563,938 consultations, averaging 43 consultations per medical officer per day and 23 per specialist, across 25 health centres in the Gaza Strip, the occupied West Bank, including East Jerusalem, Jordan, and Syria. The programme's design allowed for flexible triage, referrals, and patient counselling, contributing to improved continuity of care and efficient utilisation of health workforce expertise.

Tele-oral health services (TOHS) provided remote dental consultations, preventive guidance, and triage for patients requiring in-person interventions. Patients submitted photographs or X-rays of dental conditions, which were evaluated by a team of dental surgeons. Advice, prescriptions, and referrals were delivered digitally, complemented by educational materials promoting oral health. During the year, TOHS managed 78,793 remote consultations, representing

approximately 50 per cent of total oral health cases. Of the patients reached, 57 per cent were female, 30 per cent were under 18 years of age, and eight per cent were over 65. Of these consultations, 35 per cent resulted in advice or referrals, while 65 per cent led to medical prescriptions, with only 6.4 per cent involving antibiotics. The service leveraged WhatsApp communications to maintain coordinated case management across multiple providers and ensure timely responses.

Telerehabilitation / physiotherapy services enabled remote management of rehabilitation and mobility support, particularly for patients unable to attend in-person sessions due to security constraints, disabilities, or logistical barriers. Remote physiotherapists conducted assessments, reviewed medical histories and imaging, and provided structured exercise plans via video demonstrations, brochures, and personalised follow-up instructions. Patients received guidance on exercise technique, progression, and monitoring of recovery outcomes. In 2025, five physiotherapists delivered 22,980 remote consultations, including with 8,277 new clients, of whom 54 per cent were female. These consultations accounted for 23 per cent of total physiotherapy services, demonstrating the significant role of remote care in sustaining rehabilitation services. Each physiotherapist conducted an average of 20 consultations per day, with each session lasting 20 minutes, ensuring consistent patient support and progress monitoring.



Tele-psychosocial services addressed mental health needs by providing remote psychological, emotional, and behavioural support to vulnerable populations, including children, adults, and staff requiring guidance. Counselling sessions were conducted via WhatsApp video, audio calls, and text messaging, following informed consent and structured documentation in the e-health system. Interventions included stress management, anxiety and sleep disorder support, behavioural therapy, staff counselling, and community guidance distributed through social media. The programme maintained clear standard operating procedures to standardise service delivery, record-keeping, and follow-up. Five counsellors delivered 11,650 sessions last year, 66 per cent of the patients were female, and services supported both self-referred cases and patients identified through telemedicine stations.

Data entry services were integral to the telemedicine system, ensuring that patient records, maternal health data and vaccination logs were accurately captured and structured. Field staff submitted data digitally or via photographed hard copies, which were then reviewed, validated, and entered into e-health and other structured databases by a dedicated remote team. The data entry team successfully processed 72,365 vaccination records and 50,805 maternal health records, maintaining a daily workload of approximately 100 entries per staff member. This continuous remote oversight allowed for accurate reporting, performance monitoring, and timely intervention planning across fields.

Nutrition services focused on child malnutrition screening and follow-up, targeting children aged 6–59 months and provision of referrals to therapeutic feeding programmes for global acute malnutrition (GAM) cases. Field staff collected screening data and submitted it digitally to the remote nutrition team, which conducted HQ-level review, validation, and structured reporting. Weekly monitoring and feedback mechanisms ensured continuous oversight of malnutrition prevalence and treatment needs. In 2025, 163,704 MUAC screenings were conducted among children aged 6-59 months. More than 12,000 GAM cases were identified and referred to outpatient therapeutic feeding programme. The nutrition team coordinated closely with the Field Family Health Officer and data reviewers, maintaining high-quality data management and consistent communication with local field teams.

Collectively, these outputs reflect a comprehensive, multi-disciplinary telemedicine programme that delivered structured, remote healthcare services across the Gaza Strip, the occupied West Bank, Jordan, and Syria. By integrating technology, standardised procedures, and specialised staff, UNRWA was able to maintain essential health services, support vulnerable populations, and strengthen the resilience and responsiveness of the health system amid extraordinary challenges.

(Please see Annex 23.)



Palestine Refugees patients receiving medications from an UNRWA pharmacist at the temporary Thenaba medical point in the occupied West Bank, including East Jerusalem. ©2025 UNRWA photo.



A Palestine Refugee patient receiving NCD healthcare service from an UNRWA health staff at the Japanese Health Centre in the Khan Younis area, the Gaza Strip. ©2025 UNRWA photo.

Overview of UNRWA's humanitarian response and healthcare challenges amidst the crises in the Gaza Strip and the occupied West Bank including East Jerusalem

UNRWA health response in the Gaza Strip during the 2025 emergency

UNRWA continued to deliver one of the largest primary health responses in the Gaza Strip in 2025, amid intense hostilities, displacements, and severe shortages of medicines, fuel, and medical supplies. Despite extensive destruction of infrastructure and repeated operational constraints, UNRWA remained the principal provider of primary healthcare, accounting for around 40 per cent of all health consultations delivered in the Gaza Strip. Through health centres, medical points at shelters, and temporary service points, UNRWA provided millions of medical consultations, maternal and child healthcare, vaccination services, communicable disease surveillance, mental health and psychosocial support, and treatment for chronic diseases.

UNRWA played a key role in disease surveillance, early warning systems, and nutrition screening, helping to mitigate the public health risks associated with mass displacement, overcrowding, and poor sanitation. Innovative approaches, including mobile outreach and remote health support, were used to sustain access where movement and security constraints limited in-person services.

The response demonstrated substantial operational resilience and sustained commitment by health staff working under extreme danger and hardship, ensuring that lifesaving services continued for Palestine Refugees and other vulnerable groups throughout the emergency.



Health services provided in the Gaza Strip



8-9 Health centres



53 Medical points operational over the year



1.7 Million IDPs reached



4,531,840 Consultations



15,500 Emergency healthcare interventions

Maternal and child health



36,362

Pregnant women registered



154,414

Antenatal visits



30,212

Postnatal visits



89,026

Family planning consultations



324,027

Women received nutrition support



3

Maternal deaths reported



127,341

Vaccination visits



29,260

Children vaccinated

Nutrition response



162,925

Children screened for malnutrition



12,163

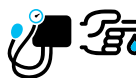
Children treated for acute malnutrition

Non-communicable diseases



~18,400

NCD consultations per month sustained



~70%

of patients rely on UNRWA services

Mental health and psychosocial support



11,826

Psychological first aid sessions



9,463

Counselling sessions



55,666

Awareness-raising sessions



4,127

GBV cases supported



6,197

Severe cases managed

Trauma and rehabilitation



351,292

Trauma and dressing cases



13,965

Physiotherapy patients



98,630

Physiotherapy & rehabilitation sessions delivered



1,838

War injuries managed

Telemedicine expansion

 **52**
Doctors delivering
remote consultations

 **78,793**
Tele-oral
sessions

 **22,980**
Tele-rehabilitation
sessions

 **11,571**
Tele-psychosocial
sessions

Vaccination campaigns

 **602,795**
Children vaccinated
(polio campaign)

32%-34%
Delivered by
UNRWA teams


No confirmed
polio cases in 2025

Pharmaceutical supply achievements

 **520**
Pallets delivered
during ceasefire

 **720**
Pallets received
from WHO

58% to 27%
Stockouts
reduction

Public health surveillance

 **147**
Guillain-Barré
cases monitored

 **11**
Diseases
under surveillance


Global
EWARS
EWARS implemented
across all sites

Workforce resilience

 **HC**
Services sustained despite
displacement and insecurity

 **4.5+**
Million
Health consultations
provided

 **400+**
Additional
staff contracts

 **24**
Staff injuries
reported

Laboratory services

 **249,179**
Tests conducted across limited-capacity labs

Oral health

 **79,896**
In-person consultations

 **78,793**
Tele-oral consultations

 **158,689**
Total beneficiaries



A Palestine Refugee student receiving vision screening from UNRWA health staff as part of UNRWA school health programme services at Nuseirat Health Centre, the Gaza Strip. ©2025 UNRWA photo.

How data was reported and validated

Reporting under bombardments, incursions, and IPC Phase 5 conditions: data quality amid a severe humanitarian crisis

The reporting channel and process for health data in the Gaza Strip, as of August 27, 2025, are outlined as follows: Data collection at the field level was based on paperwork, offline Excel sheets, WhatsApp texts, and screenshots of records. The data collectors were assigned focal points (e.g., nurses, physiotherapists, nutritionists, and medical officers) to collect raw data from health centres or medical points using the aforementioned offline tools in addition to episodically available online electronic medical records and other systems, such as the WHO's EWARS. The first-level review was conducted by reporting focal points who performed a «spot check» of the division's output report and EWARS data. They then uploaded the IMT sheet data to UNRWA's protected SharePoint. The reporting team then conducts a second-level review of the files uploaded to the encrypted SharePoint. They hold review meetings to evaluate the reporting progress and identify anomalies or irregularities. The root causes of anomalies are addressed by the Gaza Field Office (GFO) staff. The health monitoring, evaluation, reporting, and learning

officer (MERL) conducted the third level of data review and validation, which involved a comprehensive monthly review of input data, including triangulation, identification, and remediation of the root causes of potential exceptions. Additionally, the MERL officer compared the GFO input data with that of other stakeholders (e.g., the Health Cluster) for validation. Information dissemination was conducted at the GFO and HQ levels through weekly slide decks created by the reporting team. In addition, weekly situational reports and bi-weekly Gaza Crisis Reports were produced. Outputs were shared with external actors, such as the Health Cluster and WHO, for further review and action.

Reviving Gaza Strip health reports and indicators in 2025 NCD reports and indicators

Despite the severe humanitarian circumstances in the Gaza Strip, GFO staff in the Strip and Cairo made substantial efforts to report health data from 2023 onwards. The volume and quality of reports and data have improved over the past two years. In 2025, the Gaza team produced a comprehensive annual report covering health infrastructure and workforce (please see section 1), maternal and child health, nutrition, NCDs, infectious diseases, and laboratory services.

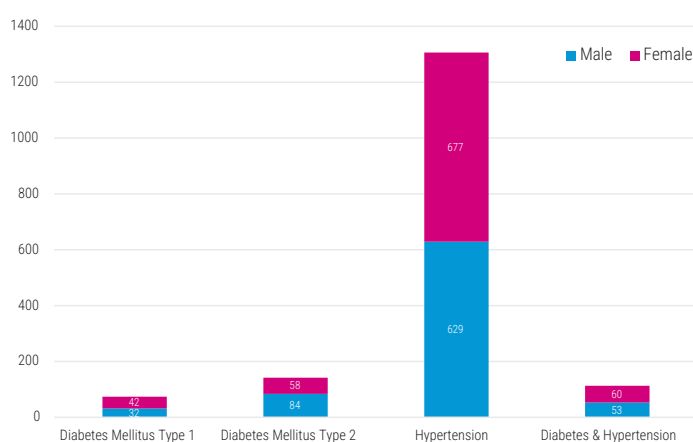


Figure 59: Number of newly registered patients with noncommunicable diseases in the Gaza Strip in 2025.

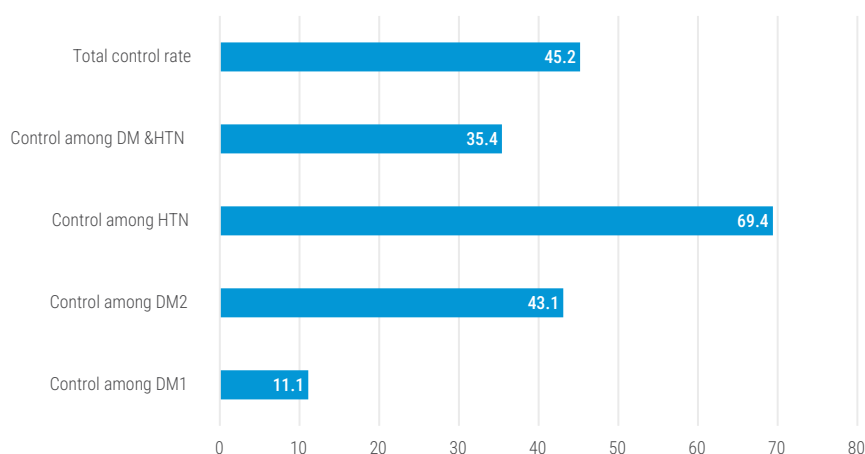


Figure 60: Proportion (%) of patients with noncommunicable diseases with controlled conditions in the Gaza Strip in 2025.

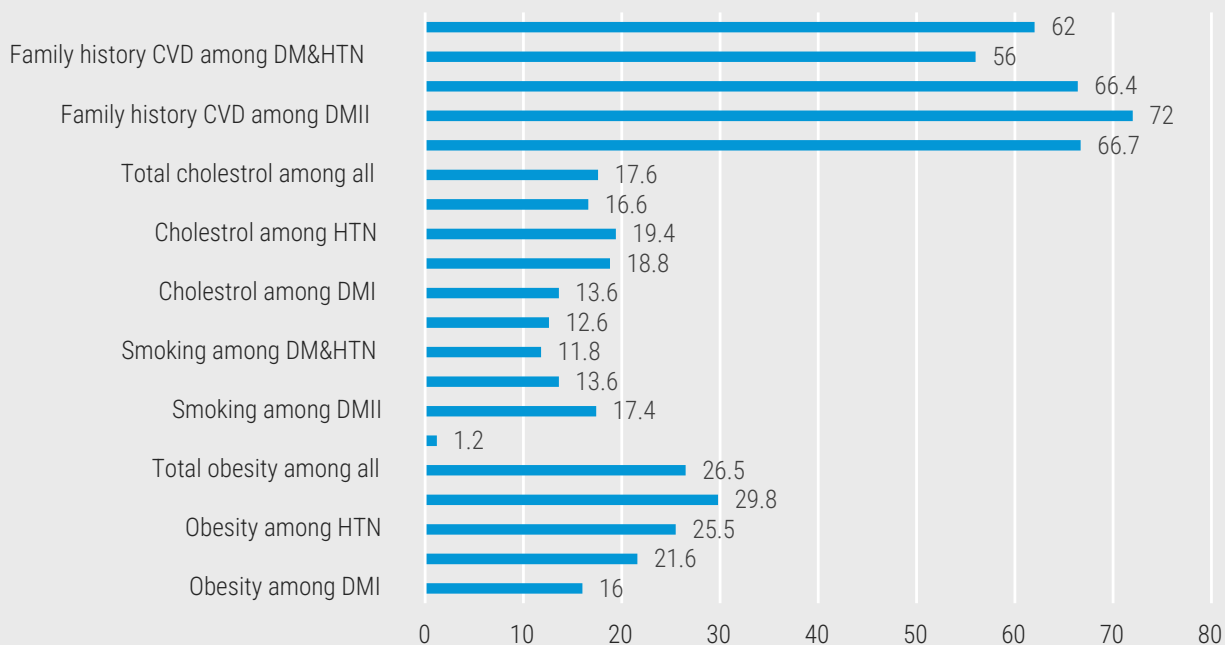


Figure 61: Prevalence (%) of risk factors among patients with noncommunicable diseases in the Gaza Strip in 2025.

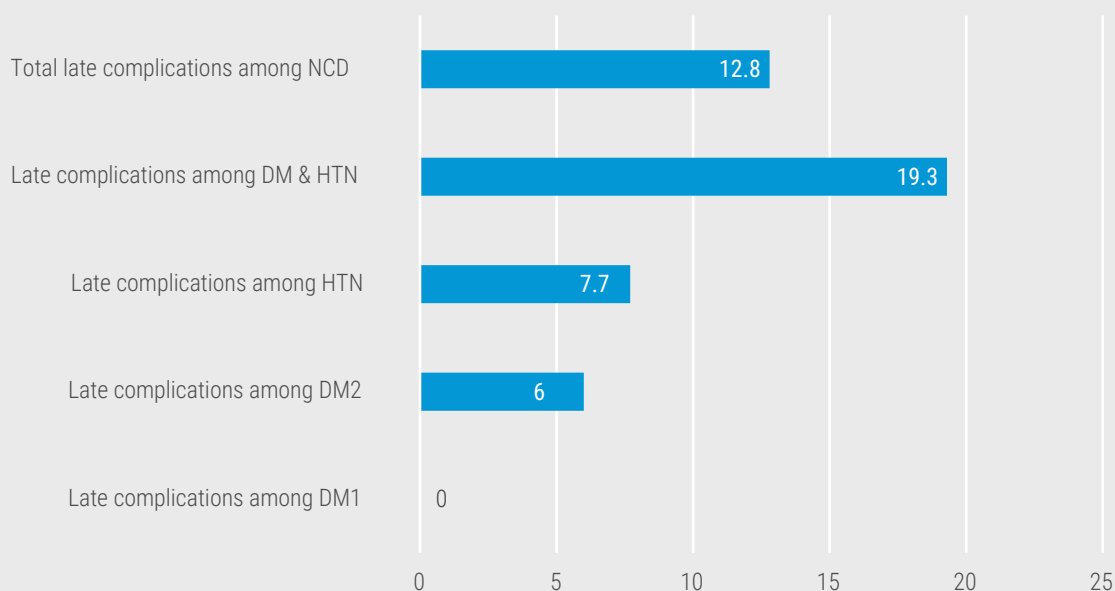


Figure 62: Prevalence (%) of late complications among patients with diabetes mellitus and hypertension in the Gaza Strip in 2025.

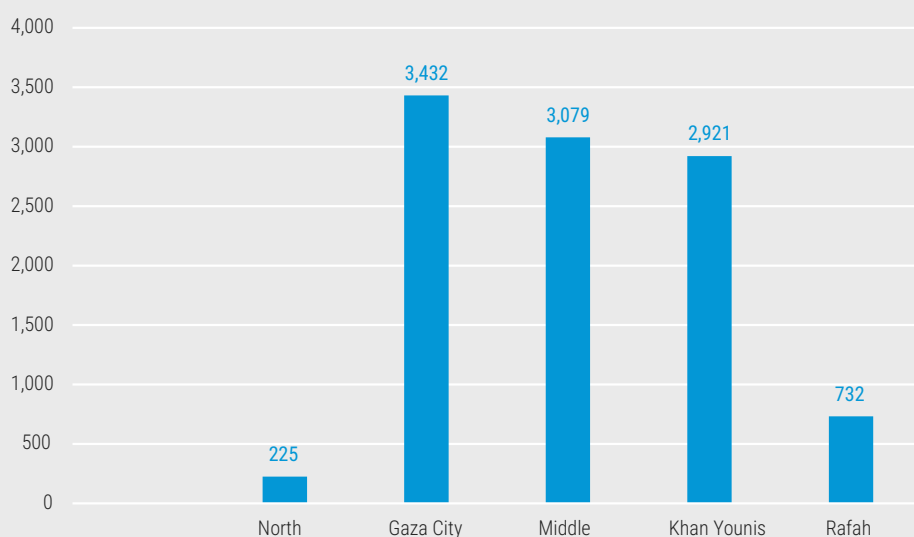


Figure 63: Number of women newly registered in preconception care service in the Gaza Strip in 2025.

Mother and child health and indicators

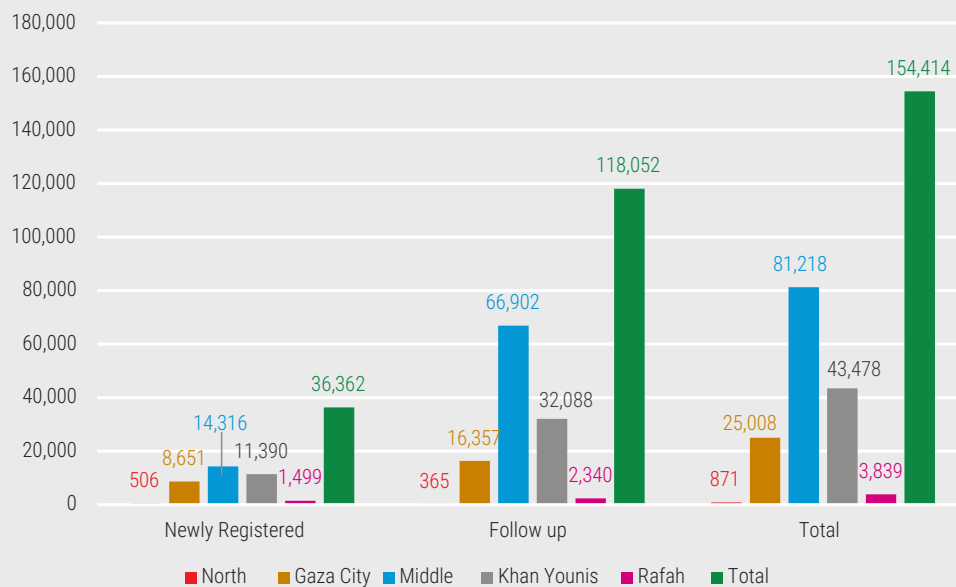


Figure 64: Number of registered pregnant women and ante-natal care visits in the Gaza Strip in 2025.

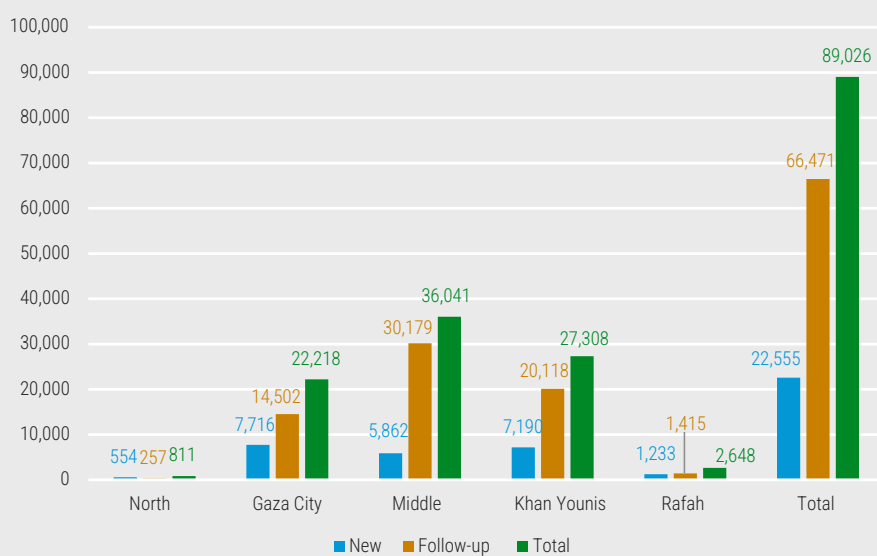


Figure 65: Number of family planning consultations provided in the Gaza Strip in 2025.

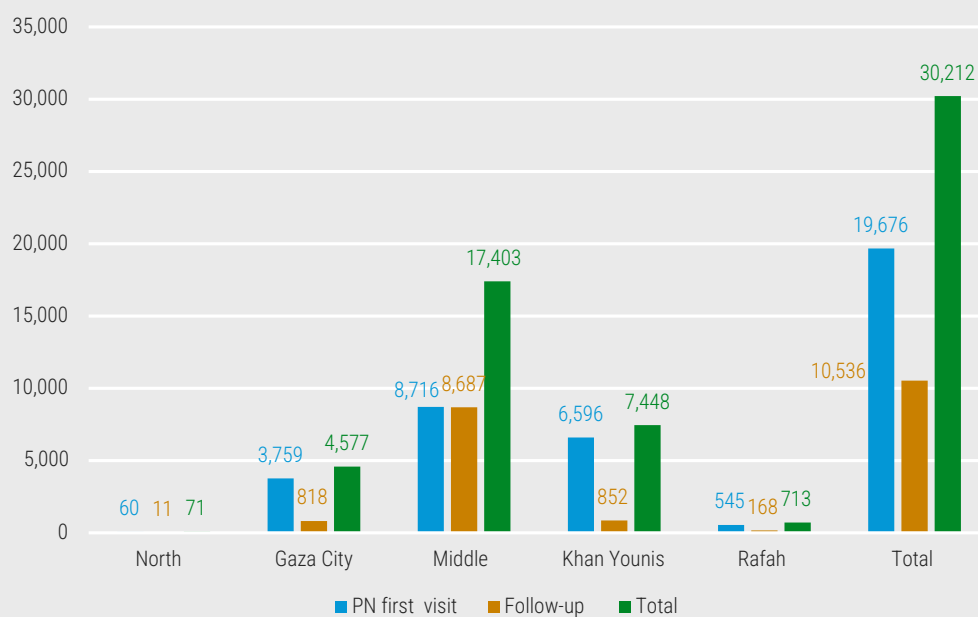


Figure 66: Number of post-natal consultations in the Gaza Strip in 2025.

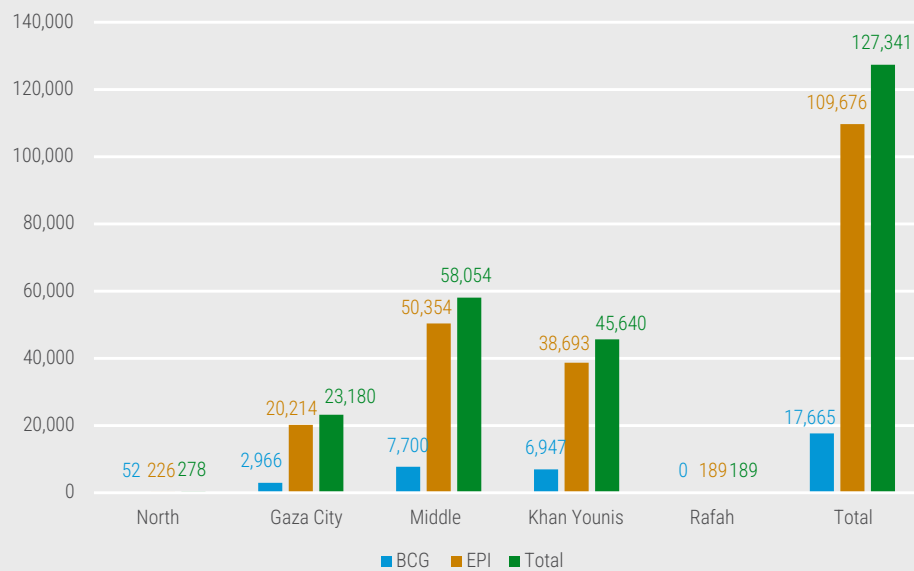


Figure 67: Number of vaccination sessions in the Gaza Strip in 2025.

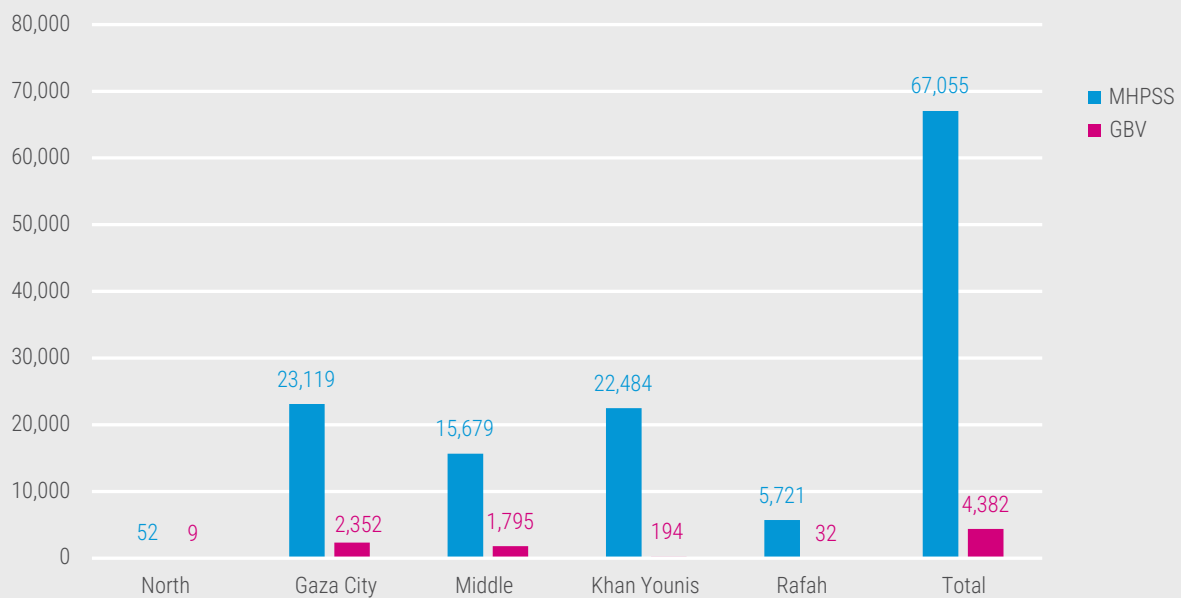


Figure 68: Number of persons who had MHPSS screening and number of persons found exposed to GBV in the Gaza Strip in 2025.



Surveillance in the Gaza Strip

Table 7: Infectious disease cases (cummulative cases) in the Gaza Strip in 2025

	Middle (Deir Al-Balah)	Gaza city	Khan Younis	North Gaza, Beit Hanoun, Beit Lahia, Jabalia	Rafah	Total
AFP number of cases	0	0	1	1	0	2
AJS number of cases	235	129	291	2	7	664
ARI number of cases	185763	108573	154592	4093	1935	454956
AWD 5 years old and above	24434	28084	37326	668	474	90986
AWD under five-year-old cases	32082	30586	43785	1010	403	107866
Bloody diarrhoea number of cases	157	100	651	6	0	914
Diphtheria number of cases	0	0	0	0	0	0
Ectoparasitic skin disease number of cases	1924	1579	5777	118	124	9522
Measles number of cases	2	0	1	0	0	3
Meningitis number of cases	31	40	150	0	0	221
Mumps number of cases	51	26	139	2	0	218
Suspect chickenpox (varicella) number of cases	126	72	306	5	1	510
Suspect impetigo number of cases	1525	1682	4406	240	65	7918
Suspected cholera 5 year old and above	0	0	0	0	0	0
Suspected cholera in under-five- year-old cases	0	0	0	0	0	0
Tetanus number of cases	0	0	0	0	0	0
Tuberculosis cases	0	0	0	0	0	0
Unusual events	0	0	0	0	0	0
Grand total	246330	170871	247425	6145	3009	673780

(Please see the full reports from Gaza starting from Annex 24-29).

The occupied West Bank, including East Jerusalem

Northern occupied West Bank

The escalations in the north of the occupied West Bank peaked in late 2024. According to UN reporting, demolitions and related displacement affected more than 4,000 people, while the operation known as "Iron Wall" reportedly displaced approximately 40,000 people in January 2025, the largest displacement recorded in the occupied West Bank since 1967. At the same time, around 70 per cent of health facilities in the north were reported as closed or only partially functional. These developments substantially reduced access to essential health services, increased protection risks for displaced families, and heightened the need for emergency outreach and continuity of care.

UNRWA operates 43 comprehensive health centres (HCs) across the occupied West Bank, including two HCs in East Jerusalem, serving 917,942 registered Palestine Refugees. In late 2024, three main HCs in the northern area were

forcibly shut down by the Israeli forces amid the escalation that began in mid-2024. The displaced people spread across various areas of the occupied West Bank, with the majority settling in villages near the closed HCs, such as Anabta, Attil, and Beit Qad. In response to this precarious situation, UNRWA established two emergency clinics and two emergency mobile clinics in Jenin and Tulkarm by the beginning of 2025. In the first three months of 2025, the UNRWA West Bank Field Office team mobilised resources and expanded the types of services provided by Qabatiya HC, adding new services to those already in place

In addition, a new emergency clinic was established in Al-Saraya location near Tulkarm to meet the huge need for healthcare in the area. All these clinics were registered on e-health for immediate reporting and monitoring. The emergency clinics played a vital role in maintaining access to healthcare for vulnerable populations.

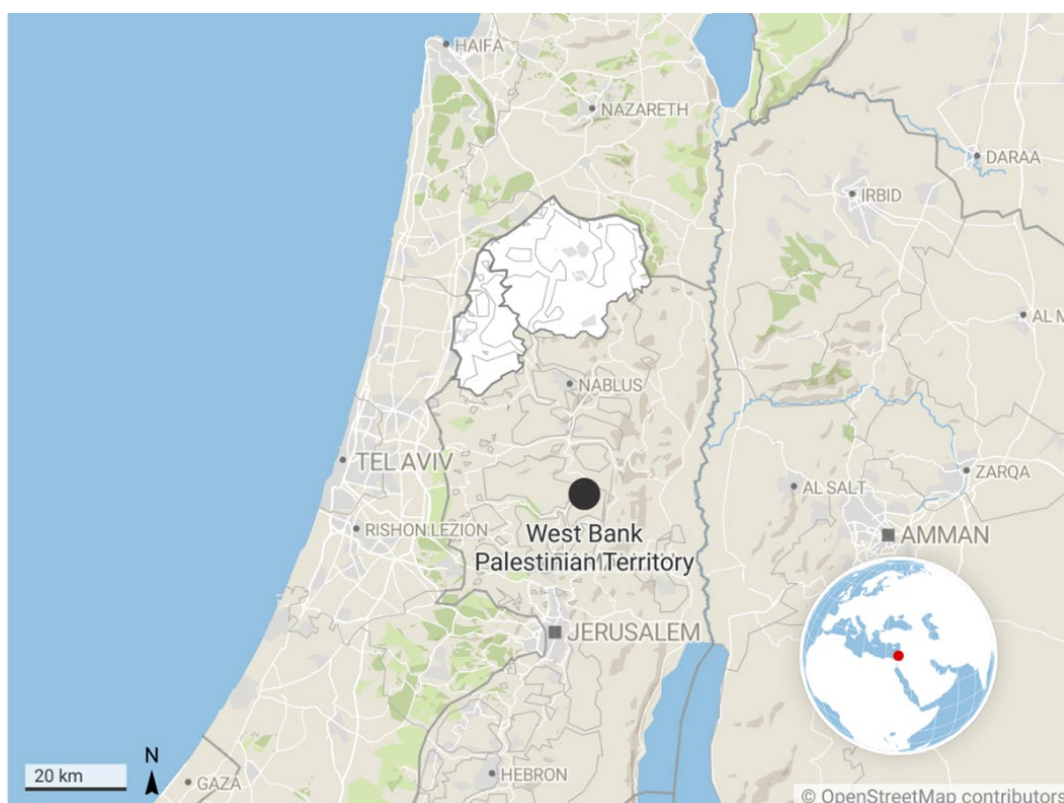


Figure 69: The central and northern occupied West Bank.

To identify locations where displaced populations settled, the West Bank field team contacted the UNRWA relief department and the local [KC38.1] municipalities. These sources of information were key during the early planning phase of the emergency response. In addition, the staff of nearby HCs began collecting information from visitors displaced from the three camps. Visitors were asked where they, their neighbours, and their relatives lived at the time, and about the characteristics of the displaced populations living around them (children, elderly). UNRWA also liaised with community committees and leaders to better understand the needs of displaced people. The UNRWA West Bank team selected the five locations with the highest numbers of displaced people. The team prioritised emergency clinic locations that were as safe and accessible as possible and offered the strongest catchment coverage. For instance, an emergency clinic was opened in the Korean Health Centre (rented) but later closed as it was close enough to Jenin and therefore unsafe for patients, especially amid the recurrent and expanded military operations around Jenin. The emergency clinic was moved to Jenin Sport Club. A similar example was the Tulkarm Emergency Clinic (Thenabeh), which was difficult to reach due to unreliable, expensive transportation and its proximity to the Tulkarm area affected by access and security constraints. Consequently, a new emergency clinic was established in the Al-Saraya area (Tulkarm Emergency Clinic 2), in addition to the one in Thenabeh. As many displaced people could not reach existing or emergency HCs due to long distances, high costs, or military

operations, mobile clinics were deployed to these areas on specific days of the week as a solution. The establishment of new clinics and the selection of locations, as well as the movement of the mobile clinics, were coordinated with the UNRWA security department. Telemedicine was implemented to improve access to emergency care, particularly for patients with difficulty commuting and those not reachable by mobile clinics.

The UNRWA West Bank Field Office team, in collaboration with and supported by headquarters in Amman, prepared a slide deck to disseminate information on technical and risk communication and to raise funds. The slide deck provided an overview of the health centres' status in the occupied West Bank, covering several domains: the number of medical consultations, emergency clinics, mobile clinics, surveillance, and data on UNRWA employees' commuting disruptions caused by roadblocks imposed by Israeli authorities. Moreover, an incident management team was formed using an IMT sheet to update, follow up on, and track the progress of actions and interventions. Weekly meetings were held to discuss updates on challenges and follow up on implemented actions. UNRWA undertook technically and operationally efforts to respond to the emergency and sustain health services. However, neither quantitative nor qualitative work has been done to examine how effective the actions were, especially for vulnerable patients such as pregnant women, infants and children, and people with morbidities.

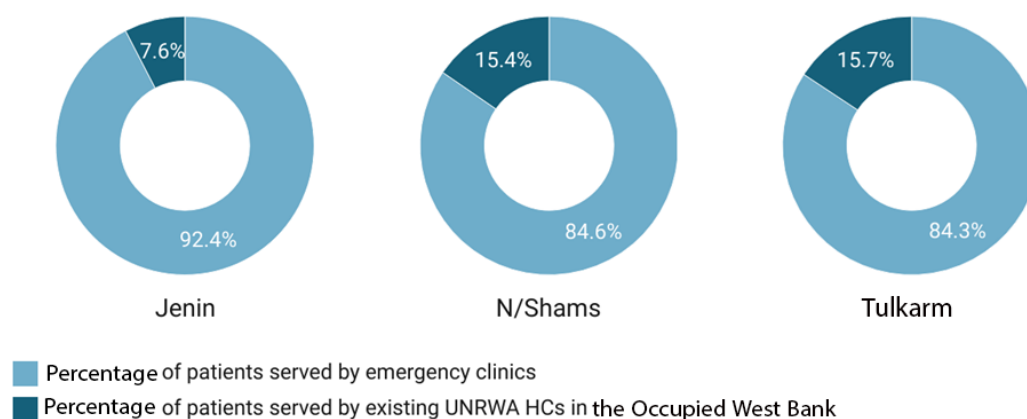


Figure 70: The percentage of patients registered in the three closed HCs who received healthcare from UNRWA health installations between January-June 2025.

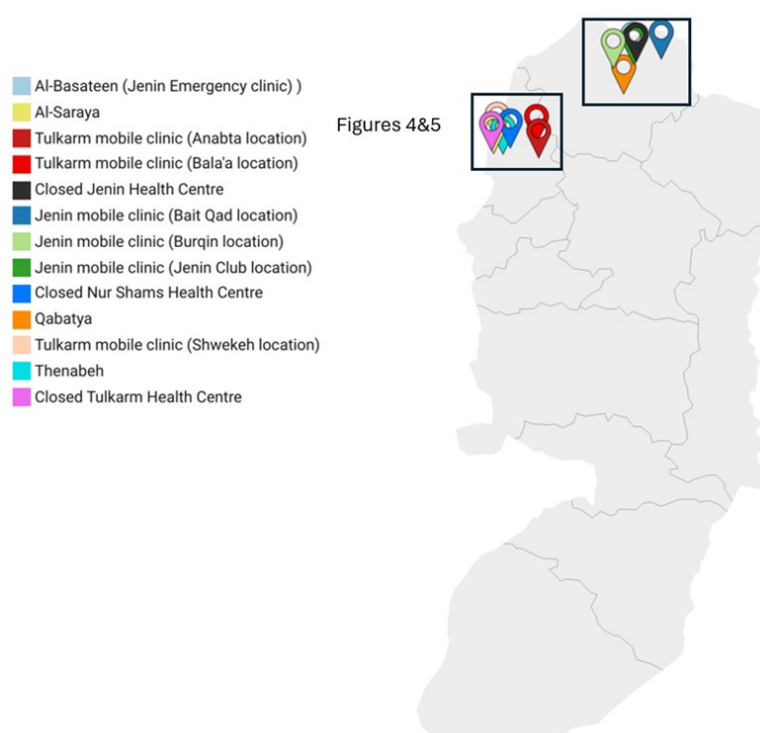


Figure 71: A map of the West Bank showing the locations of the three closed health centres and the established emergency clinics in June 2025.



Figure 72: Geographic coverage and health services of UNRWA emergency clinics provided to patients of the closed Jenin health centre (MCH: mother and child health, NCDs: non-communicable diseases, Imm/GM: immunisation and growth monitoring).

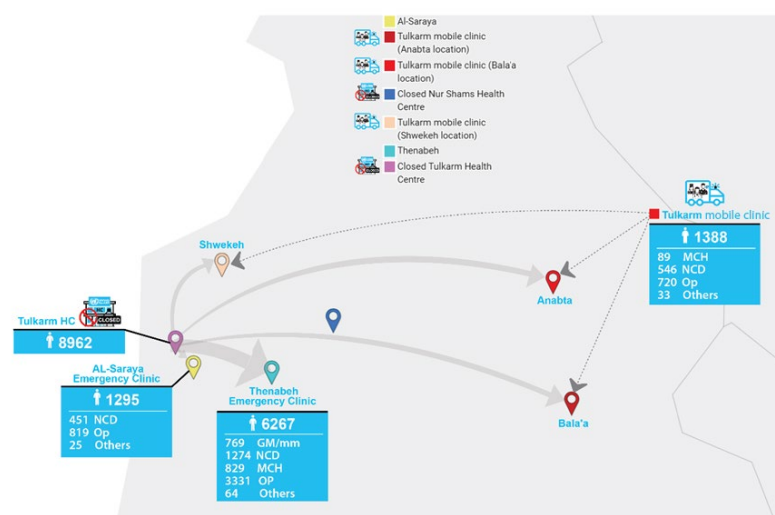


Figure 73: Geographic coverage and health services of UNRWA emergency clinics serving patients of closed Tulkarm health centre.

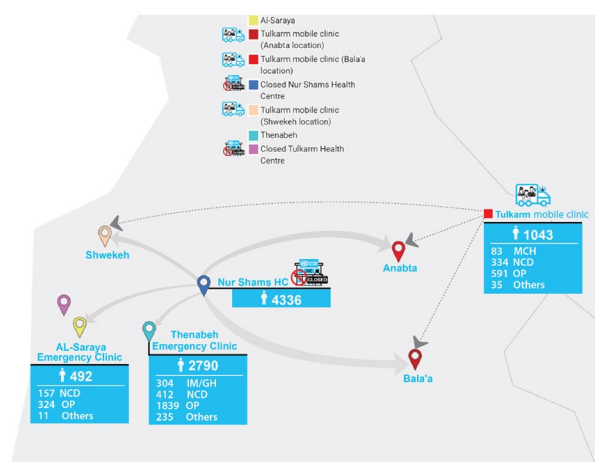


Figure 74: Geographic coverage and health services of UNRWA emergency clinics delivered to patients of closed Nur Shams health centre.

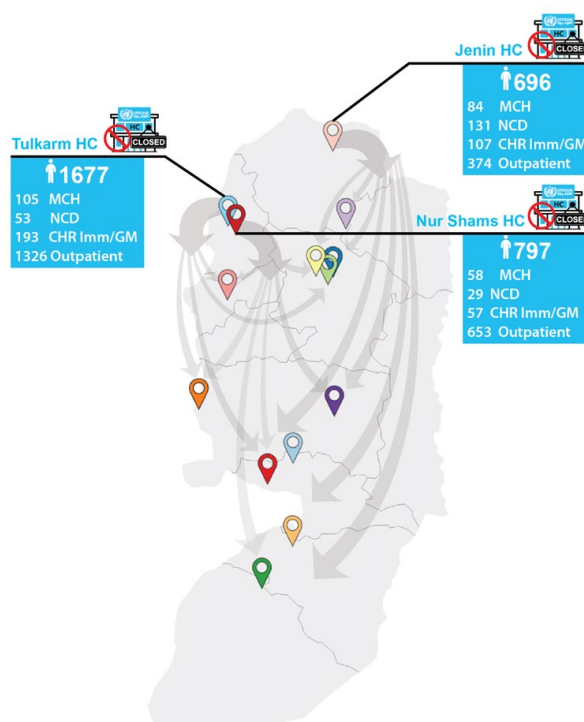


Figure 75: Number of patients from the active cohort of the three closed HCs who received healthcare from the pre-existing UNRWA HCs across the occupied West Bank in 2025 (CHR: child health record).

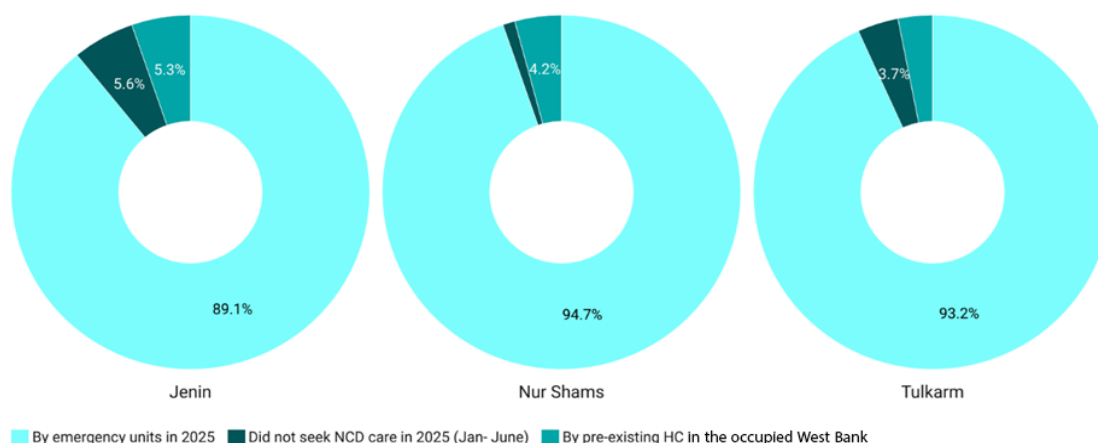


Figure 76: Percentage of NCD patients registered in the three closed health centres who received NCD care from UNRWA installations in the occupied West Bank, June 2025.

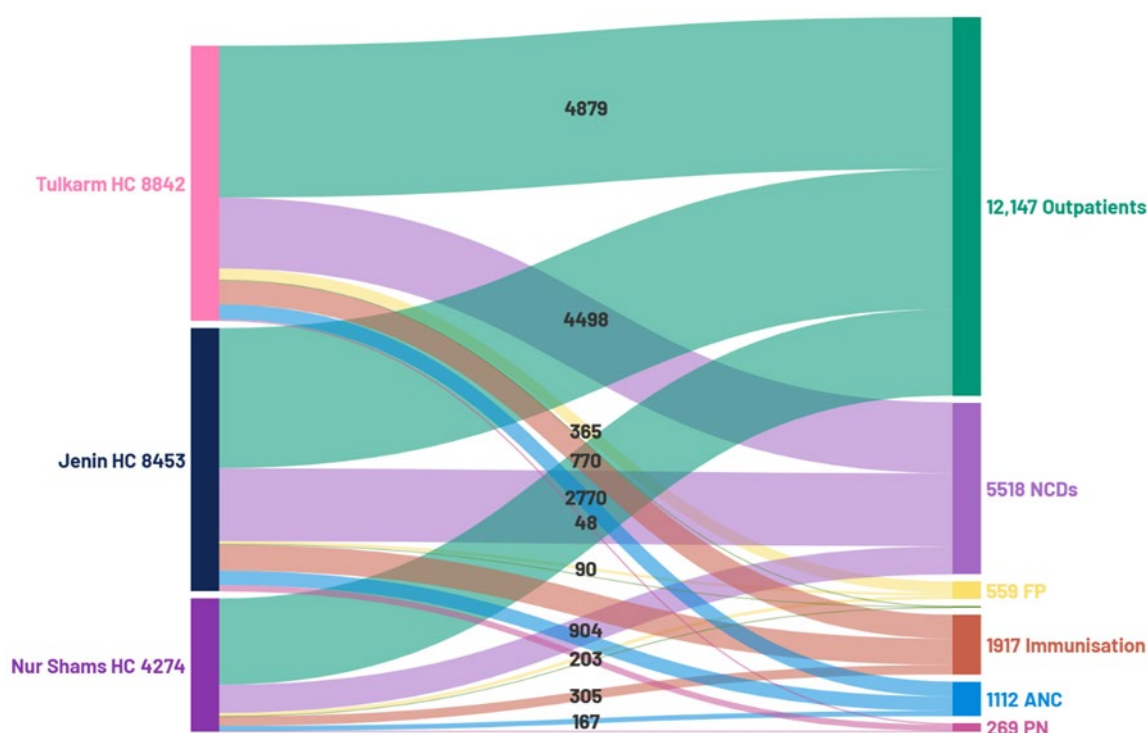


Figure 77: Number of patients who received healthcare through the emergency clinics, disaggregated by the type of service provided.

The lives of patients with diabetes depend critically on the availability of a competent health system and supply chain. Diabetic patients, especially those on insulin, are among the most vulnerable amid displacement. Insulin injections should never be interrupted as even a short interruption of one day may be associated with the risk of ketoacidosis [4]. Generally, there is little literature on NCD care provision for IDPs, and IDPs face interrupted healthcare. [5].

East Jerusalem

Where services exist but not for all: UNRWA's role in addressing access gaps in East Jerusalem

Not all Palestinians with Israeli ID are covered by Kupat Holim, the Israeli public health insurance system. Kupat

Holim and Palestinian governmental insurance do not cover several services, referrals, and medications. Patients with insurance pay a monthly subscription fee and a percentage of the medication and laboratory costs. Patients with NCDs suffer the most in terms of affording the cost of medications and laboratory services. Access to affordable family planning, ANC, and ophthalmology is severely limited. Most patients cannot afford transportation costs and cannot physically commute to health centres because of long distances and checkpoints. Services and medications are not continuously available in the Palestinian health centre. UNRWA clinics in East Jerusalem are easily accessible and provide free, holistic, and sustainable health services.

UNRWA's health centres in East Jerusalem play an indispensable role for Palestine Refugees—both those holding Jerusalem (Israeli) IDs and those holding West Bank IDs—by filling critical gaps left by the Israeli and Palestinian governmental health systems. Four interrelated factors underpin their continued high utilisation: reliable medication supply, proximity and ease of access, affordability, and comprehensive, holistic care. Taken together, these factors explain why two-thirds of Jerusalem-ID holders and nearly all Shufat clinic visitors continue to seek UNRWA services despite formal eligibility for other health systems. If the recently passed Knesset laws were enforced to close the Al-Zawya and Shufat health centres, hundreds of vulnerable patients—many with complex chronic conditions, limited mobility, or low income—would lose their primary source of affordable, accessible, and comprehensive care.

In summary, UNRWA's health centres in East Jerusalem are a critical component of health security for Palestine Refugees. Closure or prolonged disruption of these services would risk worsening inequities in access, affordability, and quality of care for some of the most at-risk populations in the Occupied Palestine Territory.

Telemedicine in the middle of the escalation

The health system in the occupied West Bank, offers telemedicine services, which, in addition to being a growing global practice, are highly needed in the Palestinian context. Elderly patients and those with disabilities can face difficulties commuting to and accessing HCs under ordinary circumstances. In the occupied West Bank, including East Jerusalem, the roadblocks and movement restrictions result in longer waiting times and detours to bypass roadblocks, checkpoints, and movement restrictions, which make it more difficult for these patients to reach the clinics. The healthcare workers in the clinics

and HCs conduct phone consultations with patients who are unable to reach the HCs, record these consultations in e-health, and dispense medications to patients' relatives. Overall, 879 patients were serviced by both the emergency clinics and the preexisting HCs through telemedicine in 2025 (January to June inclusive). Of these patients, 76 per cent were NCD patients, 15.8 per cent were outpatients, and 8.2 per cent were MCH patients.

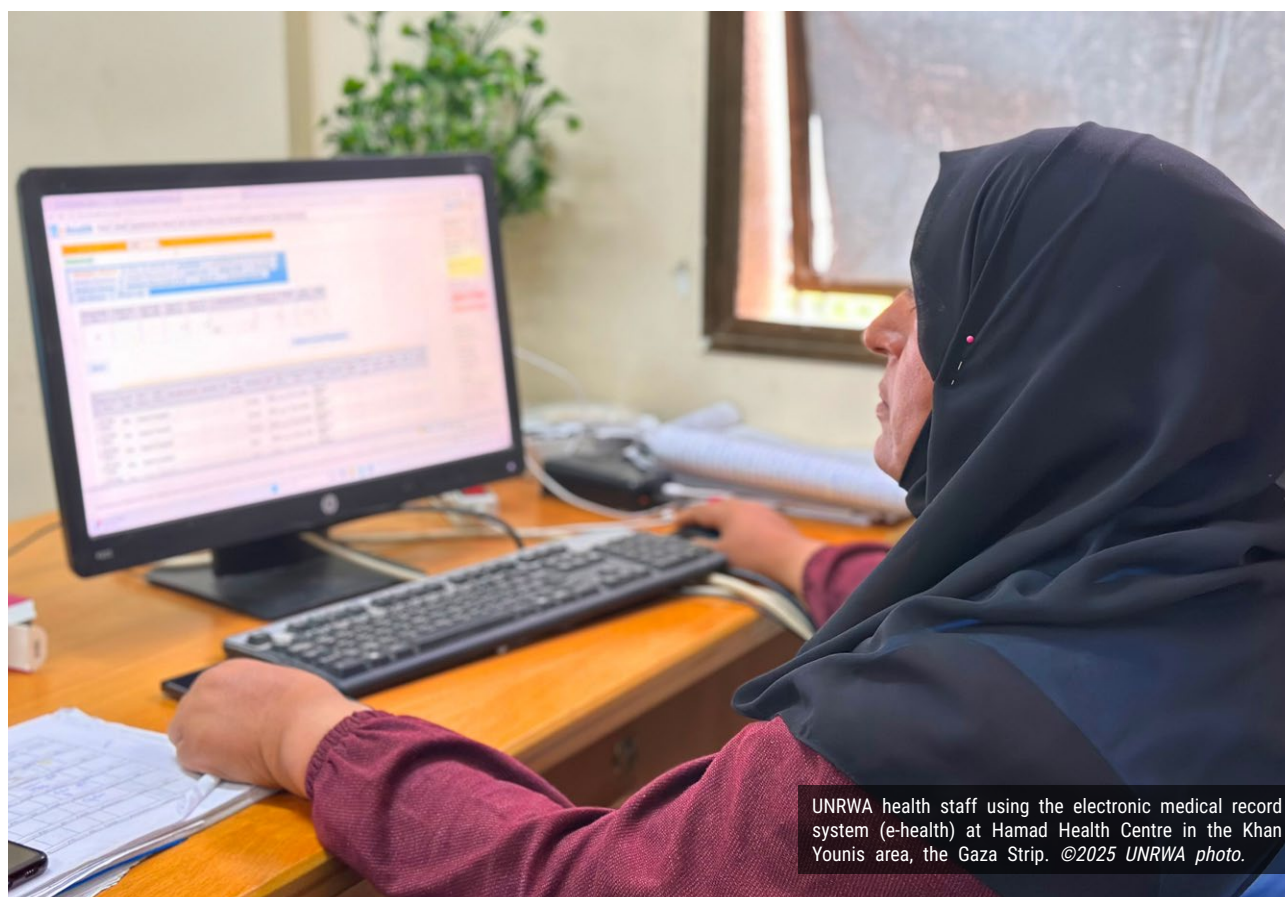
Evaluation of UNRWA's Health Department emergency response in the Gaza Strip and the occupied West Bank

In response to the severe challenges following October 2023, the UNRWA Health Department initiated a strategic collaboration with Johns Hopkins University to undertake a comprehensive and independent evaluation of its health response in the Gaza Strip and the northern part of the occupied West Bank, including East Jerusalem. This partnership reflects a deliberate shift toward evidence-driven learning, aiming to document performance during crises and critically assess how services were sustained, adapted, and delivered amid extreme disruption.

The analysis places particular emphasis on service continuity, tracking key indicators such as consultations, maternal and child health services, vaccinations, and non-communicable disease care, while exploring operational adaptations such as mobile clinics and alternative delivery mechanisms. Simultaneously, qualitative engagements aim to capture the lived experiences of providers and Palestine Refugees and communities, shedding light on determination, coordination, and system-level challenges. The findings of this evaluation are expected in the second quarter of 2026 and will serve as a critical milestone in reinforcing UNRWA's commitment to accountability, learning, and the continuous strengthening of its emergency health operations.



Palestine Refugee patient visiting the temporary Jenin health medical point (at Jenin Club) in the occupied West Bank, including East Jerusalem. ©2025 UNRWA photo.



UNRWA health staff using the electronic medical record system (e-health) at Hamad Health Centre in the Khan Younis area, the Gaza Strip. ©2025 UNRWA photo.

A step forward: Moving to a new electronic medical record software

The new EMR

UNRWA has decided to replace the current e-health system with a new Electronic Medical Record (EMR) system. The main aim is to enhance functionality to support additional business processes and patients' needs, as the e-health system requires continuous upgrades and integrations to deliver health services that meet these needs.

The following weaknesses would be improved by implementing the new EMR:

- The current e-health system is based on old technologies dating back to 2009 and is prone to performance lags and other issues
- Key functionality, required for the efficient provision and management of UNRWA healthcare services, is missing in the existing e-health system, such as:
 - An efficient and flexible ad-hoc reporting tool for administrative and epidemiological surveillance needs.
 - A restricted access to sensitive information, such as survivors and persons affected by abuse, and adopting the use of survivor-centred terminology in the context.
- Clearly specified role-based access controls, standardised consent forms, and confidentiality procedures.

- Clinical documentation and workflow support for management of mental health, management of gender-based violence (GBV), school health, and physiotherapy.
- Comprehensive clinical decision support functionality.
- Patient portal.
- Expandable telemedicine service integrated in the EMR system to include all provided health services, which will reduce walk-in clients and address the overcrowding issues in clinics.

Key Deliverables

- A modern and architecturally sound EMR system, high-performance, patient-centric, and sustainable, is implemented and rolled out in all 123 health centres Agency-wide by 2027.
- At least 3000 clinical and administrative staff of health centres, field offices and HQ are trained to use the new EMR system;
- Patient engagement portal is implemented, and e-health services are provided to Palestine Refugees;

Impact:

1. The EMR system shall be proactive, not only for entering data but also by introducing the Clinical Decision Support tool.
2. The EMR system will also include additional functionality for mental health, GBV, school health and physiotherapy bundled with their evidence-based reporting outcome and KPIs.
3. Enhanced and extended reporting and KPI monitoring capabilities of the existing BI solution, i.e. development of new data structures and management information reports in the Agency's Enterprise Business Intelligence solution, as needed for trend analysis over time and for cross-functional reporting.
4. In addition, the new EMR system will support the required integration with other UNRWA-operated IT systems, such as the UNRWA ERP system, the refugee registration system, and the education management

system. Accordingly, data across the Agency will be consistent and shall be updated where necessary.

5. The EMR system shall include a patient engagement portal as one of its components, which is compatible with computer browsers and smartphones, enabling and supporting patients through the following digital services:
 - Appointment booking,
 - Viewing patient health information.
 - Provision of alerts and reminders.
 - Provision of certificates and documents.
 - Provision of telemedicine services.

The impact will be measured on the percentages of health centres and health professionals trained and using the new EMR system. In addition, the availability of digital patient data, e.g., school entrants' medical examinations, physiotherapy records, mental health patients, and GBV survivors, will be measured. Finally, the percentage of patients using the patient engagement portal will be assessed.

Strategic health indicators across (Jordan, Lebanon, Syria, and the occupied West Bank, including East Jerusalem)

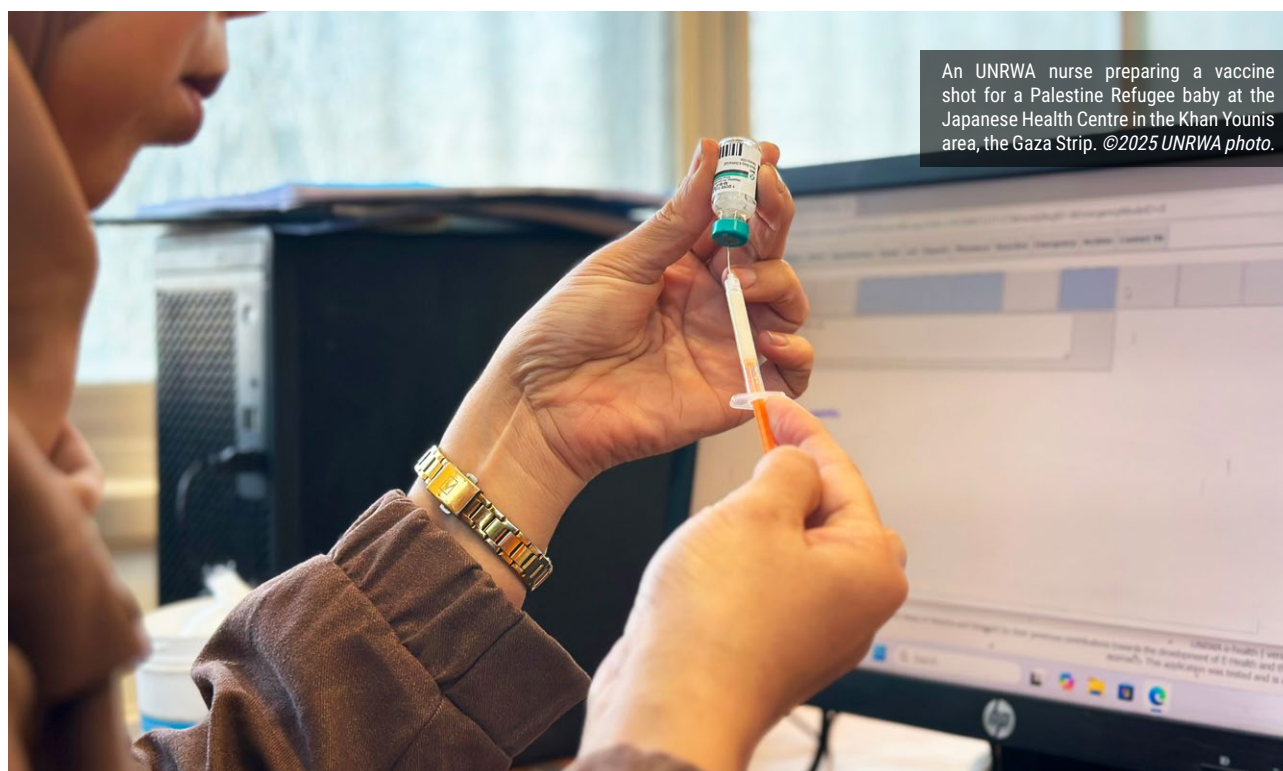
Indicator	Disaggregation	Baseline	2025 Target	2025 Actual	RBM Assessment
Percentage of women with birth intervals of less than 36 months	Total	51.84	51.32	50.3	Ahead of plan
Maternal mortality rate (Per 100,000 live births)	Total	21.73	20	18	On plan
Neonatal mortality rate (Per 1,000 live births)	Total	0.6	10.77	0.3	Ahead of plan
Neonatal mortality rate (Per 1,000 live births)	Females	1	4.46	0.6	On plan
Neonatal mortality rate (Per 1,000 live births)	Males	0.1	15.42	0.1	
Percentage of pregnant women registered during the first trimester	Total	72.51	73.24	72.78	On plan
Percentage of women with live birth who received at least four antenatal care visits	Total	84.72	85.57	86.11	Ahead of plan
Percentage of women of reproductive age using modern contraceptives	Total	66.65	67.32	64.78	On plan
Percentage of post-natal women attending post-natal care within six weeks of delivery	Total	90.41	93.12	91.55	
Percentage of 18 month old children that received two doses of Vitamin A	Total	97.68	95	97.74	Ahead of plan
Percentage of children aged 36-60 months who are screened for a disability	Total	97.46	99	98.13	On plan
Percentage of children aged 36-60 months who are screened for a disability	Females	97.58	99	98.26	
Percentage of children aged 36-60 months who are screened for a disability	Males	97.34	99	98.01	
Percentage of new entrants to UNRWA schools examined during their first school year	Total	99.46	99	99.65	Ahead of plan
Percentage of new entrants to UNRWA schools examined during their first school year	Females	99.55	99	99.87	
Percentage of new entrants to UNRWA schools examined during their first school year	Males	99.36	99	99.39	

Percentage of diabetes mellitus patients under control	Total	32.72	33.37	34.16	
Percentage of diabetes mellitus patients under control	Disability	36.87	37.61	38.31	
Percentage of diabetes mellitus patients under control	Females	35.32	36.03	36.8	
Percentage of diabetes mellitus patients under control	Males	28.8	29.38	30.14	
Percentage of targeted population 40 years and above screened for diabetes mellitus, hypertension or both	Total	45.88	50.4	52.3	
Percentage of non-communicable disease patients coming to health centres regularly	Total	82.25	83.07	82.77	On plan
Percentage of positive MHPSS cases identified through the GHQ-12 screening tool	Total	9.58	9.68	11.65	Ahead of plan
Percentage of positive MHPSS cases identified through the GHQ-12 screening tool	Disability	13.38	13.51	19.21	
Percentage of positive MHPSS cases identified through the GHQ-12 screening tool	Females	9.66	9.76	11.88	
Percentage of positive MHPSS cases identified through the GHQ-12 screening tool	Males	9.15	9.24	10.43	
Percentage of individuals identified with MHPSS needs provided with assistance	Total	97.89	95	98.65	
Percentage of individuals identified with MHPSS needs provided with assistance	Disability	98.39	95	99.67	
Percentage of individuals identified with MHPSS needs provided with assistance	Females	97.75	95	98.68	
Percentage of individuals identified with MHPSS needs provided with assistance	Males	98.74	95	98.52	
Number of vaccine preventable disease outbreaks	Total	0	0	0	On plan
Percentage of children 18 months old that received all booster vaccines	Total	98.77	95	97.91	Ahead of plan
Average consultation time per doctor	Total	2.3	3	2.34	On plan
Average number of daily medical consultations per doctor	Total	71.62	70.9	71.11	
Average number of daily medical consultations per doctor	Females	44.62	44.17	44.25	
Average number of daily medical consultations per doctor	Males	27	26.73	26.86	
Percentage of health centres with no stock out of 12 tracer medicines	Total	67.28	100	69.54	Lagging behind target
Antibiotic prescription rate	Total	20.5	25	20.34	Ahead of plan
Percentage of medical officer staffing alignment with Agency norms	Total	71.75	75	75.85	
Percentage of SSNP-eligible patients accessing hospitalization services per UNRWA defined criteria	Total	21.51	21.73	18.72	On plan
Percentage of SSNP-eligible patients accessing hospitalization services per UNRWA defined criteria	Females	20.39	20.59	17.73	
Percentage of SSNP-eligible patients accessing hospitalization services per UNRWA defined criteria	Males	23.45	23.68	20.32	
Percentage of GBV survivors identified out of the total number of women screened for MHPSS	Total	3.38	3.43	4.76	Ahead of plan

Research and information dissemination

Department of health research activities and published papers 2025

S.N	Month/Year	Authors	Title	Journal	Type	Language	Link
1	Jan-2025	Aljadba G; Elkhatib Z; Hamad K; Shah S; Zeidan W; Albeik S; Seita A	UNRWA's health service in Gaza: Challenges and response to the October 2023 crisis	Eastern Mediterranean Health Journal	Journal Article	English	
2	Feb-2025	Jamaluddine Z; Suarez Idueta L; Paixao ES; et al.	Post-term births as a risk factor for small for gestational age births and infant mortality in Brazil, Mexico, and Palestinian refugees	Paediatric & Perinatal Epidemiology	Journal Article	English	
3	May-2025	Alkhammash H; Al Najjar S; Tabaza A; Horino M; Chen J; Jaffal R; Al Jadba G; Seita A	Integrating emergency nutrition into UNRWA's humanitarian response in Gaza	Eastern Mediterranean Health Journal	Journal Article	English	https://applications.emro.who.int/EMHJV31/04/1020-3397-2025-3104-274-280-eng.pdf
4	May-2025	Horino M; Al-Khatib L; Habash R; Al-Rahahleh T; Kraemer K; Hurley KM; Seita A; West KP	Antenatal multiple micronutrient versus iron-folic acid supplement delivery in the UNRWA healthcare system: an implementation trial in Jordan	Current Developments in Nutrition	Abstract	English	https://cdn.nutrition.org/article/S2475-2991(25)02285-1/pdf
5	May-2025	Paris L; Al-Jadba G; Zeidan W; Spiegel P; Elkhatib Z; Habash R; Al Najjar S; Khammash H; Albeik S; Shaer T; Ramadan M; Seita A	Deterioration of health outcomes in Gaza: 19 months of protracted conflict	Lancet	Journal Article	English	
6	Sep-2025	Kissell MC; Pereira C; Gomes F; et al.	Acceptability of Antenatal Multiple Micronutrient Supplementation (MMS) Compared to Iron and Folic Acid (IFA) Supplementation in Pregnant Individuals: A Narrative Review	Nutrients	Journal Article	English	https://www.mdpi.com/2072-6643/17/18/2994
7	Oct-2025	Horino M; Al Najjar S; Tabaza A; Alkhammash H; Jaffal R; Chen J; Al-Jadba G; West KP Jr; Seita A	Assessment of malnutrition in preschool-aged children by mid-upper arm circumference in the Gaza Strip (Jan 2024–Aug 2025)	Lancet	Journal Article	English	https://www.thelancet.com/journals/lancet/article/PIIS0140-6736(25)01820-3/fulltext
8	Oct-2025	Horino M; Al-Khatib L; Habash R; Al-Rahahleh T; Seita A; Ahmed S; Kraemer K; Hurley KM; West KP Jr	Validation Study of Adherence to Daily Multiple Micronutrient Supplementation by Palestinian Refugee Women Attending UNRWA Clinics in Jordan	Current Developments in Nutrition	Journal Article	English	https://cdn.nutrition.org/article/S2475-2991(25)03050-1/fulltext
9	Oct-2025	Jamaluddine Z; Campbell OMR; Sato M; Seita A; Ohuma EO; Frongillo EA; Ghattas H	Association between size at birth, rapid weight gain in infancy, and overweight status among Palestinian refugees under 5 years	American Journal of Clinical Nutrition	Journal Article	English	
10	Oct-2025	Vollebregt SJC; Arafah A; Fouad H	Prevalence of tobacco use among adolescent Palestine refugees: A cross-regional UNRWA study	Preventive Medicine Reports	Journal Article	English	



An UNRWA nurse preparing a vaccine shot for a Palestine Refugee baby at the Japanese Health Centre in the Khan Younis area, the Gaza Strip. ©2025 UNRWA photo.

Disseminated abstracts: Delivering lifesaving diabetes care during the crisis

[I am the pancreas: a mother's voice from Gaza must not be silenced](#)

We are mothers from Gaza. My son has lived with type 1 diabetes for eight years. Since the war began in October 2023, caring for him has become a daily struggle for survival. With electricity cuts, limited food, and no stable access to insulin, I had to become his pancreas—playing the role of a health worker and protector. We are not alone. Through a WhatsApp group of over 40 mothers, we share life-saving tips and emotional support to manage our children's diabetes in a collapsing healthcare system. These are our shared stories. We documented our lived experiences over 20 months (October 2023 - July 2025) through conversations, messages, and voice notes exchanged in the group. This case narrative reflects our collective coping strategies and emotional burden of being under siege. We had to reduce the long-acting insulin from 32 to 10 units to make it last. On days when only one meal was consumed, rapid-acting insulin was skipped. Without power, we stored insulin in wet clothes, cooling bags, or neighbours' fridges. Sugar has become a life-saving medicine, costing up to \$100/kg, and is hidden for emergencies. One mother lost her daughter to hypoglycaemia due to a lack of sugar at home. We shared these tragedies in the group—not just as grief but as urgent lessons. We stayed up at night to monitor our children's breathing. We told our daughters, "This sugar is for your brother. His life depends on it." We became the pancreas of our children. In Gaza, where

the health systems have collapsed, mothers are the last line of defence. Diabetes care in emergencies must support us with insulin, storage solutions, access to sugar, and mental healthcare. Our voices must be heard. Our children's lives depend on this.

[Insulin supply in the Gaza Strip: Lessons from three critical cold-chain incidents across the humanitarian supply chain](#)

In conflict-affected settings, maintaining the cold chain for insulin is critical but highly challenging. In the Gaza Strip, UNRWA maintained continuous insulin availability over 20 months while managing three separate temperature-breach incidents between February 2024 and May 2025. UNRWA teams conducted an analysis to highlight the operational challenges associated with insulin delivery during conflict-related operational constraints. Three insulin cold chain breaches between February 2024 and May 2025 were reviewed using data logger records, inspection reports, and manufacturer correspondence. For each case, we assessed the type and duration of the temperature excursion, the quantity affected, the cause, the manufacturer's advice, and the mitigation measures agreed with the producer and partners. Three cold chain breaches affected the insulin stock. The first incident occurred during cross-border transport, where insulin was exposed to 16.9°C for 10 days

during a cross-border transport restriction. Although 218,000 units were affected, the manufacturer deemed them usable. The second incident, in Deir Balah, involved 148,770 units that were exposed to -3°C for 90 min due to an infrastructure failure; the stock was declared compromised and disposed of. The third breach, in a rented Jordanian warehouse, exposed 220,500 units to subzero temperatures for over 30 days; this stock was also deemed compromised. In total, over \$870,000 worth of insulin was lost, equivalent to 12 months of average consumption. Mitigation measures included redistributing safe stock, limiting buffer quantities within the Gaza Strip, and switching warehouse providers in Jordan. These incidents highlight the difficulty of maintaining cold chain integrity during the Gaza war. Israel authorities' tight control over logistics and the use of life-saving supplies as political leverage have undermined the protection of essential medicines such as insulin. Even outside the Gaza Strip, the indirect effects of the war forced reliance on a commercial warehouse in Jordan that was not originally prepared for medical cold storage. UNRWA has taken multiple adaptive responses, but the continuity of insulin treatment remains a struggle unless humanitarian principles are upheld.

Insulin regimen patterns in unrwa fields: gaps and opportunities in type 1 diabetes care

In conflict-affected settings, maintaining the cold chain for insulin is critical but highly challenging. In the Gaza Strip, UNRWA maintained continuous insulin availability over 20 months while managing three separate temperature-breach incidents between February 2024 and May 2025. UNRWA teams conducted an analysis to highlight the operational challenges associated with insulin delivery during conflict-related operational constraints. Three insulin cold chain breaches between February 2024 and May 2025 were reviewed using data logger records, inspection reports, and manufacturer correspondence. For each case, we assessed the type and duration of the temperature excursion, the quantity affected, the cause, the manufacturer's advice, and the mitigation measures agreed with the producer and partners. Three cold chain breaches affected the insulin stock. The first incident occurred during cross-border transport, where insulin was exposed to 16.9°C for 10 days during a cross-border transport restriction. Although 218,000 units were affected, the manufacturer deemed them usable. The second incident, in Deir Balah, involved 148,770 units that were exposed to -3°C for 90 min due to an infrastructure failure; the stock was declared compromised and disposed of. The third breach, in a rented Jordanian warehouse, exposed 220,500 units to subzero temperatures for over 30 days; this stock was also deemed compromised. In total, over \$870,000 worth of insulin was lost, equivalent to 12 months of average consumption. Mitigation measures included redistributing safe stock, limiting buffer quantities

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Gaza's insulin emergency: conflict-related disruption and glycemic outcomes in type 1 and 2 patients (2023–2025)

This study offers the first operational evidence from the Gaza Strip showing how prolonged conflict and access restrictions have rendered insulin-dependent diabetes care highly unstable in the region. Without access to food, refrigeration, diagnostics, hygiene, or appropriate insulin, treatment becomes unsafe. Since March 2025, food access has been severely constrained, leaving patients reliant on canned food, while physicians are forced to adjust insulin dosing based on the only available type, often unsuitable amid severe malnutrition. The lack of diabetes-appropriate nutrition highlights a critical gap, as standard humanitarian responses may not be adequately equipped to meet the needs of this vulnerable population. This study examined insulin-treated patients with type 1 and 2 diabetes at UNRWA/Gaza with at least two HbA1c or glucometer records (excluding GDM and incomplete files), analysing how conflict-related malnutrition and diagnostic gaps affect care. It compares HbA1c trends, hypoglycemia, admissions, and treatment changes across the pre-crisis ($\leq$ October 7, 2023), escalation (March –May 2024), and peak crisis (January–June 2025) periods. Using UNRWA electronic health records, a retrospective cohort of approximately 2,300 insulin-treated patients with type 1 and 2 diabetes was analysed across the three time periods. Patient records were combined with interviews to capture clinical trends and system adaptations. Worsening glycemic control, rising hypoglycemia, higher hospitalisation rates, and increased treatment improvisations are anticipated, including unsafe dose swaps, missed doses, and the use of oral agents driven by availability rather than clinical guidance. The final analysis is expected to be completed by October. This study provides critical evidence and underscores the urgent need to safeguard basic diabetes care to prevent iatrogenic harm, rising complications in patients with type 2 diabetes, and underreported NCD deaths. To avert avoidable mortality during crisis, diabetes care must evolve through adapted insulin protocols, inclusion of diabetic nutrition in NCD kits, practical diagnostic alternatives, and improved coordination between UN agencies and the pharmaceutical industry.

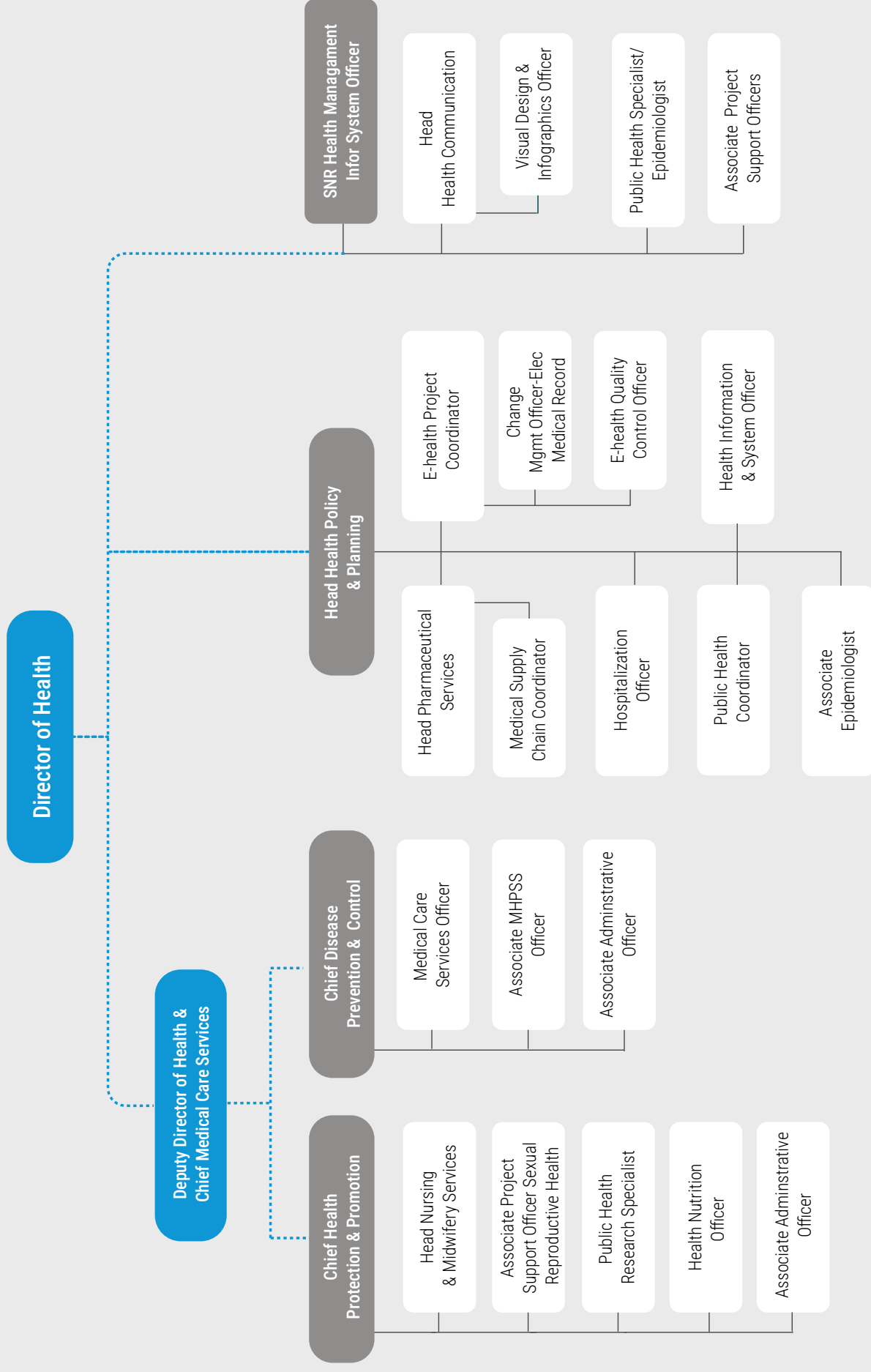
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department of health organisation chart



annexes

A link for the annexes will be posted here.

references

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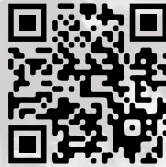
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وكالة الأمم المتحدة لإغاثة وتشغيل
اللاجئين الفلسطينيين في الشرق الأدنى